ASHA Saath

From a feasible visit plan to accountable local assistance

FULL RESEARCH REPORT

24 additional programmes | 30 platforms/tools | 10 research-agent assignments

India - research cut-off 8 October 2026
Healthcare 03, Codefiesta 5.0

Government-service navigation, participating NGOs, local catchments, consent, capacity, referral outcomes and a practical hackathon MVP.

Evidence-backed desk research and proposed design. No secured partnerships, live government integrations, guaranteed funding or measured clinical impact are claimed.

For quick reading, use the companion Hinglish decision brief. Long comparison rows are shown as labelled entries in this PDF; the editable Markdown preserves the matrices.

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ASHA Saath - a feasible visit plan and accountable local assistance

Research cut-off: **8 October 2026, India**. Prepared for the team's chosen Healthcare 03 problem at Codefiesta 5.0.

Read this first

The recommended product combines the required ASHA/ANM visit optimizer with one narrowly scoped assistance workflow. A visit produces a consented support need; the relevant public-service pathway is checked; a participating provider with the right catchment, eligibility and current capacity accepts or declines; delivery and beneficiary/worker confirmation are recorded; unresolved cases retain an owner and next action.

The proposal remains subject to local validation. No real NGO partnership, government production integration, money raised, clinical benefit, travel improvement or winning probability is claimed. A funded coordinator and real local service offers are prerequisites for a meaningful pilot.

The report catalogues **24 additional schemes/programmes** and **30 platforms/tools**. ASHA and the 2026 SSBSK operational framework are discussed separately, outside the additional 24 count. Related programmes, integrated systems, campaigns and overseas comparators are labelled; their beneficiaries and markets must not be added together. Shaishav is an officially described 2026 tool whose universal deployment was not verified.

Ten distinct research-agent assignments were used in batches: ASHA ground truth; maternal/child services; digital-health products; welfare/inclusion; NGO networks; public-service/livelihood programmes; civic access and financing; service handoffs and stakeholder perspectives; MVP engineering; and independent audit. The parent agent performed additional comparator research, synthesis and artifact preparation.

Evidence is distinguished as official documentation, provider-described capability, setting-specific research, design proposal or validation hypothesis. A feature missing from a public page is not automatically absent from the product. See the method note and the audit.

Practical reading order

For a quick decision, read **asha-ngo-decision-brief.md**. In this full report, read the operating model, ten-perspective matrix and MVP before using the catalogue for programme-specific design. The original problem requirements and 2026 policy updates matter before implementation. All detailed source links remain inline and are also indexed at the end.

Catalogue coverage

Programme group	Eight additional entries
Maternal, child and hospital care	JSY; JSSK; PMMVY; PMSMA; SUMAN; Mission Indradhanush; UIP; PM-JAY
Welfare, disability, adolescent and TB support	NSAP; ADIP; Rashtriya Vayoshri Yojana; RBSK 2.0; RKSK; NMHP (including Tele-MANAS); NTEP; PM TB Mukt Bharat Abhiyaan / Ni-kshay Mitra
Public services and livelihoods	NHM Free Drugs; NHM Free Diagnostics; Ayushman Arogya Mandir; DAY-NRLM; NFSA/TPDS; Saksham Anganwadi and POSHAN 2.0; Jal Jeevan Mission; PMBJP / Jan Aushadhi

Platform group	Reviewed entries
Digital health - 8	ANMOL; RCH Portal; U-WIN; AMRIT FLW; CommCare; eSanjeevani; ABDM ecosystem; Ni-kshay
NGO networks - 8	NGO Darpan; GuideStar India; Give ecosystem; GlobalGiving; NGOBOX; Goodera; iVolunteer; ConnectFor
Access and financing - 8	myScheme; UMANG; Rajasthan eMitra; CSR Exchange; National CSR data portal / CDM; Haqdarshak; Milaap; Ketto
Additional comparators - 6	Khushi Baby CHIP; OpenSRP; Unite Us; Findhelp; Shaishav App; Poshan Tracker

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Research method

Research method and evidence limits

Research cut-off: 8 October 2026. This work supports Healthcare 03 at Codefiesta 5.0 and the team's proposed ASHA-to-NGO support extension.

The user requested ten research agents. Ten distinct research assignments are being completed in batches because only three child agents can run concurrently. The parent also researches and synthesizes evidence. This is desk research, not stakeholder interviews, authenticated product testing, legal certification, or a field evaluation.

Evidence categories used throughout the report:

• **Official:** government guidelines, scheme pages, parliamentary answers and official review reports. Eligibility, rollout and benefits are date-sensitive.

• **Provider-documented:** product operators' descriptions and documentation. Features and claimed reach are not independently validated outcomes.

• **Study-specific:** published research with a particular geography, sample and period. Findings are not automatically national estimates.

• **Design proposal:** our suggested implementation, targets, architecture or operational model.

• **Validation question:** an unconfirmed operational gap to investigate with practitioners.

Connected Tavily research returned transport timeouts (HTTP 504); searches/extracts also encountered usage limits. Research continued using direct web searches and primary-source pages. No successful automated Tavily deep-research report is being claimed.

The official hackathon page was read earlier in this conversation with expanded statements. The current extraction reconfirmed the Healthcare 03 title but did not expand the requirements. The original expanded requirements are retained with that provenance; current direct web access to the page was also limited.

Counts describe catalogued schemes/programs and software/services, not unique beneficiary populations or unique companies. Related programs may share infrastructure, eligibility or beneficiaries. In particular, UIP and Mission Indradhanush overlap; NTEP and PM TB Mukt Bharat Abhiyaan complement each other. A large registration or expenditure total is not a startup's obtainable market, a fund balance or a count of active, available local services.

Geographic coverage, staff capacity, funding availability, API access and willingness to participate must be confirmed with each partner. A feature not described on a public webpage is **undocumented in this review**, not proven absent. No platform is declared universally broken or the proposed system globally unprecedented.

No NGOs, patients, funders or government officials were contacted; no money was requested or transferred; no live health-system access was obtained.

Part 1. ASHA and the official problem statement

ASHA/ANM ground truth for Healthcare 03 and an NGO-support extension

Research cutoff: **8 October 2026, India**. Prepared as one research contribution to the requested ten-agent review. All recommendations below are design proposals, not authorization from NHM and not clinical advice. Sources were checked with connected Tavily initially and the built-in web tool after Tavily research returned HTTP 504 and search reached its usage limit. No outreach, accounts, deployment, or movement of funds occurred.

1. What the hackathon actually asks for

The official problem-statement listing was successfully extracted on 8 October 2026 and confirms **Healthcare 03: Community Health Worker (ASHA/ANM) Visit Scheduling Optimizer with Risk-Weighted Prioritization**. The listing says 14 themes and 70 statements, released 8 October. Current extraction exposed the title, not the expanded panel. Official Codefiesta 5.0 problem statements.

The parent research brief records previously read expanded requirements: import a caseload CSV containing patient ID, location and rule-based risk; cluster visits and produce routes; prioritize with risk and travel estimates; display daily routes, risk flags and coverage. Those requirements are **carried forward from the earlier reading**, not claimed as text independently re-extracted here. The official panel should be saved or reread before presenting an exact quotation to judges.

The upgrade should therefore retain a visible working optimizer. A good demonstration starts with a CSV, explains why each household is due, produces a feasible daily visit sequence, and shows who could not be covered. NGO assistance is an additional response to barriers discovered during those visits. A general donation marketplace would weaken alignment with this PS.

2. The worker is a trusted community interface with a defined scope

NHM describes ASHA as a village-selected, trained woman linking communities to public health services. Duties include counselling, health education, mobilization for government services, limited protocol-appropriate first-contact care, and timely referrals. Selection generally prefers resident women aged 25-45 and gives preference to education through class 10, with relaxation when suitable candidates are unavailable. She receives performance incentives and depends on institutional support, including ANMs and Anganwadi workers. The NHM page has a website footer dated 14 September 2026; that footer is not proof that every item, including its old drug list, was freshly revised in 2026. NHM: About ASHA.

NHSRC, NHM's technical support institute, frames ASHA's three functions as facilitating healthcare, providing defined community services, and health activism. Its current catalogue recognizes ASHAs within Ayushman Arogya Mandir primary-care teams and describes expanded community work including chronic disease, mental health, elderly and palliative care, alongside maternal/child and communicable-disease work. It lists continuing training, certification and support materials, including an April 2026 certification letter. These are trained tasks, not unrestricted medical authority. NHSRC: Community Processes-ASHA.

Scope-to-product mapping (design recommendation)

Person or organization	Appropriate product capability	Boundary to preserve
ASHA	Confirm household location and consent, view assigned visits, record observations using approved forms, identify a need, initiate escalation/referral, track follow-up	Do not ask her to diagnose unfamiliar illness, override a clinician, or become a fundraising case manager by default
ANM	Clinical assessment and service delivery within qualifications/protocols; validate relevant findings; oversee and join indicated visits	Do not equate an ASHA visit with an ANM examination, immunization, or clinical clearance
CHO/medical officer	Own locally approved clinical pathways and technical escalation; review complex cases and appropriate referral destination	Software rankings and NGO volunteers cannot substitute for this authority
ASHA facilitator/community-process supervisor	Help with workload, mentoring, missed households, logistics, training and quality	Keep supportive review distinct from diagnosis and from punitive GPS surveillance
AWW/Anganwadi centre	Nutrition and child-development linkage, eligible supplementary nutrition and coordination	Avoid inventing dietary treatment or treating charitable food as the child's entire care pathway
NGO caseworker/volunteer	Accept a specifically consented support request, confirm service capacity, arrange nonclinical assistance and report fulfillment	No automatic access to full patient records; no ability to change clinical risk classification or certify health outcomes
District/block program authority	Approve worker mappings, rules, partner participation and monitoring arrangements	A registration badge or app account is not government endorsement

The official rural coordination table assigns ASHA mobilization/home education, ANM clinical outreach and joint support, and AWW nutrition responsibilities. The same guidelines place immediate field support with ANM and ASHA facilitator. They provide a rural norm of one ASHA per 1,000 people, allowing habitation-based relaxation for difficult terrain. They envisage approximately 3-4 hours of overall ASHA work daily on 4-5 days a week, with roughly two hours for home visits; these are historical planning norms, not observed 2026 hours. They also encourage competency-based NGO partnerships for training and support. Community Processes Guidelines, catalogued as 2014; forewords dated 26 June 2013; pp. 5-13.

3. Geography and coverage are not interchangeable

Urban guidelines use one ASHA per **1,000-2,500 population**, approximately 200-500 households, with smaller coverage possible for dispersed or distinct vulnerable groups. Mahila Arogya Samitis generally cover 50-100 households, or 250-500 people. The framework targets vulnerable urban populations and gives states contextual flexibility. This is an older framework still hosted by NHM; the web search's recent crawl/publication labels should not be mistaken for its original issue date. NUHM implementation framework, 2013-era framework; NHM orientation module: selection norms.

For scale, Parliament reported **1,030,992 ASHAs in position**, including **53,914 in Rajasthan**, using NHM-MIS data as of **30 September 2024**. The answer was issued **25 March 2025**. Those are dated counts, not an October 2026 census. It also explains that NHM releases state funds through flexible pools, rather than a separate ASHA allocation for every state. Rajya Sabha question 2830, annexure I.

Design implications:

• Model the **assigned households**, not a blanket radius around a worker. Someone living outside the efficient cluster may still be her responsibility.

• Keep village/hamlet/ward, block, district, state, rural/urban and coordinates as different fields. Administrative matching and travel matching serve different purposes.

• Rural walking paths, seasonal accessibility and urban settlement entrances need manual correction. A map coordinate does not prove a road is usable.

• National NGO visibility is not local service availability. Match requests to verified district/ward coverage, current capacity, eligibility, opening times and actual contact channels.

• Repeatedly deferred, inaccessible, migrant or unlocated households belong in the coverage gap list. Do not quietly drop them to improve a distance metric.

4. Critical 2026 update: SSBSK, not a frozen HBNC/HBYC model

On **29 June 2026**, MoHFW launched **Samagra Shishu Bal Swasthya Karyakram (SSBSK)**, integrating HBNC and HBYC into home/community care from birth through **36 months**. Its announced approach includes coordinated ASHA, ANM, CHO and AWW visits and early childhood development. PIB/MoHFW launch release, 29 June 2026.

The **25 August 2026** government workshop release confirms risk-differentiated care, additional follow-up for at-risk children, joint visits and clinical oversight. It describes **Shaishav App** for at-risk line-listing, scheduling/tracking visits, referral follow-up, supportive supervision and longitudinal child profiles. State-specific implementation readiness was under discussion. This verifies the policy direction and an existing digital product; it does **not** establish uniform adoption in every district by October. PIB/MoHFW SSBSK workshop, 24-25 August 2026.

NHM lists **SSBSK Operational Guidelines 2026**, alongside legacy HBNC and HBYC documents. The official listing was read, but the 67.1 MB guideline PDF could not be retrieved or its exact visit table independently verified in this run. An unofficial mirror contains purported new schedules; it is deliberately excluded from clinical defaults here. **Do not label legacy schedules below as the unchanged current SSBSK schedule.** Obtain the official 2026 table and the pilot district's approved implementation instructions before configuring production rules. NHM SSBSK guideline catalogue, website updated 7 October 2026.

Verified legacy schedules for a clearly labelled demonstration

NHM's community-programme page still gives HBNC visits after institutional birth on days **3, 7, 14, 21, 28 and 42**, with an additional visit within 24 hours for home delivery. It identifies additional follow-up for preterm, low-birth-weight, ill and SNCU-discharged babies. Its HBYC description gives visits at **3, 6, 9, 12 and 15 months**. Both descriptions are legacy programmes; this page's August 2026 website timestamp does not resolve the transition to SSBSK. NHM community-based child health programme.

An older official urban ASHA training module corroborates those HBNC dates and teaches prompt facility referral for danger signs. It places pregnancy assessment/referral decisions with trained facility personnel and ANMs. This provides useful role separation but should not be mined for current medication doses or modern diagnostic thresholds. NHSRC urban ASHA induction module, publication day not stated in retrieved text; hosted under 2021 directory.

Recommended rule record: `rule_id`, `program`, `version`, `source_url`, `published_date`, `effective_date_if_known`, `district_approval`, `eligible_cohort`, `due_date/window`, `required_cadre`, `joint_visit_required`, `escalation_path`, `review_date`. Unknown or conflicting rules should be shown to the supervisor; no silent rule switching.

5. Incentives and workload: avoid false current-pay claims

The official answer to **Rajya Sabha question 276, 22 July 2025** states that the fixed monthly incentive for routine/recurring activities was revised from ₹2,000 to **₹3,500** at the **4 March 2025** Mission Steering Group meeting. Task/performance incentives and state top-ups also apply; ASHAs remain envisaged as community health volunteers. This is the latest dated central revision directly verified here. It does not mean ₹3,500 is salary, total monthly earnings, guaranteed receipt, or that there were no later state changes. Official Parliament answer.

A separate **July 2025 Lok Sabha answer** describes state-funded top-ups, AAM team incentives up to ₹1,000 monthly, nonmonetary support such as cycles and phones, and a ₹50,000 recognition amount for eligible leavers with at least ten years of service. Keep these dated and eligibility-specific. The exact parliamentary day was not recovered in this run. The NHM July 2025 incentive-table URL cited by Parliament returned a 404 during direct retrieval, so task-by-task amounts were not treated as newly verified. Lok Sabha question 1082, 2025.

Observed workload differs from a guideline allowance. A primary mixed-method study published in **2025** covered **26 ASHAs** in one urban and one rural medical-college field-practice area in Bengaluru, with fieldwork in **June-July 2024**. Ten participants reported 5-6 hours daily; some described delayed or incomplete incentives. This small local study supports the plausibility of workload and payment friction, not a national 2026 average. Primary study: work sampling assessment.

NHSRC's **FY 2024-25 work report**, hosted in February 2026, records a dedicated ASHA workload assessment and says its final report had been shared for presentation to MoHFW. The retrieved passage does not provide its numerical findings. Therefore no national hours-worked statistic is asserted from it. NHSRC work report FY 2024-25, p. 38.

Product implications:

• Worker-set availability, caregiving responsibilities, travel time, service time, existing outreach sessions, review/training meetings and escort duties must fit the day before adding NGO tasks.

• Show planned minutes, buffer, overruns and reasons; allow the ASHA to correct estimates.

• Measure time saved and fewer missed essential visits. Do not assume more visits per day always improves care.

• Separate clinically urgent priority from financial incentives. A scoring model should not favor whichever task pays more.

• Support requests should have an NGO/district coordinator owner. ASHA should not need to chase every donor or upload repeated proofs to complete her own visits.

• Record support completion independently of clinical visit completion and legitimate incentive documentation.

6. Existing government systems and credible differentiation

MoHFW's **Annual Report 2024-25** describes RCH as individual reproductive/child lifecycle tracking, and ANMOL as an ANM application with service due lists, workplans, VHND sessions, a dashboard, digital RCH register and counselling aids. It reported ANMOL in 28 states/UTs and approximately 1.82 lakh registered ANMs at that older report's reference point. Its list omits Rajasthan; do not assume the pilot uses ANMOL just because it is a national app. MoHFW Annual Report 2024-25, section 1.2.6. **Newer chronology:** the 2025-26 report states 29 States/UTs and about 1.82 lakh ANMs as of 31 December 2025; use that dated figure for the current platform comparison. MoHFW Annual Report 2025-26, p.50.

On **24 March 2026**, MoHFW reported that U-WIN already line-lists children due for vaccination, sends reminders and generates lists of people who did not attend for follow-up. Counts were explicitly as of **18 March 2026**. A scheduling app should not advertise vaccination due-list generation as a first-of-its-kind invention. PIB: Update on U-WIN, 24 March 2026.

The official Swasth Bharat portal's search-indexed FAQ describes a single access point for authorized program users and mapped accounts across health programs. The direct page returned only a JavaScript shell, so this is a discovery lead, not verification of every integration or launch date. Official Swasth Bharat portal.

Original design inference: The strongest positioning is an operational companion that combines due visits with terrain-aware routing, worker availability, joint-visit constraints and local nonclinical assistance. It can demonstrate import/export using a synthetic CSV. Actual RCH/U-WIN/Shaishav/state-system access requires authorized integration; no public API, government credential sharing or bidirectional sync was verified. Do not imply one.

7. NGO support already has a policy foothold, but must fit the health system

Urban ASHA/MAS guidelines explicitly describe NGOs as additional capacity for training and supportive community processes. They require rigorous, transparent selection, defined terms of reference and integration with the urban health system. This is a stronger basis than inventing a parallel clinical chain. NUHM guidelines for ASHA and MAS, NGO section p. 23.

Government entitlements should be checked before a charitable funding request. Under **JSSK**, NHM states that eligible public-facility delivery and specified pregnancy/infant care include free drugs, diagnostics, blood, diet and transport, including referral transport. The scheme launched **1 June 2011** and extended relevant entitlements in **2014**; the retrieved page was website-updated July 2026. Availability failures should be escalated and tracked, rather than automatically treated as a donor's obligation. NHM: Janani-Shishu Suraksha Karyakram.

Recommended referral workflow (design proposal)

1. **Plan the health visit.** Pull due tasks from authorized records or the demo CSV. Preserve scheduled visits and required cadre.

2. **Escalate urgent findings immediately through the locally approved clinical route.** Routine optimization must not defer an emergency to tomorrow or await NGO acceptance.

3. **Record a distinct support barrier.** Examples: cannot reach the appointment, missing documents for entitlement, unavailable caregiver, nutrition access, disability access or unsafe travel.

4. **Check government entitlement and existing facility/Anganwadi pathways.** Ask what assistance is actually missing.

5. **Obtain specific consent for a limited NGO referral.** Select necessary fields and recipient. Consent to a health visit is not consent to public fundraising.

6. **Match a partner by actual service.** Confirm geographic coverage, eligibility, cost, capacity, access hours, quality/safeguarding requirements and the date of the latest operational confirmation.

7. **Have the partner accept or decline.** A directory match is not a promise. Provide a coordinator fallback and expiry/escalation for an unanswered request.

8. **Coordinate fulfillment with the scheduled health care.** The NGO may arrange a ride or accompaniment; ANM/CHO/facility retains the clinical decision.

9. **Confirm completion with minimal evidence.** Record who accepted, when support happened and who followed up; protect household identity.

10. **Close two loops separately.** Support supplied does not mean referral completed or clinical risk resolved. A health worker confirms the latter within her role.

Candidate support types, without claims about any particular NGO

Support need	Appropriate potential NGO contribution	What the app must verify
Travel barrier	Accessible transport or accompaniment; transport vouchers administered by an accountable partner	Local availability, cost, consent, safeguarding and whether public transport entitlement already applies
Nutrition access	Entitlement help, delivery/access coordination, suitable food assistance under approved counselling	Program eligibility, actual supply, approved plan and follow-up owner
Documents/benefits	Help completing forms and accessing schemes	No sale of data, no retention of unnecessary Aadhaar/bank information, no promise of approval
Disability/elder support	Accessible equipment referral, mobility aid, caregiver support	Qualified assessment where required, correct fit, maintenance and service capacity
Health literacy	Approved-language education, local mobilization and digital assistance	Training, language, supervision and no unsupported clinical advice
Supplies	Funds or approved procurement for equipment, connectivity or logistics through an accountable organization	Procurement policy, stock, quality, responsible custodian and duplication with government supply
Treatment-related financial barrier	Case navigation and assistance through a verified organization's own accountable process	Eligibility, legitimacy, documented expenses, benefit coverage and protection of patient identity

Funding should be a later, explicitly governed support feature. In the hackathon MVP, show **noncash request, partner acceptance, fulfillment and audit history**. If a funding scenario is displayed, use synthetic data and a labelled simulated ledger. Do not pool money, promise a medical outcome, rank households by donor appeal, charge workers referral fees, or let an NGO payment alter clinical priority. A verified organization's existing funding process is a safer next-step concept than making this prototype a payment intermediary.

8. Preserve the optimizer and add accountable support

Recommended ordering for the scheduling engine:

1. Immediate locally defined escalation bypasses ordinary route scoring.

2. Required program dates/windows, indicated post-discharge follow-up and joint-cadre availability constrain the plan.

3. Risk tier, overdue duration and repeated missed coverage influence priority using explainable, approved rules.

4. Availability, care duration, route/access constraints and capacity determine feasibility.

5. Distance/time minimization chooses among clinically acceptable alternatives.

Show a reason for each priority, the rule/source/version, travel estimate confidence and a supervisor override with a reason. If mandatory work exceeds capacity, report the conflict and request coordination rather than fabricate a feasible schedule. A distant household's priority should not decay simply because it costs more time to reach.

Recommended MVP outputs: daily worker route, risk and due-date explanation, required joint visits, estimated travel/care time, unvisited/overdue queue, map correction, referral log, limited partner request and partner acceptance/decline. Suggested evidence metrics: risk-weighted visits completed before deadlines; essential overdue visits; travel minutes per valid visit; hard-to-reach household coverage; successful clinical referral follow-up; time to accepted nonclinical assistance; ASHA time spent on data entry/support chasing. All clinical outcome claims require a real evaluation beyond a hackathon.

9. Verification gaps to resolve before a pilot

• Exact SSBSK 2026 schedule/risk criteria from the **official PDF**, and state/district adoption and training status.

• The pilot site's existing system: RCH/ANMOL/Shaishav/Janani/U-WIN or a state implementation; permissible data exchange and duplication burden.

• Locally approved escalation destination, coverage hours, ambulance/transport arrangements and service capacity.

• Who actually approves the rule pack and NGO participation, and how the ASHA will access help when offline.

• Each partner's real district/ward service availability, eligibility, capacity, request channel and safeguarding practice. Registration alone does not verify delivery capability.

• Current state-specific incentive orders and actual local payment workflows if incentive tracking is included.

• Real observed travel/care times, missed-visit reasons and worker-set availability. Older national norms and small studies should not be substituted for local observation.

The proposed upgrade is credible when a high-priority visit leads to an accepted, consented, traceable support action that helps the person complete authorized care. Its strength is connecting operational barriers to responsible local assistance while retaining the original risk-weighted route planner.

Part 2. 24 programmes - maternal, child and hospital care

Eight maternal, child and health programs for ASHA-NGO coordination

Research cutoff: **8 October 2026, India**. This module contains exactly eight distinct program entries. Official sources were checked with connected Tavily search/extraction and direct web access; the Tavily deep-research request failed with HTTP 504, so findings were verified against the primary sources below. A website footer update is not treated as the date of a new scheme rule. Benefits require the administering authority's verification; local facilities, implementation orders and beneficiary status must be checked before a referral.

Comparison table

1. Janani Suraksha Yojana (JSY) - conditional maternity cash assistance

Beneficiary focus / eligibility gate: Institutional delivery; eligibility differs between low/high performing states and public/accredited private facilities

Principal verified benefit: Mother's institutional-delivery payment: LPS rural ₹1,400 / urban ₹1,000; HPS rural ₹700 / urban ₹600. ASHA payment is separate

Coverage and practical service point: National NHM scheme, state/district execution; delivery-capable public facilities and accredited private institutions

ASHA + NGO coordination opportunity: Explain eligibility, prepare delivery/referral plan, record application and payment status

2. Janani Shishu Suraksha Karyakram (JSSK) - free public maternity/infant services

Beneficiary focus / eligibility gate: Pregnant women using public facilities; antenatal/postnatal complications; sick newborns/infants up to one year

Principal verified benefit: Free delivery including C-section, specified drugs/diagnostics/blood/diet and transport; sick-infant treatment entitlements

Coverage and practical service point: National initiative delivered through state public health and referral networks, including rural and urban facilities

ASHA + NGO coordination opportunity: Facility/transport navigation, entitlement explanation and denial escalation

3. Pradhan Mantri Matru Vandana Yojana (PMMVY) - maternity DBT

Beneficiary focus / eligibility gate: Eligible disadvantaged women; first child or second child if girl; documentary, age and milestone rules

Principal verified benefit: First child ₹5,000 in two instalments; second girl ₹6,000 in one instalment, conditional on required records/milestones

Coverage and practical service point: PIB 12 Aug 2026: 35 States/UTs including Odisha; Telangana onboarding. AWC/ASHA, approved health facility and citizen portal

ASHA + NGO coordination opportunity: Document assistance, consent-based application support, ANC/immunization follow-up and payment tracking

4. Pradhan Mantri Surakshit Matritva Abhiyan (PMSMA) - fixed-day ANC program

Beneficiary focus / eligibility gate: Pregnant women in second/third trimester

Principal verified benefit: Free comprehensive ANC at designated government facilities on the ninth of each month; high-risk identification and referral

Coverage and practical service point: National program, locally designated rural/urban ANC sites and referral hospitals

ASHA + NGO coordination opportunity: Mobilize missed ANC visits, confirm session, close high-risk referral loop; appropriately registered clinicians can volunteer

5. Surakshit Matritva Aashwasan (SUMAN) - maternal/newborn service assurance

Beneficiary focus / eligibility gate: Women/newborns seeking public maternity/newborn services

Principal verified benefit: Respectful, quality care without expense; service-guarantee and grievance framework

Coverage and practical service point: National assurance initiative integrating existing services; verify district's designated facility network

ASHA + NGO coordination opportunity: Birth planning, respectful-care support, follow-up and beneficiary-led grievance assistance

6. Mission Indradhanush (MI) - targeted immunization catch-up campaign

Beneficiary focus / eligibility gate: Missed/unvaccinated/partially vaccinated children and pregnant women; campaign age range set by each round

Principal verified benefit: Due UIP vaccines delivered through intensified outreach; IMI 5.0 in 2023 included children up to five years

Coverage and practical service point: National strategy, district/urban/rural microplans, priority settlements and scheduled campaign sessions

ASHA + NGO coordination opportunity: Identify missed families, address access concerns, mobilize and link back to routine follow-up

7. Universal Immunization Programme (UIP) - continuing routine vaccination program

Beneficiary focus / eligibility gate: Children/adolescents and pregnant women according to the National Immunization Schedule

Principal verified benefit: Free scheduled vaccines, with vaccine-specific age and geographic rules

Coverage and practical service point: Nationwide routine fixed and outreach sessions: village/AWC/subcentre/PHC/urban sites/hospitals

ASHA + NGO coordination opportunity: Maintain due lists with ANM/AWW, remind families and reconcile administered doses

8. Ayushman Bharat PM-JAY - hospital health assurance

Beneficiary focus / eligibility gate: Verified eligible families and applicable added groups; separate universal income-independent pathway for Indian citizens aged 70+

Principal verified benefit: Up to ₹5 lakh per eligible family per year for covered secondary/tertiary hospitalization; 70+ rules include a separate senior top-up for already-covered families

Coverage and practical service point: National portable benefit through empanelled hospitals, subject to state implementation and package rules

ASHA + NGO coordination opportunity: Help verify eligibility/card and locate suitable hospital; hand off to Ayushman help desk and track discharge/referral

The table is a comparison of pathways, **not eight additive populations or eight NGO funding sources**. JSY, PMMVY, JSSK, PMSMA and SUMAN can concern the same pregnancy. MI delivers UIP vaccines; beneficiaries and doses must not be added twice. PM-JAY is a hospital benefit, distinct from primary-care services and from an ABHA identifier.

1. Janani Suraksha Yojana - JSY

Purpose and eligibility. JSY promotes institutional delivery to reduce maternal/neonatal mortality. NHM's current page distinguishes low performing states (LPS) from high performing states (HPS): all women delivering in government facilities in LPS; BPL/SC/ST women in HPS government facilities; BPL/SC/ST women in accredited private facilities in either group. The same page lists mother's cash packages as LPS ₹1,400 rural/₹1,000 urban and HPS ₹700 rural/₹600 urban. ASHA packages are **₹600 rural and ₹400 urban**, separate from the mother's benefit; the national page splits them into ANC and delivery-facilitation components. Do not advertise a combined mother-plus-ASHA figure as the mother's payment. Check the state's current classification and instructions, particularly where the national page retains legacy geography. NHM JSY page (publication date unstated; site footer 7 Oct 2026; accessed 8 Oct 2026).

Where/how to refer. National centrally sponsored scheme under NHM, implemented by states/districts. Identify the woman's ASHA/ANM and a delivery-capable government subcentre, PHC, CHC, first-referral unit or district/state hospital, or a currently accredited private institution. Obtain the facility's own eligibility/document checklist, record the submission acknowledgment, and separately track whether the mother's payment was received. Facility accreditation is a referral check, not an assumption based on private ownership.

ASHA and NGO complement. ASHA links the woman to ANC, delivery and post-delivery care. An NGO can help with language access, document preparation, travel planning, appointment reminders and payment-status escalation, with the woman's consent. An NGO's support should strengthen the ASHA/ANM/facility workflow, not make eligibility decisions or collect the woman's benefit.

Bottleneck evidence. CAG's Haryana Report No. 2 of 2024 documented delayed/non-payment and missing records in tested facilities for earlier periods, including 2016-21. This establishes a historical, location-specific payment-tracking problem; it does **not** establish the October 2026 delay rate in a new district. Missing banking/documents or late submission are operational risks to test locally, not claimed national prevalence. CAG Haryana public-health audit (2024, chapter 7).

NGO funding status. JSY payments are beneficiary/ASHA entitlements under applicable rules. No automatic NGO grant or outreach contract was verified. Discuss a costed support proposal with the District Health Society/state NHM only through a documented procurement or partnership route.

2. Janani Shishu Suraksha Karyakram - JSSK

Purpose and benefit. JSSK addresses out-of-pocket costs of public-facility delivery and sick-infant treatment. The NHM page confirms free delivery including C-section, drugs/consumables, diagnostics, blood when required, maternal diet for three days after normal delivery and seven after C-section, and home-to-facility/interfacility/drop-back transport. Entitlements extend to antenatal/postnatal pregnancy complications and sick newborns/infants **up to one year** in public institutions. These are service entitlements, not a cash transfer or a promise of equivalent reimbursement at any private hospital. NHM JSSK page (launch June 2011; extension recorded 2014; site footer 6 Oct 2026; accessed 8 Oct 2026).

Coverage and referral. National public-health initiative, operationalized by states through rural and urban public facilities and transport/referral systems. Map the nearest delivery point, C-section/referral hospital and facility able to treat a sick infant. Ask the ANM or facility to confirm transport booking, referral contact, diagnostics and required supplies; ASHA can explain entitlements and accompany/facilitate an appropriate referral within local duties. A family should not wait for a benefit application before seeking urgent clinical care.

NGO complement. Provide a local-language entitlement card, help families contact the public transport/facility team, support accessible navigation, and document charges or denial for beneficiary-led escalation. If an NGO offers backup travel or accompaniment from its own funds, clearly label it as NGO support and record the reason; it is not evidence that JSSK's public transport entitlement has disappeared.

Bottleneck evidence. The NHM page records high delivery-related expenses as the scheme's original rationale. Its historical background percentages must not be presented as current prevalence. Transport non-arrival, supply gaps and inappropriate charging are **local operational risks** requiring district service records or a current audit before being described as demonstrated failures.

Funding. No automatic NGO grant was verified. Referral-transport or other service contracts, where invited by a state, require their own tender, authorization and service standards; do not infer a contract merely because JSSK provides transport.

3. Pradhan Mantri Matru Vandana Yojana - PMMVY

Purpose, amounts and milestones. PMMVY partially compensates maternity wage loss and promotes health-seeking. WCD's official FAQ specifies first-child ₹5,000 as ₹3,000 after pregnancy registration and at least two ANC checks, preferably within six months of LMP, plus ₹2,000 after registered birth and completion of the first immunization cycle at fourteen weeks. For a **second child who is a girl**, ₹6,000 is paid in one instalment after the corresponding pregnancy, birth and immunization conditions. JSY can also apply, subject to its own rules; do not promise a fixed combined total. Citizen application and ASHA/AWW-assisted online application are supported. WCD PMMVY page (date unstated; accessed 8 Oct 2026).

Eligibility caveats. Accepted categories include SC/ST, disability of at least 40%, BPL/NFSA ration card, PM-JAY, e-Shram, MGNREGA job card, PM-Kisan beneficiary, family income below ₹8 lakh per year, and pregnant/lactating AWW/AWH/ASHA, with prescribed proof. This is not universal unconditional payment to every pregnancy. The 18 July 2023 order adds NFSA ration-card eligibility and clarifies income-proof acceptance. The 17 April 2023 order specifies age **18 years seven months-55**; the FAQ describes eligibility at childbirth as under 55. The 20 February 2023 order sets an application window of **270 days after childbirth**. Check portal application of the endpoint and any subsequent state instruction. Eligibility order, age order, time-limit order.

Mission Shakti guidelines exclude regular Central/State Government or PSU employees and women receiving similar maternity benefits under another law. For second-girl-child benefits, registration **during pregnancy is mandatory**; do not interpret the later application window as permission to omit pregnancy registration. The current FAQ allows fresh-beneficiary treatment after miscarriage/stillbirth in a future pregnancy and removes mandatory husband's Aadhaar. Mission Shakti implementation guidelines (2022 framework; accessed 8 Oct 2026).

Where and current geography. Use the nearest AWC/ASHA, state-approved health facility and PMMVY citizen portal. A **12 August 2026** parliamentary reply says implementation is in **35 States/UTs including Odisha**, which began FY 2025-26; **Telangana was still being onboarded**. This newer source supersedes the FAQ's older statement excluding both Odisha and Telangana. Confirm current Telangana onboarding before routing applications. PIB/WCD update.

ASHA/NGO workflow. With consent, check one accepted eligibility proof, beneficiary Aadhaar, Aadhaar-mapped bank/post-office account, mobile number, MCP/RCH records, ANC, birth registration and vaccine records. ASHA/AWW can submit and track within authorized jurisdiction. An NGO can assist the woman to obtain corrections from the actual bank/civil-registration/health authority, preserve acknowledgment and refer unresolved cases to the local administering officer. PMMVY 2.0 FAQ (undated document; accessed 8 Oct 2026).

Bottleneck and funding. The official FAQ evidences documentary/payment prerequisites; whether Aadhaar-bank mismatch, missing birth records or connectivity causes delays locally is a **hypothesis to audit**, not a verified nationwide rate. The ministry announced **1515** for POSHAN/PMMVY from 1 Nov 2025; confirm local connection. Benefit money is not NGO income; ASHA/AWW honorarium rules do not create NGO entitlement. Helpline announcement, 24 Oct 2025.

4. Pradhan Mantri Surakshit Matritva Abhiyan - PMSMA

Purpose and eligibility. PMSMA offers free quality ANC for pregnant women in the **second or third trimester** at designated government facilities on the **ninth of each month**. The official portal describes diagnostics/counselling, detection and line-listing of high-risk pregnancy, management of conditions and birth/complication planning, with specialist/physician review. Confirm the locally scheduled session; an emergency must use the immediate clinical referral route. Official PMSMA portal (program introduced 2016; portal carries September 2026 activity; accessed 8 Oct 2026).

Geography and pathway. National program under MoHFW; states and districts identify participating rural/urban government sites and referral facilities. An NGO should obtain the local facility roster, session timetable and named high-risk referral contact from the district maternal-health team. Routine ANC remains necessary between program visits.

ASHA and NGO complement. Official guidelines assign ASHA awareness, beneficiary line-listing and mobilization; ANM/facility staff provide ANC and organize clinical referral. NGOs can support appointment reminders, interpretation, transport/navigation and tracking whether the referred woman reached the treating facility. Qualified obstetricians/radiologists/physicians may explore the official volunteer registration channel; NGO affiliation alone does not authorize clinical practice. PMSMA operational guidelines (2016 document; sections on ASHA/ANM roles).

Bottleneck evidence. Missed ANC and high-risk pregnancy identification are explicit program objectives. A specialist being absent, diagnostics being unavailable or a high-risk referral failing to close are **site-specific risks** to verify from attendance, test and referral records; the reviewed sources do not prove their current district prevalence.

Funding. The volunteering channel is a service opportunity, not an NGO grant or salary commitment. Any funded outreach, transport or clinical arrangement requires its own state/district authorization and terms.

5. Surakshit Matritva Aashwasan - SUMAN

Purpose and entitlement boundary. SUMAN aims for dignified, respectful and quality maternal/newborn care at no cost, with zero tolerance for denial in public facilities. Its service charter includes at least four ANC checks, home-based newborn-care visits, trained delivery, complication management, breastfeeding support, transport/referral, newborn vaccination and grievance redressal. It integrates existing initiatives including JSSK and PMSMA. It is **not a separately additive cash benefit**; the charter refers beneficiaries to conditional transfers under the applicable schemes. Official SUMAN portal and charter (page last updated 10 July 2021; accessed through official indexed content 8 Oct 2026).

Geography and pathway. National assurance initiative implemented through states' public maternity/newborn networks. Ask the district for its current designated SUMAN facilities and referral map, and confirm which level handles normal delivery, obstetric complications and sick newborn care. The older portal's reference to referral within an hour is a service-guarantee aspiration; do not promise an actual travel time for every village.

ASHA and NGO complement. ASHA can explain entitlements, help birth planning and link facility care to authorized home follow-up. NGOs can provide language/accessibility support, birth-companion navigation where facility policy permits, beneficiary feedback and grievance assistance. Record the woman's account of a service denial with consent, retain referral/receipt details, and help her approach the facility help desk and district maternal-health officer. The portal lists a grievance number **1800-180-1104** and web registration; because its page is dated 2021, validate the working channel before printing it locally.

Bottleneck evidence. A zero-denial charter and complaint mechanism establish standards; they do not establish how frequently denial occurs in 2026. Poor respectful-care experiences, missed home visits and inaccessible complaints are **implementation hypotheses** until supported by a current facility audit or beneficiary feedback sample.

Funding. No automatic NGO grant was verified. Community engagement can be proposed to state/district NHM through a written partnership, approved outreach plan or procurement; any NGO clinical service requires distinct permissions.

6. Mission Indradhanush - MI, including intensified rounds

Purpose and beneficiaries. MI targets missed and partially vaccinated children and pregnant women, particularly underserved/high-risk settlements, through intensified routine-immunization outreach. **IMI 5.0 was a dated 2023 campaign**, conducted in all districts, including children up to five years and pregnant women; previous rounds commonly used younger child age bands. It delivered vaccines already in UIP according to the National Immunization Schedule. Do not describe IMI 5.0's August-October 2023 sessions as an open October 2026 campaign. MoHFW/PIB IMI 5.0 release, 12 Oct 2023.

Geography and access. National strategy implemented through state/district microplans and notified rounds; rural hamlets, remote habitations, urban slums and migratory/temporary settlements are practical outreach targets. Ask the District Immunization Officer/ANM whether a current catch-up round is notified and where sessions will operate. If no campaign session is current, route due vaccines to the continuing UIP service.

ASHA and NGO complement. Under ANM/district microplanning, ASHA/AWW/community mobilizers identify due beneficiaries and bring families to sessions. NGO staff can help find missed mobile families, translate approved information, address travel/time barriers and support reminders, while trained authorized vaccinators administer and document vaccines. Vaccine hesitancy or access barriers should be discussed respectfully; do not claim an NGO can create independent government immunization records or access U-WIN through its own system without approval.

Bottleneck evidence. The official 2023 release identifies missed/dropped-out beneficiaries as the campaign's target. Local reasons-migration, session timing, misinformation, transport or stock availability-must be measured rather than assumed.

Overlap and accounting. MI is a campaign mechanism **within the UIP immunization ecosystem**, not a second vaccine entitlement. Use one child/pregnancy record and one dose event. Tag the event's delivery setting as MI/catch-up if relevant; do not sum MI participants with UIP participants to estimate unique reach, and do not count each return dose as a new beneficiary.

Funding. No automatic NGO grant was verified. Support requires a district-approved campaign role, a formal implementation arrangement where available, or the NGO's own funding.

7. Universal Immunization Programme - UIP

Purpose and benefit. UIP is the continuing national routine program providing free scheduled vaccination for children/adolescents and pregnant women. MoHFW's 2024-25 annual report supplies an updated national schedule including newborn and infant doses, later boosters, Td at ages ten/sixteen and pregnancy Td, with Japanese encephalitis vaccination in endemic districts. Clinical staff must determine due/catch-up doses from the current schedule and past records. Do not use an old web PDF showing phased vaccine introduction as proof of October 2026 availability. MoHFW Annual Report 2024-25, immunization chapter and schedule, p. 72.

Geography and service point. National program, delivered by state/district immunization teams through fixed public-facility and outreach sessions in both rural and urban areas. Local access includes village/AWC/subcentre sessions, PHCs/UPHCs and hospitals. Maharashtra's state NHM page explicitly describes free institution/outreach vaccination and illustrates state-specific endemic-district arrangements; its details must not be copied as another state's service map. Maharashtra NHM routine immunisation page, 17 Apr 2025.

ASHA and NGO complement. With ANM/AWW, maintain a due list, explain the next visit, remind caregivers and record referral completion against official vaccination documentation. NGO assistance is useful for temporary settlements, migrants, accessible information and continuity across relocation. Vaccination and adverse-event assessment belong to trained authorized health personnel; NGOs can assist navigation back to the health team.

Bottleneck evidence. The official health-worker handbook explicitly requires microplanning that includes hamlets, urban subwards, migratory populations and temporary settlements, and tracking dropouts. It demonstrates recognized operational needs, not the local share of missed children. Validate local due lists, session cancellations and coverage before selecting an intervention. NHM Immunization Handbook for Health Workers, 2018 (use here only for planning roles; clinical schedules require newer guidance).

Overlap/funding. A dose delivered during MI remains a UIP dose and is counted once. No automatic NGO grant, vaccine-administration authorization or government-data integration entitlement was verified.

8. Ayushman Bharat Pradhan Mantri Jan Arogya Yojana - AB PM-JAY

Purpose and core benefit. PM-JAY protects eligible families against large covered hospital expenses: **up to ₹5 lakh per family per year** for secondary/tertiary hospitalization at empanelled public/private hospitals, cashless at the point of service, including specified pre/post-hospitalization expenses. Pre-existing conditions are covered from day one; family size/age/gender limits do not restrict members of an eligible family. This is not ₹5 lakh cash paid to each patient, and routine outpatient care or every private-hospital service is not automatically covered. MoHFW RML Hospital official key-features page (last updated 5 Feb 2025).

Eligibility and 70+ expansion. Verify the household in the official beneficiary system and applicable state-added categories rather than treating poverty, ASHA referral or an ABHA number as sufficient eligibility proof. The 11 September 2024 Cabinet approval expanded eligibility to Indian senior citizens **70+ irrespective of income**. Already-covered families receive an additional up-to-₹5-lakh senior-only annual top-up, not a general ₹10-lakh entitlement for younger members. For other senior families, ₹5 lakh is shared on a family basis. Seniors with CGHS/ECHS/CAPF benefits must choose the applicable public scheme; the official clarification allows seniors with private insurance/ESI to use this expansion. Cabinet approval, 11 Sep 2024, NHA senior-citizen FAQ (FAQ date unstated; accessed 8 Oct 2026).

Geography and referral. NHA sets the national framework, State Health Agencies operate state arrangements, and care is delivered by currently empanelled hospitals with applicable specialties/packages and authorization. **Delhi became the 35th implementing State/UT on 5 April 2025**; older sources excluding Delhi and Odisha are stale. This dated count is not a guarantee of an accessible hospital in every district. Verify participation, hospital empanelment, relevant package

and portability before travel. PIB Delhi implementation update, 5 Apr 2025.

ASHA and NGO complement. ASHA can help households learn about coverage and obtain a verified referral; NGOs can assist official card/eligibility checks, language access, transport planning and handoff to the hospital's Ayushman help desk. Clinical admission/authorization is decided through the hospital/SHA workflow. The official April 2025 release also reports inclusion of frontline ASHA/Anganwadi workers in health coverage; verify the worker/family record rather than assuming an ID alone activates it.

Bottleneck/funding. Incomplete identity verification, a distant suitable empanelled hospital and authorization/document gaps are **operational hypotheses** here, not claimed October 2026 national failure rates. Count completed covered admissions separately from card creation. No NGO grant was verified. A charitable hospital can explore provider empanelment only if it meets current requirements; reimbursement for authorized care is provider payment, not general NGO fundraising.

Evidence notes and district implementation checklist

1. **Current state exception verified:** PMMVY's August 2026 PIB reply takes precedence over the older FAQ's Odisha exclusion. Telangana onboarding should be rechecked at use. The February 2023 time-limit order describes one ANC check; the currently linked WCD FAQ describes at least two. This report uses the current FAQ for milestone preparation and flags the difference for the administering officer/portal rather than promising approval under the older wording. Scheme benefits and application rules still require local validation.

2. **Do not turn old totals into market size:** No historical launch estimate, cumulative campaign total, visits, card count or hospital admissions is used here as the number of unique October 2026 eligible people. Portal activity totals were deliberately omitted.

3. **Program overlap:** One pregnancy may use JSY, PMMVY, JSSK, PMSMA and SUMAN. Store each scheme's status separately but deduplicate the woman/pregnancy. UIP and MI must share one child/dose history; campaign tags describe how a dose was delivered.

4. **Evidence labels:** Historical CAG findings establish a documented past/local bottleneck; charter standards and documentary requirements establish expected process. Unmeasured local obstacles are labeled hypotheses, not national findings.

5. **Practical first contact:** District Health Society/CMHO or equivalent for NHM maternal services; District Immunization Officer for UIP/MI; state-selected WCD/social-welfare/health PMMVY officer and AWC/ASHA for PMMVY; State Health Agency and empanelled-hospital help desk for PM-JAY. Titles vary by state.

6. **Minimum field directory:** Facility name and geography, service capability, designated session, clinical/administrative contact, referral destination, transport route, applicable eligibility/document checklist, escalation channel, last verified date, and a responsible staff member. A national page cannot replace this district directory.

7. **NGO proposal route:** Agree a narrow problem and outcome-completed high-risk referral, due-dose completion, acknowledged maternity application or resolved denial-with the competent authority. Obtain a written role, scope, permissions, escalation contacts and data-sharing terms. Funding must come from a documented grant/contract/CSR/own-resource arrangement; none of these eight program names itself creates NGO funding eligibility.

8. **Data and beneficiary rights:** Use informed consent, minimal access and authorized records. NGOs should not ask for staff credentials or promise direct integration with government beneficiary systems. Provide assistance without representing the NGO as the sanctioning or clinical authority.

All URLs above are primary Indian government sources. Source dates are given where published; otherwise the date is explicitly recorded as unstated and access was 8 October 2026. Local service performance and approved NGO agreements remain to be verified district by district.

Part 3. 24 programmes - welfare, disability, adolescent and TB support

Welfare and care access: eight reserved programmes

Research cut-off: **8 October 2026, India**. This note counts exactly eight catalogue entries. NSAP is counted once, NMHP includes Tele-MANAS, and NTEP includes its nutrition-payment component. PM TB Mukt Bharat Abhiyaan/Ni-kshay Mitra is a linked community-support initiative rather than an independent TB treatment system. Software portals named below are access/infrastructure channels, not additional schemes.

All sources below are official government or government-implementing-agency sources. Publication/effective dates are distinguished from website-update dates. National programme design does not establish that every facility, camp or NGO partner is available in a particular village.

1. National Social Assistance Programme (NSAP), umbrella social assistance

Eligibility / benefit: Central pension assistance for BPL older people, eligible widows and persons with severe disability; eligibility varies by component. Central amounts are only part of the pension where a State adds support. MoRD, 1 Jan 2026

Geography and access channel: Rural and urban; Gram Panchayat/municipality and State social-welfare/pension machinery. State caps apply to central assistance. MoRD, 17 Mar 2026

Practical ASHA role and NGO complement: Guidelines explicitly involve ASHAs, voluntary organizations and SHGs in awareness. NGOs can help prepare applications and follow sanction/payment status; local authorities determine eligibility. NSAP guidelines, Oct 2014

2. ADIP, assessed assistive-device subsidy

Eligibility / benefit: General rule: Indian citizen of any age, at least 40% disability, UDID/card enrolment with certificate, Aadhaar/enrolment documentation, individual/guardian income at most ₹30,000/month. Subsidy tier and device caps apply. Revised guidelines, effective 26 Sep 2024

Geography and access channel: National; ALIMCO, National Institutes, CRCs, DDRCs and approved agencies; assessment/distribution camps or agency centres; individual requests via ARJUN.

Practical ASHA role and NGO complement: Proposed ASHA contribution: identify access needs and connect households to assessment/documentation. Qualified implementing NGOs may receive grants; NGO transport, fitment follow-up and additional funding can complement the subsidy. Eligibility/prescription stays with the authorized agency.

3. Rashtriya Vayoshri Yojana (RVY), in-kind assistive living devices

Eligibility / benefit: Indian senior citizens aged 60+, BPL or income at most ₹15,000/month, with relevant age-related infirmity. Devices are free after assessment; this is neither a pension nor general health insurance. ALIMCO official FAQ, undated

Geography and access channel: National scheme, ALIMCO implementation coordinated with State/UT and district committee; local assessment and distribution availability matters. RVY is an AVYAY component. DSJE annual report 2025-26, pp.90-92

Practical ASHA role and NGO complement: Proposed ASHA contribution: flag hearing, vision or mobility access needs and facilitate a camp appointment. Local NGOs can arrange accessible transport, accompaniment and device-use/repair follow-up in coordination with ALIMCO.

4. Rashtriya Bal Swasthya Karyakram (RBSK 2.0), child screening and referral

Eligibility / benefit: Birth to 18 years; the 2026 framework covers **38** common conditions across the established 4Ds, with added NCD, mental-health and behavioural priorities. MoHFW, 24 Jul 2026

Geography and access channel: Community, Anganwadi and school screening with Mobile Health Teams; onward facility diagnosis/treatment and referral tracking. MoHFW, 3 May 2026

Practical ASHA role and NGO complement: Established ASHA role includes newborn home screening and referral facilitation. NGOs can help families attend referrals, obtain rehabilitation support and follow through; these NGO workflow suggestions are hypotheses for local validation. 2013 operational framework

5. Rashtriya Kishor Swasthya Karyakram (RKSK), adolescent services

Eligibility / benefit: Adolescents 10-19, girls/boys, married/unmarried, rural/urban, in/out of school, with attention to underserved groups. Services address SRH, nutrition, mental health, NCDs, substance misuse, injuries/violence. NHM adolescent-health page, updated 3 Aug 2026

Geography and access channel: Peer education, Adolescent Health Days and AFHC clinical/counselling services in designated PHCs, CHCs, district hospitals and medical colleges. NHM AFHC page

Practical ASHA role and NGO complement: ASHA is village Saathiya coordinator; helps select peer educators and engages families. NGOs can assist outreach and confidential navigation with local teams; do not assume each village has all channels. NHM peer programme, updated 14 Sep 2026

6. National Mental Health Programme (NMHP), including Tele-MANAS access

Eligibility / benefit: Broad mental-health service programme, not income-tested cash assistance. DMHP includes assessment, counselling, continuing care, drugs and outreach. It was sanctioned in 767 districts as of the cited reply. Government reply, 1 Apr 2026

Geography and access channel: District/PHC/CHC service network. Tele-MANAS offers a 24×7 national remote channel: **14416 / 1800-89-14416**, with links to in-person services. DGHS NMHP page, undated

Practical ASHA role and NGO complement: Proposed ASHA contribution: consent-based awareness, supported contact and appointment follow-up according to training. NGOs may complement with qualified counselling, accompaniment and social support. The optimizer should route to professionals, without generating diagnoses.

7. National TB Elimination Programme (NTEP), care programme with nutrition DBT

Eligibility / benefit: Free screening, diagnosis and treatment in public and identified private facilities. Notified TB patients receive nutrition support of **₹1,000/month for treatment duration**, effective 1 Nov 2024. MoHFW, 19 Aug 2025, MoHFW, 9 Dec 2025

Geography and access channel: Nationwide under NHM; decentralized to Ayushman Arogya Mandirs and district TB services; Ni-kshay records notified patients and related programme information.

Practical ASHA role and NGO complement: ASHAs and treatment supporters are recognized within patient-support arrangements. NGOs/CSOs can assist outreach, service navigation and support continuity with the TB team. DGHS NTEP page

8. PM TB Mukt Bharat Abhiyaan / Ni-kshay Mitra, voluntary community support

Eligibility / benefit: Additional donor/community support for TB-affected people and households: nutrition and psychosocial support, alongside other support options. This complements NTEP; donor linkage and fulfillment must be confirmed rather than promised as automatic cash. CTD guidance, 2026

Geography and access channel: National initiative, implemented locally with District TB Officers; donors register via community-support portal.

Practical ASHA role and NGO complement: 2026 guidance assigns ASHA/community teams food-basket distribution, patient feedback and linkage roles. NGOs/CSOs and trained volunteers can provide non-clinical household follow-up, accompaniment and support under district supervision and consent/confidentiality rules.

NSAP amounts and eligibility nuance. The 1 January 2026 MoRD table gives central old-age pension assistance of ₹200/month at 60-79 and ₹500 at 80+; widow pension ₹300 at 40-79 and ₹500 at 80+; disability pension ₹300 at 18-79 for BPL people with 80% disability and ₹500 at 80+. These are **central contributions**, not uniform total pensions. State pension programmes/top-ups can have different criteria. The umbrella also contains non-pension support, which is not separately counted here. MoRD year-end review, 1 Jan 2026

The 2014 guidelines say an otherwise eligible person absent from the BPL list should have eligibility established rather than simply be excluded. They require application and local verification, with proactive household identification. This is not evidence that all low-income residents automatically qualify. The 17 March 2026 reply documents State/UT beneficiary ceilings; this is a structural limit on central funding, not proof that every locality has a waiting list. NSAP guidelines, 2026 funding reply

ADIP verification warning. The authoritative September 2024 PDF provides full/50% assistance tiers at income up to ₹22,500/₹22,501-30,000 monthly, subject to device-cost ceilings. General same-purpose repeat support is after three years; for under-18s, customized items have a one-year interval, with named five-year exceptions. ADIP-SSA has special certification pathways, including some children below 40% disability, so the general threshold must not be applied mechanically to every child. Assessment, prescription, fitment and user training require qualified rehabilitation professionals. 26 Sep 2024 guidelines, sections 6-8

The DEPwD scheme page still states continuation approval only through **31 March 2026**. The live e-Anudaan portal, checked 8 October 2026, invites ADIP proposals for **FY2026-27**. This establishes ongoing operational activity, but a fresh continuation order was not located. Confirm applicable funding/current limits with the implementing agency before promising an award. The ARJUN-hosted ADIP PDF returned an April 2024 document despite a newer search label; use the department-linked September PDF for repeat-assistance rules.

RVY documentation and logistics. ALIMCO's undated FAQ gives Aadhaar plus BPL/pension or accepted income evidence and lists mobility aids, hearing aids, spectacles and other assessed devices. RVY does not require treating an age-related infirmity as an ADIP-certified 40% disability. The 2025-26 annual report confirms RVY remains an ALIMCO-delivered AVYAY component with camp delivery. For routing, verify assessment date, device suitability, delivery and repair contact rather than assuming the next camp can supply the requested item. ALIMCO FAQ, DSJE annual report

RVY continuation evidence gap: this pass did not verify a fresh RVY/AVYAY continuation order for the period after 31 March 2026. The live ARJUN registration portal still includes RVY, but portal availability alone does not establish formal renewal or a funded local camp. Confirm the current authorization and local availability with ALIMCO/district authorities.

RBSK and the 2026 child-service transition. The 3 May 2026 release describes strengthened community-to-facility pathways, digital health cards and referral follow-up. The 24 July 2026 parliamentary reply establishes 38 conditions; the older NHM programme-page list of 32 is superseded for catalogue descriptions. The established 2013 framework includes free treatment and tertiary surgical follow-up for programme conditions and used Anganwadi/government/government-aided school touchpoints. Exact local 2.0 implementation, eligible-school reach and service availability still require district verification; the full 2026 PDF could not be retrieved in this research pass. Established framework

Separately, **SSBSK launched 29 June 2026**, combining HBNC and HBYC into a birth-to-36-month home/community framework. Its release envisages integration with the RBSK 2.0 portal. It does **not** say RBSK was abolished or its age range replaced. Use district-adopted SSBSK schedules when building infant/young-child visit rules; treat promised platform integration as a rollout claim, not a verified public API. MoHFW launch, 29 Jun 2026

RKSK access design. NHM describes four Saathiyas per village/1,000 population/ASHA habitation; ASHA facilitates selection, coordinates peer activity and helps engage families. Quarterly Adolescent Health Days and monthly clubs provide service-linkage touchpoints. AFHCs require trained clinicians/counsellors and privacy/confidentiality. Store verified clinic location, adolescent service hours and supported languages; do not require a sensitive reason for referral to be visible to an NGO dispatcher. These product choices are proposed local-validation hypotheses. Peer programme, AFHCs

NMHP scope and availability. Tele-MANAS is described by DGHS as a digital NMHP component providing universal remote access and assured service linkages; it should not inflate scheme counts. "767 districts sanctioned" does not prove staffing or appointment availability in all 767. Use district confirmation to distinguish sanctioned, operational and accepting-referrals states. A local NGO directory should record professional qualifications, cost and capacity rather than

classifying every volunteer as a mental-health provider. DGHS NMHP, 1 Apr 2026 reply

NTEP, nutrition DBT and digital access. ₹1,000 replaces the older ₹500 nutrition figure. The DGHS NTEP overview still displays ₹500 and historic 2023 statistics, so it is suitable for programme structure/ASHA recognition, not the current transfer amount. The 9 Dec 2025 reply and live MyScheme entry, checked 8 October 2026, support ₹1,000 for notified TB patients. The MyScheme entry describes bank-account support; the optimizer can track documentation/referral status with authorized staff. An NGO must not independently determine TB diagnosis, medication or preventive-treatment eligibility.

Ni-kshay Mitra evidence and limits. The 2026 guidance explicitly documents food-basket coordination/logistics delays, inconsistent support quality and digital onboarding/matching gaps. It broadens household nutrition and psychosocial support and specifies district-supervised, non-clinical volunteer activity. These are documented programme implementation challenges; the document does not establish their prevalence in every district. Its 2026 proposals for digital matching, delivery tracking and monitoring are not proof of an open third-party integration interface. CTD guidance, 2026, programme-challenges and volunteer sections

Research implications to test locally. These are hypotheses, not established failure claims: (a) document preparation plus appointment/camp routing may improve assistive-device uptake; (b) explicit referral acknowledgement may reduce lost child/adolescent follow-up; (c) matching NGOs by coverage, skills and capacity may improve transport and support completion; (d) separating NPY payment state from Ni-kshay Mitra delivery state may prevent false "benefit delivered" flags. Measure completed referrals, elapsed time, failed hand-offs and reasons, confirmed support delivery, ASHA time saved and household consent withdrawals. Preserve clinician/district authority for clinical and entitlement decisions; use authorized, consent-based data access and locally verified service directories.

Part 4. 24 programmes - public services and livelihoods

Eight community, livelihood and public-service programmes

Research cutoff: **8 October 2026, India**. Exactly eight reserved programmes are counted below. These eight, the eight maternal/child programmes and the eight welfare programmes form the requested **24 additional programmes**, not 24 mutually exclusive beneficiary populations. A household may use several of them. Primary government sources establish programme design; they do not establish service availability at a particular village, facility or NGO.

Evidence labels: E = a role or mechanism expressly supported by the cited government material. **H** = a proposed local NGO complement or ASHA referral workflow to validate with the responsible department; it is not a nationally mandated task, proven local gap, or automatic funding entitlement.

1. NHM Free Drugs Service Initiative

Eligibility and service scope: Essential medicines free at public health facilities; the cited national description does not impose a BPL-only criterion. The facility/state essential-medicines list and clinical prescription determine the actual medicines available. This is not reimbursement for every medicine bought privately.

Rural / urban and access channel: Rural and urban public facilities under state implementation. Start with the patient's public facility/pharmacy and referral facility.

Responsible authority and ASHA boundary: MoHFW/NHM supports states; state health departments, procurement/supply systems and public facilities deliver medicines. ASHA can support care navigation and adherence within her training; she does not control procurement or independently prescribe.

NGO complement: evidence versus local hypothesis: E: official programme supports IEC, grievance redressal and supply-management systems. **H:** NGO-assisted medicine-access information, consented transport assistance and documented stock-availability escalation through the facility. No general NGO grant follows from the free-medicine entitlement.

Dated primary evidence: NHM programme page, undated, records guidelines shared **2 July 2015**; NHSRC programme description, undated.

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2. NHM Free Diagnostics Service Initiative

Eligibility and service scope: Essential diagnostics free at public facilities, appropriate to the facility's level and clinical need. Current cited policy includes **14 tests at sub-centres and 63 at PHCs**; this does not promise every investigation at every site.

Rural / urban and access channel: Rural and urban public facilities; in-house tests, referral/sample transport arrangements and approved contracted providers depend on state design.

Responsible authority and ASHA boundary: MoHFW/NHM and state health departments; facility clinicians/laboratories and authorised contracted providers conduct/interpret tests. ASHA's navigation and follow-up role does not make her a laboratory service provider.

NGO complement: evidence versus local hypothesis: E: NHM describes hub-and-spoke sample transport, outsourcing and contracting-in, with service-provider selection through open tenders. **H:** appointment information, travel support, follow-up and grievance assistance. An NGO needs separate authorisation/qualified staff and a contract to perform contracted diagnostic services.

Dated primary evidence: MoHFW parliamentary reply, PIB, **1 August 2025**; NHM programme page, baseline guidelines **2 July 2015**.

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3. Ayushman Arogya Mandir (AAM)

Eligibility and service scope: Public comprehensive primary care for the catchment population: reproductive/child and communicable-disease care, common NCD services, additional primary-care areas, medicines/diagnostics, wellness and teleconsultation. AAM is a facility initiative; do not attach PM-JAY insurance eligibility or its hospital-cover amount to this row.

Rural / urban and access channel: Upgraded sub-health centres and rural/urban PHCs, with community outreach and referrals. Actual service readiness must be checked locally.

Responsible authority and ASHA boundary: State health department/NHM; the primary-care team delivers care. **E:** ASHA is a primary-care team member supporting community access and health promotion; the team undertakes enumeration, screening mobilisation, timely referral, treatment adherence and follow-up.

NGO complement: evidence versus local hypothesis: E: community outreach is part of the model. **H:** coordinate NGO volunteers for accessible information, accompaniment or transport around the public team's care plan. Clinical screening/treatment and official reporting remain with authorised personnel.

Dated primary evidence: MoHFW year-end report, PIB, **1 January 2026**, service data to **30 November 2025**; NHSRC Community Process-ASHA, undated.

4. Deendayal Antyodaya Yojana-National Rural Livelihoods Mission (DAY-NRLM)

Eligibility and service scope: Seeks at least one woman from each rural poor household, identified through SECC 2011 and Participatory Identification of Poor, in SHGs. Supports community institutions, financial inclusion, livelihoods and access to entitlements; membership does not automatically entitle every individual to a cash grant or bank loan.

Rural / urban and access channel: Rural; village SHG/federation, block mission unit, State Rural Livelihoods Mission (SRLM), and bank linkage. Do not apply this rural eligibility to an urban household.

Responsible authority and ASHA boundary: Ministry of Rural Development, state/UT missions, community institutions and banks. Bank Sakhis/community resource persons have defined mission roles. ASHA may make a locally agreed referral; she is not automatically a Bank Sakhi, credit approver or NRLM caseworker.

NGO complement: evidence versus local hypothesis: E: official explainer recognises SRLMs, partner institutions and NGOs in intensive implementation. **H:** locally approved financial-literacy/entitlement assistance or livelihood mentoring through the SRLM. A partnership/tender is separate from SHG support; beneficiary credit is not an NGO grant.

Dated primary evidence: MoRD parliamentary reply, PIB, **11 February 2025**; PIB programme backgrounder, **23 October 2025**; official explainer, **6 December 2023**.

5. National Food Security Act / Targeted Public Distribution System (NFSA/TPDS)

Eligibility and service scope: State-identified Antyodaya Anna Yojana households: **35 kg/household/month**; Priority Households: **5 kg/person/month**. National coverage limits are up to 75% rural and 50% urban population; household identification criteria are state-specific. Covered foodgrains are free under the five-year decision beginning **1 January 2024**, through **31 December 2028**.

Rural / urban and access channel: Rural and urban; state ration-card authority and Fair Price Shop (FPS). ONORC is portability within this umbrella, not an additional programme counted here.

Responsible authority and ASHA boundary: Department of Food and Public Distribution allocates centrally; state Food/Civil Supplies departments identify households, issue cards and deliver through FPSs. ASHA can identify food insecurity and refer through an agreed workflow; she cannot determine NFSA eligibility or issue ration cards.

NGO complement: evidence versus local hypothesis: E: the Act provides district/state grievances, disclosure, social audits and vigilance committees. **H:** NGO assistance with applications, correct grievance channels and accompaniment. Distribution/licensing requires the state's approval; ration entitlement is not NGO funding.

Dated primary evidence: DFPD/NFSA responsibilities, Act **2013**, current undated explainer; NFSA salient features, page updated **6 October 2026**; Cabinet decision, PIB, **29 November 2023**.

6. Mission Saksham Anganwadi and POSHAN 2.0

Eligibility and service scope: Umbrella for Anganwadi Services, POSHAN Abhiyaan and Scheme for Adolescent Girls. Supplementary nutrition covers children **6 months-6 years**, pregnant/lactating women, and girls **14-18 years in Aspirational Districts and all North-Eastern districts**. Early stimulation is for 0-3 and preschool education for 3-6. Baseline guidelines allow eligible beneficiaries irrespective of caste, religion/income; register through the nearest AWC.

Rural / urban and access channel: Rural and urban Anganwadi Centres (AWCs). Nutrition, education and community mobilisation combine with health services through NHM/public health infrastructure.

Responsible authority and ASHA boundary: Ministry/state Women and Child Development departments, CDPO/DPO and AWW/AWH deliver the Anganwadi programme. **E:** ASHA/ANM support health checks/referrals and convergence; AWW's responsibilities and digital records are distinct.

NGO complement: evidence versus local hypothesis: E: Jan Andolan/community events, mothers' groups and Poshan Panchayats provide participation platforms. **H:** NGO-supported nutrition education, accessibility, gardens or referral accompaniment with WCD approval. Food procurement, beneficiary verification or medical treatment requires the applicable authorised arrangement; no blanket NGO grant is established here.

Dated primary evidence: WCD scheme guidelines, baseline approval **2021-22 to 2025-26**, sections 2.4, 3.1 and 3.11; current PIB backgrounder, **14 April 2026**; Union Budget expenditure profile, **2026-27**, Statement 4A.

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7. Jal Jeevan Mission (JJM), including JJM 2.0

Eligibility and service scope: Rural household potable tap-water service; the earlier national service standard is **55 litres/person/day**, prescribed quality, regular long-term supply. **JJM 2.0 was approved 10 March 2026 and extends the mission to December 2028**, emphasising sustained service, governance and accountability. Household eligibility is not an income-tested NGO benefit.

Rural / urban and access channel: Rural; Gram Panchayat/Village Water and Sanitation Committee (VWSC/Paani Samiti), district/state water-supply agency. Do not present it as the urban household water scheme.

Responsible authority and ASHA boundary: Ministry of Jal Shakti/DDWS and state PHED/Rural Water Supply institutions, with local-government/community management. ASHA is not the implementing water engineer or department. Historical guidance includes trained grassroots workers, including ASHAs, in water-quality surveillance; a universal new ASHA mandate was not verified.

NGO complement: evidence versus local hypothesis: E, historical: 2019 guidelines recognise NGO/VO/CBO Implementation Support Agencies and sector partners; 2021 official publication illustrates ASHA/NGO water-testing training. **H/current check required:** confirm roles/empanelment under the new 2026 guidelines before proposing paid implementation, mobilisation or testing.

Dated primary evidence: Cabinet approval, PIB, **10 March 2026**; 2025 service description, **21 August 2025**; government guidelines index, new English guidelines **22 May 2026**.

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8. Pradhan Mantri Bhartiya Janaushadhi Pariyojana (PMBJP)

Eligibility and service scope: Affordable quality generic medicines, consumables and surgical items for the public through Janaushadhi Kendras. Purchases are **not generally free** and are not restricted to BPL households. Clinical appropriateness/prescriptions and availability still matter.

Rural / urban and access channel: Rural and urban; authorised PMBJP Kendra and PMBI's application channel for proposed operators.

Responsible authority and ASHA boundary: Department of Pharmaceuticals; Pharmaceuticals & Medical Devices Bureau of India (PMBI) and licensed Kendra operators. ASHA may provide access information within her health role; she is not automatically a pharmacist or Kendra licensee.

NGO complement: evidence versus local hypothesis: E: trusts/societies/charitable organisations may apply; reputed NGOs/charities are preferred agencies in government hospital premises. Applicants need a qualified pharmacist, drug-sale licence, premises and agreement; posted guidelines also request NGO Darpan ID. Operator margins/incentives

are conditional support for authorised Kendras, not a general NGO health grant. **H:** map an existing outlet before proposing a new one.

Dated primary evidence: PMBI Kendra opening guidelines, currently posted **undated**, includes distance-policy amendment **16 November 2023**, sections III, V-IX and document checklist; operator agreement, undated.

Caveats that affect the ASHA visit optimiser and local NGO pilot

• **Separate service ownership from the visit route.** Record the responsible facility/department, referral destination, verified opening hours, service availability, required documents and last verification date. Route optimisation cannot itself guarantee a medicine, diagnostic slot, water connection, ration-card approval or loan. Proposed NGO referral assistance in this document is a local design hypothesis, not evidence of a nationwide programme failure.

• **Do not add overlapping benefits/populations.** AAM includes medicines and diagnostics supported by the two NHM initiatives; the same household can also receive NFSA foodgrains and Anganwadi services. NFSA's statutory nutrition provisions overlap with Anganwadi delivery. Count the eight programmes once each, then deduplicate people/households and service events in impact metrics. ONORC and the components subsumed in POSHAN 2.0 do not add extra rows.

• **Funding follows a different decision path.** Household service entitlement, an SHG bank loan, an NGO implementation contract, and a licensed pharmacy's operator incentive are different. The programme list does not identify an unrestricted funding source for a local NGO. Validate actual state empanelment, procurement, grant or operator rules before assigning funds or describing a funded partnership.

• **The latest POSHAN evidence reveals an existing scheduler.** The official **14 April 2026** backgrounder reports an **AWW Home Visit Scheduler integrated into Poshan Tracker in April 2026**, with **23 structured visits** for pregnant women/children up to three, automatic scheduling and counselling/video prompts. This is material prior art for a visit-optimiser proposal, while AWW and ASHA workflows remain distinct. Do not claim that frontline-worker home-visit scheduling does not exist. PIB current POSHAN backgrounder.

• **Use newer policy for current numbers.** The NHM programme page contains **31 August 2018** diagnostic rollout counts and old illustrative test lists; those are not October 2026 status. The cited **1 August 2025** reply gives 14 sub-centre/63 PHC tests. Likewise, the POSHAN baseline approval period ends in 2025-26, but the 2026-27 Budget and April 2026 government description show ongoing programme activity; do not infer that it ended. NHM baseline page, 2025 reply, 2026-27 Budget.

• **JJM evidence limitation:** the new JJM 2.0 English guidelines were identified in the official index but could not be fully extracted because the PDF was too large for the browsing tool and the alternate extractor failed. Its NGO/ASHA clauses are therefore **unverified** here. Historical NGO ISA provisions are in the December 2019 operational guidelines, section 5.7 and institutional responsibilities; a December 2021 official Samvad publication gives a state example of grassroots ASHA/NGO water-quality training. A historical state example is not a current nationwide mandate.

• **Use the correct NFSA grievance channel.** The central portal's online grievance utility says submitted input **does not constitute a complaint**. A pilot should verify the state's formal complaint route/District Grievance Redressal Officer rather than promise legal redress through that feedback form. NFSA responsibilities.

All linked primary sources were checked or identified by **8 October 2026**. Undated pages are labelled as such; page-update dates are not treated as dates on which every underlying rule changed. Tavily searches produced some rate-limit errors during the final checks, so the built-in web tool was used for the remaining government-source verification. No outreach was performed.

Part 5. 30 platforms - India-facing digital health

Eight India-facing digital health platforms: ASHA and NGO referral relevance

Research cut-off: **8 October 2026**. Exactly eight platforms are counted: ANMOL, RCH Portal, U-WIN, AMRIT, CommCare, eSanjeevani, ABDM ecosystem and Ni-kshay. AMRIT's FLW app and its API are one platform; ABDM's ABHA/HFR/HPR/HIE-CM building blocks are one ecosystem. ANMOL and RCH are distinct applications in the same government programme, with substantial integration and overlap, rather than independent competitors.

Finding: ASHA worklists, offline collection, counseling media, task prioritization and clinical referrals already exist in this landscape. A defensible opportunity is an additional, locally operated service-fulfillment layer: verify an NGO's actual service and coverage, obtain consent, identify an eligible beneficiary, secure acceptance from an available provider, complete the service, return the outcome and escalate failure. This is a hypothesis requiring district validation, not a claim that all eight platforms fail or cannot be configured to do it. Ni-kshay provides a particularly important existing counterexample: its TB community-support programme explicitly connects consented patients, district officers and donors/NGOs and includes follow-up confirmation.

Evidence labels: **Official** = government/operator technical documentation; **Provider claim** = product description, not an independently verified performance result; **Field evidence** = a primary evaluation in the named setting; **Not demonstrated** = not established by the sources reviewed, not proof of absence. Dates below are source dates or the explicit date attached to a statistic; search-engine crawl/publication estimates were not treated as document dates.

Platform matrix

1. ANMOL (ANM Online); MoHFW; Android frontline RMNCH job aid linked to RCH

Users and verified provisions: Primarily ANMs/MPWs; ASHAs benefit from ASHA-wise plans. Records eligible couples, pregnancy, delivery, PNC and child services; offline entry, due lists, VHND planning, dashboards, call function, audio/video counseling and ABHA creation/linkage. **Official:** v5.0.12 manual, 28 May 2024.

Geography and access: Government deployment: **29 States/UTs and about 1.82 lakh registered/using ANMs as of 31 Dec 2025**, according to the MoHFW 2025-26 annual report, p.50. This is a dated deployment statement, not universal use by every ANM or direct app access for all ASHAs.

Beneficiary and district workflow: RCH administrator maps an active ANM to villages and enables the district; first download requires internet and registered-phone OTP. ANM records services and syncs to RCH; workplans guide field follow-up. **Official manual** requires correct mapping and synchronization.

Verified interoperability / API and access caveat: RCH synchronization and ABHA/POSHAN functions are documented. The NHM IT initiatives e-booklet describes JSON APIs and RCH exchange; this older technical listing is not permission for an NGO to read/write production data. No general NGO developer onboarding or live NGO-capacity API was verified.

Gap assessment and boundary: Confirmed constraints in the 2024 manual: Android 7.1+, first-download connectivity, ABHA services online; direct report/data downloading removed from v5.0.9. **Not demonstrated:** an independently verified local NGO roster with service eligibility, availability, acceptance and beneficiary-confirmed completion. A district would need to validate current version behaviour before designing integration.

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2. RCH Portal; MoHFW, government reproductive/maternal/child beneficiary tracking and programme reporting

Users and verified provisions: Health providers, ANMs, data-entry staff, ASHA-linked catchments and managers. Includes beneficiary identification, due services, high-risk line lists, tracking, SMS and dashboard/reporting. NHM Maharashtra official workflow.

Geography and access: MoHFW annual report, 2025-26, p.50: 29 States/UTs running the portal, three sending data by web services, four in integration process; **27.97 crore pregnancy records and 24.63 crore child records since inception to 31 Dec 2025**. These cumulative records are not unique current beneficiaries.

Beneficiary and district workflow: Maharashtra's documented route: register in RCH register → enter at designated point → assign RCH ID → generate monthly provider/ASHA workplan → distribute printouts → deliver services → return completed plans → update portal. District-to-subcentre line lists support tracking; contact details require validation.

Verified interoperability / API and access caveat: The annual report names integration with ABDM, ANMOL, Karnataka eJanma/Kutumba, Kilkari and Mobile Academy; some state exchange uses web services. This proves selected programme integrations, not a publicly accessible production API for arbitrary NGO applications.

Gap assessment and boundary: Documented workflow dependency: service-date entry and paper-plan return determine record freshness in the Maharashtra process. **Not demonstrated:** general cross-sector NGO matching, capacity reservation or closed-loop nonclinical support. RCH 2.0 is described as an upgrade under development in the report; proposed features should not be treated as deployed everywhere.

3. U-WIN; MoHFW; Universal Immunization Programme digital vaccination platform

Users and verified provisions: Pregnant women/children, vaccinators, programme managers; ASHAs can support mobilisation. Citizen registration, anywhere vaccination, dose records, QR certificate, ABHA/Child ABHA, offline vaccinator entry and SMS three days before due doses. PIB official update, 29 Nov 2024.

Geography and access: National rollout is confirmed in Rajya Sabha answer, 10 Mar 2026, including Haryana. A PIB 2026 factsheet reports **11.87 crore children and 3.96 crore pregnant women registered as of 18 Mar 2026**. Registrations and recorded vaccination are distinct metrics.

Beneficiary and district workflow: Vaccinator registers/identifies beneficiary, records administered doses and uses due-beneficiary line lists; beneficiary receives reminders and a portable certificate. District teams can use line-listing for vaccination follow-up. Offline recording addresses poor connectivity, while synchronised records are required for cross-location visibility.

Verified interoperability / API and access caveat: The **10 Mar 2026 parliamentary answer** explicitly says U-WIN is integrated with ABDM, POSHAN Tracker and SAFEVAC. No publicly documented third-party production API/onboarding route for an NGO referral network was verified in reviewed official material.

Gap assessment and boundary: Scope boundary: immunization recording and timely delivery. **Not demonstrated:** routing a family to independently verified local transport, nutrition, protection or other NGO support, with acceptance/completion status. An NGO-assisted immunization programme could operate alongside U-WIN; data access and ownership need agreement. Offline mode is an existing provision, not a proposed innovation.

4. AMRIT, including Front Line Worker app (Sakhi / Utprerona); Piramal Swasthya Management and Research Institute; open-source primary-care platform

Users and verified provisions: ASHAs, beneficiaries and linked primary-care programmes. **Provider claims:** offline/real-time sync, patient history, ABHA, speech-to-text, multilingual input, family planning/MCH/immunization/NCD tracking, worklist/workplan, training and incentive forms. Official product page, undated.

Geography and access: Product page lists Bihar/Assam governments and partners; Hindi Bihar variant and Assamese Assam variant. Operator FLW repository identifies Sakhi/Utprerona and English/Hindi/Assamese support. These establish India relevance and named implementations; they do not establish national access, uniform uptake or all modules working in every deployment.

Beneficiary and district workflow: ASHA registers households/beneficiaries, follows pregnancy/child/NCD and immunization lists, high-risk cases, a dashboard and to-do scheduler. Repository modules substantiate a designed work-management workflow. District deployment depends on configured users, organisational mapping, backend and programme support.

Verified interoperability / API and access caveat: Public GPL-3.0 repositories include FLW app, FLW REST API microservice and AMRIT platform module map. Open source enables evaluation/self-hosting or authorized integration; it does not confer access to existing state patient databases. Production agreements and credentials still matter.

Gap assessment and boundary: Not demonstrated in these sources: nationwide, service-specific NGO verification and live intake capacity, with portable referral ownership and completion evidence across unrelated NGOs. Worklists, voice and offline sync are already advertised. Treat those as product provisions, not independent field effectiveness; no such evaluation was established here.

5. CommCare; Dimagi; configurable open-source case-management / frontline programme platform

Users and verified provisions: NGOs, governments, ASHAs/other FLWs, supervisors and administrators. Current product site documents no-code apps, longitudinal cases, offline job aids, synchronisation and reports. Precision Tasking article, Sep 2021, still publicly available describes prioritized tasks, shared patient view, CHW/supervisor reporting and SMS-generated appointment tasks.

Geography and access: Current India services official India page and historical Bihar/UP deployments establish India-facing relevance. Organisations deploy their own project applications; there is no assumption of a single national ASHA instance. Current plan, implementation cost and programme configuration must be obtained separately.

Beneficiary and district workflow: Configured programme: create beneficiary/case → schedule/prioritize tasks → complete forms/counseling/referral algorithms → sync → supervisor monitors. Case-sharing/referral workflows are explicitly described in the operator's 2025 security playbook.

Verified interoperability / API and access caveat: Operator API documentation covers cases/forms/users/integrations and says API feature requires Standard plan or above; account/project permissions apply. A 4 Mar 2026 operator announcement requires API keys for SSO-enabled project integrations from 13 Apr 2026. Verify current plan/permissions; an API is not cross-project data authorization.

Gap assessment and boundary: Field evidence: Bihar ICT-CCS RCT improved visit coordination and several behaviours, but supervision was not significantly changed. Primary study, 2019, using 2012-14 data. **Not demonstrated:** ready-made India-wide NGO verification/capacity network. Configurability means NGO referral fulfillment could be implemented; claiming CommCare lacks referrals or prioritization would be incorrect.

6. eSanjeevani; MoHFW with C-DAC Mohali; national telemedicine

Users and verified provisions: Citizens, CHOs/frontline staff, generalists and specialists. Two routes: assisted provider-to-provider AAM/HWC consultation and home patient-to-provider OPD. Official MoHFW service summary. e-prescriptions and local assistance described in 2026 parliamentary response.

Geography and access: All States/UTs with state implementation (parliamentary response). C-DAC product page reports **over 493 million consultations, >240,629 onboarded/trained providers, >141,707 spokes and >19,362 hubs as of 16 Aug 2026**. Consultations are encounters, not unique people or NGO referrals.

Beneficiary and district workflow: Patient visits local spoke; CHO facilitates specialist consultation from a district/secondary/tertiary hub, or citizen joins a home OPD. NHM Maharashtra workflow illustrates district-hospital hubs supporting AAM spokes. Clinical advice/prescription is a service outcome, while transport, medication access and social assistance can require further local action.

Verified interoperability / API and access caveat: C-DAC documents ABDM integration. The official Android listing, updated 24 Aug 2026 documents assisted consultation and ABHA workflow enhancements. No public general NGO-referral API or self-service production integration was verified.

Gap assessment and boundary: Not demonstrated: verified NGO support capacity, acceptance and closure across nonclinical services. This is a scope/evidence boundary, not proof that a state cannot arrange NGO support. **Documented operational dependency:** staffed hubs, local facilitation, connectivity/equipment; the parliamentary response describes government measures addressing these needs.

7. ABDM ecosystem; National Health Authority/MoHFW; digital health infrastructure (one count)

Users and verified provisions: Citizens, health facilities/professionals and software integrators. ABHA identity, HFR facility registry, HPR professional registry and HIE-CM consented exchange; UHI health service discovery/interoperability. Official HFR announcement, 6 Jul 2022.

Geography and access: National ecosystem, including public/private health establishments. Registration, verification and connected software are separate steps; a health facility entry is not proof of real-time availability. NGO-run clinical facilities may participate within applicable facility/provider categories, while a nonclinical NGO is not automatically an HFR-verified health provider.

Beneficiary and district workflow: Patient uses an identity/PHR and controls record sharing; a connected provider generates records; another compliant provider requests consented access. District adoption/onboarding can connect facilities but does not itself assign an ASHA a funded local support provider.

Verified interoperability / API and access caveat: Official: PIB, 5 Aug 2025 documents HIE-CM, consent, sandbox validation and security audit. NRCeS current published FHIR guide v6.5.0, generated 8 May 2025 defines HIP/HIU roles, records and ServiceRequest/Appointment resources. Public schemas ≠ open patient data or blanket production access. The v7.0.0 page is a **preview/development build**, not the current published release.

Gap assessment and boundary: Not demonstrated: generic NGO due diligence, safeguarding, eligibility, funded intake capacity or responsibility for service completion. ABDM is compatible infrastructure, not evidence that a verified local NGO referral network already exists. Its ability to represent service requests also means it would be wrong to call referrals technically impossible. Health-data consent and service acceptance must be handled separately.

8. Ni-kshay (Nikshay); CTD/MoHFW with NIC and WHO India; national TB patient management and community-support ecosystem (one count)

Users and verified provisions: Public/private TB health functionaries; patients, treatment supporters, ASHAs, family caregivers and Ni-kshay Mitras/NGOs. Registers TB, orders tests, records treatment, monitors adherence, transfers patients and supports surveillance. Official About page.

Geography and access: National NTEP system; professional access is role-based. Donors/NGOs can register willingness for community support; district TB officers broker linkage. 2022 official community-support guide.

Beneficiary and district workflow: ASHA/field staff approaches TB patient for voluntary consent; Mitra registers geography/type/duration of support; DTO assesses need and finalizes delivery; portal records linkage and follow-up confirmation. The 2023 family-care guide shows tagging a patient to an enrolled caregiver, with ASHA and NGO-volunteer examples.

Verified interoperability / API and access caveat: Official About page confirms transfer between providers and lab-linked ordering; linked official knowledge base describes treatment/transfer/DBT workflows. No open third-party production NGO API or blanket access to TB records was verified. Registration/linkage does not entitle unrestricted beneficiary-data reuse.

Gap assessment and boundary: Strong existing precedent, not simply a gap: consent, donor registration, DTO coordination, assistance reporting and grievance route. **Boundary:** TB-specific. **Not demonstrated:** multi-sector nationwide NGO capacity/eligibility routing outside NTEP or uniform completion effectiveness in every district. The 2022 guide's operational rules are historical; verify later revisions before copying them.

Where a national-to-local service network could break

The following are **failure hypotheses to test**, except where an explicit documented dependency is identified. They should not be reported as proven nationwide failures of the platforms.

Handoff	Potential break and why it matters	Concrete validation / design requirement
National NGO identity → district service listing	Legal identity, a health facility registration or a donor commitment does not establish that a particular district office supplies a specific service today.	Verify district contact, service eligibility, geographic coverage, staff qualifications where relevant, delivery mode, charges and a renewal date. Keep organisation verification separate from service availability.

Handoff	Potential break and why it matters	Concrete validation / design requirement
ASHA due task → appropriate referral	A clinical due-list or consultation can identify need without naming an accessible nonclinical provider. An app may already support referral forms, but the recipient still needs to be connected operationally.	Test the actual beneficiary journey for transport, food assistance, disability support or other chosen services; identify who can refer, what data they see and who is responsible for follow-up.
Directory match → accepted slot	A listing can be accurate while the NGO is full, outside catchment, unable to fund the service or unwilling to accept a case.	Require explicit acceptance/decline, current capacity, response deadline, queue ownership and an alternative provider/escalation. Measure unanswered referrals, not just referral counts.
Consent → appropriate data sharing	ABDM consented health-record exchange and Ni-kshay's support consent are documented, but purpose, roles and data minimization differ for social support.	Record service-specific consent; share only necessary referral data with the accepting NGO; maintain an assisted route and withdrawal handling. Reuse existing official exchange only with authorized access.
Offline task → consistent shared state	ANMOL/AMRIT/CommCare/U-WIN already support offline use. Offline functionality does not ensure that a second provider sees newly accepted/completed tasks before synchronization.	Test sync latency, duplicate identities, conflicting updates and failed delivery under real district connectivity. Do not advertise offline as a unique new feature.
Accepted referral → completed service	Consultation, reported dose, recorded support count and actual beneficiary experience are different outcomes. Ni-kshay already prescribes follow-up confirmation, showing the importance of this step.	Record accepted, scheduled, attempted, delivered, beneficiary-confirmed, declined and failed states; use proportionate evidence, independent sample checks and a complaint route.
Completion → ASHA/supervisor accountability	App dashboards can track activities while unresolved cross-organisation referrals have no clear owner. Historical CommCare field evidence shows supervision did not automatically improve with digital tools.	Assign a named case owner and supervisor escalation, return concise outcomes to the ASHA, and monitor unresolved time and beneficiary-reported delivery alongside entries.
National scale → sustainable district operation	Software, state deployment, training and physical service capacity must all be funded. Existing pilots/implementation documents do not prove uniform scale or lasting capacity.	Pilot a few services in two contrasting districts; cost case coordination and field verification; obtain district/state workflow ownership; document minimum staffing and financing before scale.

Evidence cautions and exact source notes

1. **ANMOL features vs current state.** The 28 May 2024 manual, pp.5-8, 53-59 and 97, is operational documentation, not a 2026 impact evaluation. The MoHFW Play listing names the operator and was updated 18 Mar 2025 in the fetched listing; it mentions immunization synchronization fixes. User reviews describe particular difficulties but are not representative evidence and were not used to declare nationwide failure. Offline counseling media should not be confused with voice data entry.

2. **RCH current deployment.** The MoHFW annual report 2025-26, p.50, is the dated source for RCH and ANMOL deployment and cumulative records. It describes RCH 2.0 as an upgrade and should not be used to claim universal implementation of future features. The state Maharashtra workflow was undated in the reviewed text; its printout/data-entry route is specific documentation, not a measured current performance result for all states.

3. **U-WIN chronology.** PIB 29 Nov 2024 establishes completed nationwide rollout and features. Rajya Sabha 10 Mar 2026 verifies present integration and offline recording. Registration totals used here have an explicit 18 Mar 2026 date; no extrapolation to 8 Oct 2026 was made. SMS reminders require a reachable recorded phone; their availability is not

evidence of universal receipt or action.

4. **AMRIT marketing vs code vs impact.** Piramal product page is an undated operator claim for speech entry and workplans; FLW repository documents modules and offline/multilingual design. API repository establishes published REST service code. These do not independently measure voice accuracy, sync reliability, referral completion or state-wide active use. Named Bihar/Assam variants must not become a claim of nationwide ASHA coverage.

5. **CommCare prioritization is established.** September 2021 Precision Tasking article remains available at the current CommCare domain and linked from the current India services page. It is a customizable framework/reference application, not evidence that every deployment contains the framework. The older API Authentication page, updated 19 Apr 2024 is explicitly **archived** and points to API Overview; it should not be used to describe current SSO rules. The March 2026 announcement supplies the newer API-key requirement.

6. **Primary CommCare field evidence and limitation.** Bihar trial, 2019 used baseline 2012 and follow-up 2014 in Saharsa. It reports improved ASHA/AWW coordination and certain home visits/behaviours; FLW supervision was not significantly affected. The Mathematica issue brief, 20 Oct 2015 notes technical/logistical challenges and underused features despite positive effects. These are historical setting-specific findings, not proof that the current platform universally fails. No current RCT of cross-NGO India referral fulfillment was identified.

7. **eSanjeevani source interpretation.** C-DAC product page supplies the explicit 16 Aug 2026 network/consultation counts. The 2026 government response supplies all-State/UT rollout, frontline facilitation and state/NHM implementation. Service volumes establish real scale but do not measure continuity of social support or NGO referral closure. No statewide absence of specialist capacity was inferred.

8. **ABDM standards are broader than identity.** Official HFR release, 6 Jul 2022 covers public/private health facilities and interoperable APIs. Official 5 Aug 2025 release covers security audit/sandbox/consent. Published FHIR v6.5.0 includes ServiceRequest, Appointment, CarePlan and Organisation; therefore describing ABDM as only a health ID or unable to carry referral information is inaccurate. The v7 preview labels itself a local development build with draft date 15 Jul 2026. Official sandbox portal returned 403 to this research browser; this access failure was not treated as proof that APIs are unavailable to approved integrators.

9. **Ni-kshay already has service support closure.** CTD 2022 guidance, pp.7-17, describes patient consent, donor/NGO registration, district needs assessment, agreed delivery and DTO follow-up confirmation. Its instructions include involving NGOs/SHGs/supply chains and restrictions on repurposing patient data. This is a designed operational pathway, not independent evidence of completion for every registered commitment. The family caregiver guide, March 2023 file, p.33, confirms caregiver tagging examples; neither donor registration nor a caregiver role establishes broad NGO accreditation.

Method: read the requested Tavily research skill; connected Tavily search initially retrieved official sources but later returned HTTP 429 and extract returned HTTP 432 plan usage limit. Continued with official-source web search/open and primary research papers. No outreach, accounts, login, private patient data or production integration was attempted. Public documentation review cannot establish universal operational absence.

Part 6. 30 platforms - NGO discovery, funding and volunteering

Eight NGO discovery, funding and volunteering platforms

Research cut-off and retrieval date: **8 October 2026**. Scope: exactly eight platform ecosystems. Give.do, Give Discover and Give Grants are counted together as **Give**. This is public desk research using operator/government sources; no accounts, paid tools, API calls, outreach or referral tests were undertaken. "Documented" means provider-described, not independently demonstrated performance.

These platforms can help an ASHA coordinator identify organizations and mobilize resources. Their public evidence does not establish a ready-made district-level ASHA referral service with accepted cases, current village catchments and confirmed spare capacity. That is an **integration and field-validation question**, not proof that every operator lacks such capabilities.

Eight-row comparison

1. NGO Darpan / NPO Darpan. Maintained by NITI Aayog's Voluntary Action Cell; NGOs/NPOs, government departments and people identifying organizations.

Geography and documented service/resource capabilities: Indian NPO registration identity and directory. Darpan ID is required for specified government grant/compliance interactions; this does not itself allocate money or make a grant successful. NITI describes the directory as usable by CSR teams to connect with NGOs. VAC, directory use.

Verification and realistic ASHA connection: Use to cross-check an organization's identity and discover candidate partners; then confirm its actual health/nutrition program and village coverage directly. The portal explicitly states that its ID does **not** confer NITI endorsement, affiliation or recognition. A registration ID is neither a quality seal nor evidence of today's service availability. Portal notice.

API/integration evidence and limits: No current public developer API/access terms were verified in this review. Government-to-government integrations in older documents do not establish startup access. MVP: maintain consented partner records and links; request an authorized data arrangement before bulk integration.

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2. GuideStar India. An initiative of Civil Society Information Services India (CSISI), a public charitable trust. Serves donors, institutions, researchers and NPOs.

Geography and documented service/resource capabilities: India-focused NPO discovery by state/district/pincode, cause, financial/compliance information and certification level; profiles and partner-identification/due-diligence support. Site also supports opportunities to give, volunteer or contribute in kind. Official site.

Verification and realistic ASHA connection: Its Trust & Transparency certification has graduated levels and an assessment framework. Check the particular organization's current level and validity, rather than treating every directory entry as equally vetted. Good seed source for an ASHA-linked NGO shortlist; confirm service catchment separately. User-submitted asks/offers/events carry an explicit warning that they are **not vetted** by GuideStar India. Official site and warning, active certification resources.

API/integration evidence and limits: The homepage advertises "Access API solutions" but labels them **"Coming soon"** and provides contact. A presently usable API was not verified. Treat it as a possible future partnership, not an available dependency. API card.

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3. Give ecosystem - Give.do / Give Discover / Give Grants. Current platform terms name SaathiRe Social Impact Solutions Private Limited as owner/operator and name US affiliates. Donors, NGOs, CSR teams and grantmakers are the users.

Geography and documented service/resource capabilities: Primarily Indian impact: Discover supplies NGO profiles/search and grantmaker profiles; Give.do supports donations/fundraising; Give Grants provides nonprofit identification and CSR/grant support. Discover's FAQ mentions international listings, but its main demonstrated directory emphasis is India. Fundraising/CSR access is not guaranteed unrestricted cash. Platform terms, Discover, Give.do overview.

Verification and realistic ASHA connection: Give's Bronze, Silver and Gold diligence differ in depth; Gold includes site visits, Silver virtual leadership/donor assessment, Bronze compliance checks. Transparency stars concern information completeness, distinct from diligence seals. An ASHA coordinator can find/contact an NGO; an eligible NGO can frame a program/fundraiser for aggregated needs. Do not assume a listed profile is authorized to fundraise or accepts individual cases. Diligence service, seal criteria.

API/integration evidence and limits: No Give.do/Discover public developer API was verified in this review. Claiming and updating NGO profiles is documented. Use links/manual onboarding or negotiate permission; unrelated "Give Protocol" APIs are not Give.do evidence. Claim process.

4. GlobalGiving. Operated by GlobalGiving Foundation (US) and GlobalGiving UK. Donors, nonprofits and corporate philanthropy users.

Geography and documented service/resource capabilities: A **global**, not India-only, fundraising/grantmaking ecosystem; vetted nonprofits can raise funds and use training/support. Indian organizations/projects are potential partners, subject to eligibility and lawful receipt/disbursement requirements. Supports local projects internationally, but does not guarantee a local responder or a grant. Nonprofit terms, how vetting works.

Verification and realistic ASHA connection: Legal, financial and program information is reviewed, with recurring project reports and due-diligence renewal. An existing eligible nonprofit could raise program or general support funds for ASHA-linked needs. GlobalGiving explicitly documents the ability to raise unrestricted funds; that **possibility** is distinct from a guaranteed award or immediate patient cash. Vetting, 2025 diligence explanation.

API/integration evidence and limits: Official API documentation exists for organization/project data and donation options. Keys and usage terms apply; donation enablement and commercial use have additional requirements. This is philanthropy data/payment integration, not proof of live beneficiary referral capacity. Public docs include legacy material; confirm supported version with the operator before building. API, developer terms.

5. NGOBOX. Operator footer: Renalysis Consultants Pvt Ltd; associated with the CSRBOX ecosystem. Development professionals, students, NGOs, social businesses and CSR users.

Geography and documented service/resource capabilities: India-centered opportunity/knowledge platform: jobs, fellowships, grants, RFPs, tenders, events, resources and outreach/advertising; links to CSR research and partnership services. This is an opportunity discovery channel, not an automatic source of money. About and operator footer.

Verification and realistic ASHA connection: Its current About page establishes curation and outreach, but does not substantiate a uniform organization-level verification standard for every listed opportunity or NGO. An NGO partner can monitor relevant calls, recruit a coordinator or locate CSR networking events. Verify the original issuer, geography, deadline and eligibility for each call. Large marketing audience/database counts do not establish local service capacity. About/services.

API/integration evidence and limits: No public developer API/access terms were verified. Practical MVP use is links and manual opportunity research; bulk reuse requires confirmed permission. Avoid equating NGOBOX's informational listings with CSRBOX's separate contracted implementation/diligence services.

6. Goodera. Its published terms name Nextgen Project Management Systems Private Limited in India and Nextgen Impact Inc outside India. Corporate clients/employees and nonprofit partners.

Geography and documented service/resource capabilities: Global corporate volunteering: matching and scoping with nonprofits, hosted virtual/in-person/hybrid activities, logistics, in-kind/digital outputs and impact reports. Nonprofit partnerships have no platform/subscription fee according to its FAQ; specific in-kind wishlists can be discussed with a partnership manager. Terms, nonprofit service.

Verification and realistic ASHA connection: Operator pages document nonprofit sourcing/vetting, but this review did not establish a public country-specific vetting rubric. A local NGO could propose suitable resource-pack assembly, translated materials, campaign support or skilled volunteer work for ASHA-linked programs. Match and corporate scheduling must be agreed; the global network count is not district-level staff availability or patient funding. Vetting service, nonprofit workflow.

API/integration evidence and limits: Official REST API exists, with request-access link and API-key examples: opportunities, events, registrations and participation/time tracking. An operator-announced MCP service also exists. Availability, contract, scope and credentials require agreement; neither implies free public access to NGO clinical referrals. REST docs, MCP announcement.

7. iVolunteer. Flagship program of MITRA Technology Foundation. Individuals, NGOs/community organizations and companies seeking employee volunteering.

Geography and documented service/resource capabilities: India-focused volunteering; metro centers plus operator-described wider partnerships/fellowships. Health/HIV/AIDS, disability, child/elderly welfare and other causes are included. Partner NGOs receive a free volunteer-management account with opportunities, applicant communication, attendance/hours and reports; specialist, training and strategic support are also described. MITRA, iVolunteer scope, NGO services.

Verification and realistic ASHA connection: The operator describes credible/verified partners; a detailed public vetting rubric was not established here. Partner signup captures organizational identity, location/contact and volunteer needs. A participating local NGO can recruit and manage nonclinical support volunteers, subject to suitability, training and actual rural reach; volunteer provision is not a cash award. Organization signup, NGO services.

API/integration evidence and limits: A Salesforce-based VMS is documented; this is **not** evidence that its underlying APIs are available to third-party startups. No iVolunteer public developer access terms were verified. Work through a partner account/coordinator until integration authorization is established. VMS details.

8. ConnectFor. A program of the Rosy Blue Foundation. Individual volunteers, NGOs, institutions and corporate volunteering/CSR users.

Geography and documented service/resource capabilities: India-focused matching by causes, skills and availability, NGO volunteering needs and corporate engagement services. Health is an explicit cause. Current corporate pages describe volunteering strategy and activities. About, NGO service, volunteer filters, corporates.

Verification and realistic ASHA connection: Publicly indexed NGO onboarding terms require registration proof, PAN and verifiable address. These terms explicitly restrict the opportunity channel to time/skills and exclude requests for donations/financial transactions. Source is on its **beta hostname** with older Section 25 terminology; confirm current contractual terms. A local NGO could recruit trained support volunteers, not assume this channel accepts an ASHA cash request. Published onboarding terms.

API/integration evidence and limits: No public developer API/access terms were verified. Manual NGO onboarding/opportunity matching is the demonstrated route. Volunteer applications, hours or attendance are not evidence of a patient referral handoff, local transport inventory or case completion.

What this means for an ASHA visit optimizer

Proposed pilot approach: use Darpan for identity cross-checking, GuideStar India and Give Discover for a shortlist, and direct partner onboarding to establish service availability. Approach iVolunteer/ConnectFor for suitable volunteer support and Goodera for scheduled corporate participation or in-kind work. Treat Give, GlobalGiving and relevant NGOBOX calls as longer-term program/funding channels, with eligible nonprofit partners and explicit budgets.

Keep organizational discovery separate from operational matching. An organization's registered office, pincode, district label or national presence claim is not its actual beneficiary catchment. A directory total is not the number of active organizations available to support one village this week. GuideStar India, iVolunteer and Give pages have multiple historical counts; this report deliberately avoids turning them into an addressable active-service estimate.

Pilot directory fields to validate with each partner: legal name/identifiers; evidence source and certification expiry; service category; covered villages/blocks; referral eligibility; exclusions; current staff/stock/budget; response window; contact owner; accessibility/languages; referral route; acceptance/decline reason; next availability date; and completion feedback. These are proposed product requirements, not features claimed to exist on every platform.

An ASHA-recorded request for food, accompaniment, disability support or entitlement assistance should reach a partner only after a coordinator checks suitability and the resident consents to necessary sharing. Aggregated needs can inform a donor proposal without disclosing patient names publicly. Clinical urgency must continue through the appropriate health-system pathway; a fundraising or volunteering search is not an emergency response mechanism.

Evidence distinctions and corrections for synthesis

• **Darpan identity is not endorsement.** The portal's notice expressly denies endorsement, affiliation and recognition; do not label an NGO "NITI approved." Its signup guide describes document checks for ID generation, which should not be translated into impact certification. Notice, official signup guide.

• **Verification differs by level and content.** GuideStar certification is separate from its explicitly unvetted user announcements. Give transparency stars describe profile completeness, while diligence seals describe checks. A historical badge or listing must be checked for current validity. GuideStar, Give standards.

• **Funding is conditional.** Access to donors, grant listings, CSR buyers or a fundraising account does not guarantee funds, unrestricted spending, medical reimbursement or fast disbursement. GlobalGiving does document unrestricted fundraising as an option; do not overcorrect by claiming all platform funding is restricted. GlobalGiving explanation.

• **Two APIs are documented; one is announced as future.** GlobalGiving and Goodera have official API documentation. GuideStar India's homepage says "Coming soon." API access for the remaining five was not verified, rather than proven absent. No authenticated functionality, pricing, SLA or production access was tested.

• **Operators must be named accurately.** Current Give platform terms identify SaathiRe; NGOBOX's current footer identifies Renalysis Consultants; GuideStar India is CSISI; iVolunteer is MITRA's program; ConnectFor is Rosy Blue Foundation's program. Historical GiveIndia legal documents should not override current platform terms.

• **Capacity and workflow gaps remain hypotheses.** None of the reviewed public pages proves a ready live directory of rural health-support capacity integrated with ASHA schedules. This supports testing a coordinator-managed referral layer; it does not establish that existing platforms cannot add or already privately provide one.

Source provenance and practical limits

All substantive factual sources above are government or platform-operator pages. NITI's VAC page establishes Darpan's current institutional role. Direct Darpan page extraction returned no body text; the official portal's indexed notice and official signup PDF supplied the registration/endorsement evidence. NITI's directory-use article is older context: its historical NGO count was not reused as a 2026 total.

GuideStar's current homepage supplies operator identity, search, certification categories, user-content warning and the future API label. Its resource site distinguishes active certification lists. Give's January 2026 hosted platform terms supply current operator identity; Discover, Give.do overview, seal criteria and Give Grants diligence page establish distinct discovery, fundraising and assessment services. Discover shows inconsistent/historical seal counts in different blocks; those counts were excluded.

GlobalGiving's nonprofit terms identify US/UK operators. Its vetting/compliance sources establish recurring review/reporting; its 29 May 2025 operator interview explicitly explains unrestricted fundraising. API terms/documentation establish an integration offering, not acceptance of this proposed use case. Ongoing compliance.

NGOBOX's About page/footer supplies current operator and listing/services evidence. Goodera's nonprofit pages, terms and official API page establish current service and developer offerings; its MCP article is a **provider announcement**, not independently validated readiness or an endorsement of the article's "first" claim. iVolunteer's NGO service and signup pages establish the VMS and onboarding workflow; no unpublished verification methodology was inferred. ConnectFor's current About/NGO/volunteer/corporate pages establish current scope, while its indexed beta onboarding terms are a specifically identified older/legacy contractual source.

Page retrieval or crawler recency is not a feature launch/update date. Public documentation can be incomplete or outdated; contract, access rights, local reach and service availability require operator/partner confirmation before a deployment.

Part 7. 30 platforms - scheme access, CSR and crowdfunding

Eight India civic-benefit and funding services for an ASHA visit optimizer

Research cut-off and access date: **8 October 2026**. This is an assessment of official public documentation and operator claims; no accounts were created, applications submitted, donations collected, or outreach performed. Dated releases establish historical facts; current pages establish advertised capabilities, not independently measured service quality. The handoff column below is a **proposed operational use**, not a claim that each platform already implements a shared ASHA-to-district workflow.

Exactly **eight service rows** are reserved here. National CSR Exchange and National CSR data reporting have different functions but belong to the same Ministry of Corporate Affairs ecosystem. They are not two independent companies or independent funding markets. The historical National CSR Portal and current CDM CSR-data access are counted once in row 5.

1. myScheme - NeGD, supported by MeitY/DARPG and participating departments; national scheme discovery. Operator

Verified features and user access: Current FAQ: over 4,700 central/state/UT schemes; profile and scheme-specific eligibility questionnaires; benefit/document/application information; 15 content languages. Browsing needs no sign-in; MeriPehchaan enables saved profiles/bookmarks. FAQ

Geography / practical reach: India, with eligibility depending on state, residency and the individual scheme. National discovery coverage does not prove every local entitlement is listed.

Proposed beneficiary → district / NGO handoff: During a visit, ASHA/NGO uses agreed household attributes to identify candidate schemes and missing documents. Assign an application owner; follow the linked department application route and record its receipt/status.

Integration and evidence limits: Discovery and screening do not constitute government approval. The FAQ says applications currently redirect to the responsible department. Direct application through myScheme/UMANG is described as a future phase. No public third-party integration API was verified.

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2. UMANG - NeGD/MeitY; government service aggregation. NeGD project

Verified features and user access: Android/iOS and web access to participating central, state and local services; service applications, bills and status features; DigiLocker document access; support and grievance links. Current service homepage

Geography / practical reach: Pan-India interface; the catalogue and transaction requirements vary by department and state. An available national app does not make every service available in every district.

Proposed beneficiary → district / NGO handoff: ASHA helps the beneficiary find the relevant participating service; beneficiary completes required authentication. Preserve department acknowledgement; district/NGO case owner follows up through the responsible service rather than interpreting app access as sanction.

Integration and evidence limits: Official procurement documentation establishes service connectors and API architecture, including third-party access architecture; it does **not** establish an unrestricted public ASHA API or permission to read beneficiary accounts. NeGD RFP

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3. Rajasthan eMitra - Rajasthan government service-delivery ecosystem, DoIT&C/RISL and kiosks; assisted transactions. State department description

Verified features and user access: Government/private services through a state platform and assisted kiosks; certificate applications, scheme enrolment and payments. Official tariff lists ₹50 government application service, ₹10 fee payment up to ₹2,000, additional ₹2 per ₹1,000 above that, and ₹20 pre-printed certificate print. Departmental charges are additional. Tariff

Geography / practical reach: Rajasthan-specific service delivery; kiosk presence and the actual service must be checked locally. Avoid treating historical district/kiosk counts as current coverage.

Proposed beneficiary → district / NGO handoff: ASHA refers a document or enrolment task to an identified kiosk, explains expected cost, and captures the official receipt/application number. NGO/district owner pursues missing documents or departmental decisions.

Integration and evidence limits: Fees should be disclosed in the referral; free access should not be assumed. No current publicly documented general-purpose ASHA integration API was verified. A kiosk helps submit a transaction; the competent department makes the decision.

4. National CSR Exchange Portal - MCA; corporate-implementing-agency project exchange. Portal

Verified features and user access: Free registration for companies with a valid CIN and implementing agencies with a valid CSR registration number; corporate RFPs, agency requests for funds, project linking, notifications and project management. Individuals cannot register in those roles. Official FAQ

Geography / practical reach: India; nearby listings use registered locations, which may differ from project delivery locations.

Proposed beneficiary → district / NGO handoff: ASHA identifies a pattern of unmet need; a district partner NGO develops a costed project and verifies its implementing-agency eligibility. That eligible organization submits a proposal; the company handles selection/due diligence.

Integration and evidence limits: Registration does not guarantee funding; MCA does not independently verify every listed agency/company. CSR remains a corporate board decision. No verified open integration API. This is a proposal channel, not an individual emergency-fund entitlement.

5. National CSR Portal / CSR data reporting, historically csr.gov.in; current official MCA CDM CSR-data view - count this service once. Current CSR data

Verified features and user access: Public reporting interface with year, state, district, sector and PSU/non-PSU filters; report/export controls and implementing-agency master. Visible exports require CAPTCHA. Current banner: snapshot **31 March 2026, FY 2024-25**.

Geography / practical reach: India-wide reported expenditure, including categories for pan-India/not-classified spending. Reported geography is not an inventory of funds currently available locally.

Proposed beneficiary → district / NGO handoff: District/NGO analyst uses past local health/nutrition/sanitation spending to form a funder prospect list, then checks companies' current policies and actual project eligibility. An ASHA case itself receives no payout through this data view.

Integration and evidence limits: This reports company filings and historical expenditure. It is not the Exchange's RFP/RFF workflow and not a grant application. No public data API was verified; an export button is not API evidence. Legacy URL timed out during this audit; no authoritative migration date or redirect was established.

6. Haqdarshak - Haqdarshak Empowerment Solutions Pvt. Ltd.; independent benefits-information and facilitation business. Operator

Verified features and user access: Operator advertises discovery/eligibility, trained in-person agents, ecosystem training, post-application grievance support and a QR Yojana Card. Its citizen app provides scheme information, eligibility questions and requests for guidance to official channels. Developer listing

Geography / practical reach: Operator reports agent training across 30+ states/UTs; this does not verify an active agent or full-service programme in a chosen block. App lets users select their state/language.

Proposed beneficiary → district / NGO handoff: NGO/district can explore a local facilitation partnership: obtain permission for referral, confirm the available field team and fees, assign document/application tasks and retain official application receipts.

Integration and evidence limits: Operator explicitly advertises a modular **SaaS/API-based stack**. This verifies a commercial integration claim, not a public endpoint, open licence, live district integration or grant-approval authority. The app expressly disclaims government affiliation and benefit approval/guarantees.

7. Milaap - Milaap Social Ventures India Pvt. Ltd.; donation crowdfunding intermediary. About

Verified features and user access: Individual and NGO fundraisers, campaign dashboard, donations/receipts/withdrawal management and support. Pricing advertises 0% platform fees but payment-processing fees; 80G-enabled organizations can issue receipts using its dashboard. Pricing

Geography / practical reach: Campaigns for causes across India; donors may be wider geographically, subject to recipient and campaign requirements. There is no verified district case-service network.

Proposed beneficiary → district / NGO handoff: After relevant public-benefit screening, family/authorized NGO prepares a documented residual funding request. Platform review and bank/KYC approval precede withdrawal; district case owner tracks money actually received and its use.

Integration and evidence limits: Campaign publication does not guarantee donations or emergency payment. No public campaign integration API was verified. Platform fees, gateway fees, optional tips and any agreed outreach costs must be distinguished.

8. Ketto - Ketto Online Ventures Pvt. Ltd.; donation crowdfunding intermediary. Operator overview

Verified features and user access: Individuals/NGOs create fundraisers; web/mobile tools and support vary by plan. Bronze is free; Silver lists 5% and Gold 7% platform fees; paid services can add campaign support/visibility. Pricing also lists 3% gateway charges and GST on relevant fees. Pricing

Geography / practical reach: India-oriented platform; an online campaign does not establish local NGO capacity, a district referral endpoint or donor availability.

Proposed beneficiary → district / NGO handoff: Beneficiary/authorized organization supplies evidence and approved bank/documents. For a fundraiser raised by an individual for an NGO, the NGO must withdraw; the individual can remind it. Withdrawal help

Integration and evidence limits: Terms expressly say campaigns may receive no donations. A free starting plan does not imply zero total deductions. No public integration API verified. Use actual net approved payout in the case ledger. Terms

Current and dated evidence, with detail URLs

1. **myScheme.** PIB records dedication of the discovery platform on **4 July 2022**: Digital India Week release. The current official FAQ bears **18 September 2026, v3.1.27**. It distinguishes 15-language content from a Hindi/English voice/text chatbot, and acknowledges continued scheme onboarding. A successful screening answer is provisional evidence for a department application, not a sanctioned benefit.

2. **UMANG.** The official eBook dates dedication to **23 November 2017**. The current homepage bears **15 January 2026** and provides support hours; its machine-readable statistics displayed zero placeholders, so those counters were not treated as actual totals. A PIB June 2026 account reports 11.39 crore registrations as of **2 June 2026**. The government eBook describes RCH ANC/immunization calendar access, but this historical description alone does not verify a live ASHA scheduling or household-routing feature.

3. **eMitra.** The tariff page, indexed with **26 February 2026** update date, says the listed service rates include taxes, while departmental charges are extra. It offers an overcharging complaint route. Confirm service-specific exceptions and the amount shown on the receipt. The Rajasthan department page discusses scheme enrolment and rate-setting; some scheme names in that descriptive page may be historical and must be checked against the live service catalogue.

4. **CSR Exchange.** A 6 June 2022 MCA/PIB release announces the next day's launch. A Lok Sabha answer dated 18 December 2023 confirms launch in June 2022 and, **as of 7 December 2023**, 838 companies, 6,123 agencies, 34 RFPs and 435 RFFs. These are dated participation counts, not 2026 totals or evidence of completed grants. The same answer documents login/registration/data queries and a helpdesk response; it does not support alleging universal platform

failure.

5. **CSR data service and counting.** The 9 February 2018 MCA/PIB release identifies csr.gov.in as a disclosure-based National CSR Data Portal. The current CDM CSR-data view supplies present public access to CSR reports. This report uses one historical/current data-service row, rather than inventing a separate additional competitor for CDM. The CDM FAQ identifies MCA21 forms/attachments as report inputs; the CDM about page explains the analytics/data-warehouse role. The precise transition date from the legacy site was not established. Snapshot date and reporting fiscal year must both be shown.

6. **Haqdarshak.** The operator-linked Google Play listing was updated **28 July 2026**, lists 6,000+ schemes and 10+ languages, and characterizes the app as information/guidance rather than government processing or approval. The operator website instead reports 5,000+ programmes and more than 14 local languages across its broader offer. Record these as different current operator descriptions, not one reconciled audited number. The website's field/API offer and app's narrower disclaimer can coexist; a partnership still needs scope and coverage verification. No public rate card, endpoint documentation or district service-level commitment was established.

7. **Milaap.** The operator overview dates founding to **2010**. The official pricing page publishes gateway ranges effective **1 April 2026**: **0.6-1.5% for INR** and **0.25-2.8% for USD**. Its 0% platform-fee statement applies to individuals/NGOs/organizations; selected outreach is subject to case assessment. The terms contain generic platform-fee language and expressly allow holds, withdrawal approval and outreach costs. Therefore retain the current campaign's actual written fee arrangement; do not equate the headline with guaranteed zero deductions or a payout deadline. An operator NGO article dated 13 July 2024 describes partial withdrawals and cause/expense verification; these are conditional processes.

8. **Ketto.** The operator overview dates founding to **2012**. The current plan page lists **18% GST on the success/platform and gateway fees**, rather than on the entire amount raised. It reserves pricing changes and mentions possible advertising and other campaign expenses. Exact total deductions therefore need campaign-specific confirmation. Withdrawal help requires approved documents and cleared funds; its indicated working-day timelines are conditional guidance, not universal emergency guarantees. The terms also require permission for depicted people and parents/guardians for minors, with informed explanation of wide image use.

Design implications for the ASHA + local NGO assistance layer

These are **design hypotheses to test locally**, not findings that every existing service fails:

• Separate the beneficiary's record into scheme candidates, applications, official decisions, NGO service commitments, corporate proposals, donations received and verified expenditure. A discovery match, CSR expression of interest and published fundraiser represent different stages.

• Give each referral an owner, receiving organization/contact, requested action, permission record, acknowledgement, expected next update and escalation route. The public sources reviewed do not establish one shared district referral contract across these eight services.

• Start with linked official pages and receipt-based reconciliation. Pursue a Haqdarshak commercial API or government integration only after scope, access rights, security responsibilities and permitted data exchange are agreed; avoid inferring access from an app's internal APIs.

• For Rajasthan, make kiosk charges visible and distinguish service fees from departmental fees. Elsewhere, identify the appropriate local assisted-service channel rather than importing Rajasthan eMitra as a national network.

• Keep a district NGO directory independently checked for service category, operating area, present capacity and eligibility. CSR portal registration, campaign presence and historical spending are insufficient evidence of an NGO's ability to accept a particular referral today.

• Use aggregate, documented unmet needs for possible CSR projects. The government release and Exchange FAQ describe company-board control; neither creates a pooled cash balance that the optimizer can allocate. Crowdfunding may supplement a documented residual need but donor response and net payout remain uncertain.

• Clinical urgency and ASHA visit priorities need their own authorized health workflow. None of the reviewed documentation established an end-to-end clinical triage, route optimization, NGO acceptance and funding approval engine. This is a bounded evidence gap to validate through product demonstrations and a district pilot, not proof that

such private integrations do not exist.

Key corrections for synthesis

myScheme ≠ sanction; UMANG availability ≠ department approval; Rajasthan eMitra ≠ nationwide/free assisted delivery; CSR disclosures ≠ available grants; CSR Exchange ≠ pooled cash or guaranteed funding; Haqdarshak ≠ government authority and is more than a discovery-only website; 0% crowdfunding platform fee ≠ zero total deductions; published crowdfunding campaign ≠ donations or immediate payout.

No claim of an open API should be made for six services where public documentation was not verified. For **UMANG**, distinguish official integration architecture from access authorization. For **Haqdarshak**, distinguish an operator's commercial SaaS/API offer from published endpoint documentation. Across all eight, absence of public verification is a research limit, not evidence that no private integration exists.

Part 8. 30 platforms - close analogues and current tools

Six additional comparators: close analogues and current policy overlap

Research date: 8 October 2026. These six entries extend the three eight-platform modules to **30 platforms/services**. They are not six unique new markets. Government tools, configurable infrastructure and overseas providers have different availability. This is public-documentation research, not authenticated testing.

25. Khushi Baby CHIP / Nirogi Rajasthan; Khushi Baby; India, with Rajasthan and other government partnerships

Documented features or announced scope: Integrated community/primary-health coordination, worker applications, health-official analytics and implementation support. The older annual report explicitly separates existing features from planned enhancements. Operator overview, 2021-22 annual report

Relevance and evidence boundary: Very relevant local comparator: an NGO already works with government on ASHA/ANM digital infrastructure. Undated newer and older webpages show different claimed reach totals; neither is used as independently verified current adoption. Their broad primary-care coordination scope should not be dismissed as a simple directory.

Practical implication: Ask what the pilot district already uses and explore a compatible support layer. Reviewed sources do not establish an all-NGO service-capacity network, but that absence of documentation is not proof that CHIP cannot support or already customize referrals.

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26. OpenSRP; Ona and contributors; configurable international health-system software

Documented features or announced scope: FHIR-native Android, administration and analytics; patient/household registries, care plans, due/overdue tasks, offline sync, reporting and commodity management. Operator documentation, features as of release v1.0.1 / Sep 2023, overview

Relevance and evidence boundary: Existing configurable software. The current docs describe household needs, interventions and referral history/outcomes. Immunization package. Features depend on implementation; no ready-made India-wide NGO directory or approved government record access was verified.

Practical implication: Reuse concepts/standards and investigate adaptation before rebuilding an entire health record system. Offline tasks, due lists and referrals are established capabilities, not novel inventions.

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27. Unite Us; Unite Us; primarily United States social-care ecosystem

Documented features or announced scope: Social-needs screening, secure referrals, provider response, service-specific resolved/unresolved outcomes, shared case communication, integrations and reporting. Official referral product

Relevance and evidence boundary: Strong existing analogue to the proposed ASHA-to-support loop. Public materials also describe community organizations receiving referrals and reporting services. Community organization offering. Its reported outcomes and US funding/privacy ecosystem cannot be copied as evidence for an Indian district.

Practical implication: Study accountable partner onboarding and outcome definitions. Indian ASHA roles, programme eligibility, weak connectivity, district operations and NGO catchments still require local implementation. Do not claim closed-loop social referral as a world-first.

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28. Findhelp; Findhelp; primarily United States social-care ecosystem

Documented features or announced scope: Configurable social-needs screening, programme recommendations, private cross-organization referrals and individual/population outcomes; integration, benefits enrollment and fulfillment are advertised in its offering. Official platform page

Relevance and evidence boundary: Direct social-support comparator. Its help documentation distinguishes referral mechanisms and programme-established next steps; referrals may use existing intake channels. Sending programme

referrals. No operational India ASHA network or India-government integration was verified.

Practical implication: Design assisted/local intake and explicit service-specific closure rather than pretending one workflow fits every provider. Discovery and fulfillment exist internationally; local verified capacity and governance remain execution questions.

29. Shaishav App; MoHFW / SSBSK implementation; India

Documented features or announced scope: Official August 2026 workshop describes at-risk line-listing, scheduling/tracking home visits, referral follow-up, supervision and longitudinal child profiles. MoHFW workshop, 24-25 Aug 2026

Relevance and evidence boundary: Announced/described tool with state readiness discussions, not verified universally live deployment by October. The June SSBSK launch announced national-platform integrations in future tense; this is not proof of live connections. SSBSK launch, 29 Jun 2026

Practical implication: An essential current overlap: home-visit tracking and clinical referral follow-up cannot be advertised as absent from government plans. Position any prototype as an optional routing/nonclinical-support companion subject to state approval and compatible data exchange.

30. Poshan Tracker; Ministry of Women and Child Development; India, AWW/Anganwadi workflow

Documented features or announced scope: Anganwadi service monitoring, beneficiary/growth records and activity content. An AWW home-visit scheduler was integrated in April 2026: automatically schedules 23 structured visits for pregnancy and children through three years, with relevant counselling/video prompts. Official backgrounder, 14 Apr 2026, home-visit section

Relevance and evidence boundary: A live, documented scheduling overlap for **AWWs**, not evidence that the same tool is every ASHA's schedule. The source describes programme-level monitoring; its broad claims about eliminating delays are not independent proof of perfect delivery. No public NGO-capacity integration or uniform local reliability was verified.

Practical implication: Coordinate AWW and ASHA tasks where authorized, rather than adding conflicting visit calendars. Access, joint ownership and physical support availability must be agreed locally. Programme services and its software are separate categories, not additive beneficiary populations.

Market conclusion

Public sources show a **developed category**, not an empty market. Near-term value depends on which local handoff remains unresolved after existing systems are used. The defensible pitch is a testable, small operational improvement: a feasible worker day plus a consented, service-specific request to an available local provider with an accountable outcome. Build-versus-configure and partnership decisions require real district interviews.

No US platform's marketing completion rate, total reach or financial return is used as a benchmark promise for this Indian proposal. No current government API access, NGO participation or platform integration is claimed.

Part 9. Network breakpoints and ten stakeholder perspectives

Service breakpoints: patient, caregiver and operations review

Research cutoff: **8 October 2026, India**. Contribution 8 of the requested ten research perspectives. Read alongside `01-asha-ground-truth.md`, `04-welfare-access-schemes.md`, `03-digital-health-platforms.md` and `11-solution-design.md`. This review adds a narrow primary-evidence check and an operating model; it does not establish local partner participation, capacity, government approval or clinical effectiveness. No outreach occurred.

Strongest practical correction: make a named, resourced coordinator and a next action mandatory before a support case enters the provider queue. The ASHA should capture a consented barrier once, while that coordinator obtains acceptance, resolves non-response, checks fulfillment and returns a short outcome. A directory and an algorithm cannot perform those jobs. Pilot the process on paper/phone with real service offers before building a large network.

Evidence ledger: five focused primary sources

Labels used below: **E** = implementation observation or study finding; **P** = official prescribed responsibility/process; **H** = proposed feature or locally unvalidated failure hypothesis. E does not establish national prevalence, and P does not prove implementation.

S1

Source and timing: CTD Ni-kshay Mitra Guidance 2026; official posting 24 March 2026; preface dated 30 January 2026

What it establishes; limit: E: programme guidance acknowledges fragmented food-basket delivery, inconsistent support quality and delayed donor linkage. **P:** DTO linkage/validation, district supervision and confidential, consented volunteer engagement. See printed pp.6-7, 15-17, 21 and 36. A programme diagnosis and prescribed redesign, not a prevalence survey or proof that new dashboards work everywhere.

S2

Source and timing: NHSRC/MoHFW 16th Common Review Mission, 2024; November 2024 visits, official file hosted March 2025

What it establishes; limit: E: state-varying upward/downward referral arrangements; post-discharge continuity gaps at visited sites. Printed pp.131-133 document duplicate reporting and Himachal Pradesh block-held RCH/OTP dependence; p.140 documents referral findings. Rapid field review of selected places, not a nationally representative 2026 failure rate. Gujarat's functioning severe-anaemia/malnutrition pathway is a counterexample to blanket absence claims.

S3

Source and timing: Kar et al., RBSK caregiver study, 2020, 31(1):30-46

What it establishes; limit: E: 115 caregivers across selected Pune rural blocks/municipal area, interviewed 3-9 months after referral. Among 29 children with developmental disabilities, treatment stopped for 24; reported barriers included waiting, distance, transport and lost wages. Condition- and district-specific historical evidence; cannot be projected to all RBSK referrals or 2026 RBSK 2.0. Journal abstract was retrieved through search; direct article opening failed.

S4

Source and timing: Shetty et al., ASHA work sampling, 2025, J Family Med Prim Care 14(6):2138-2142

What it establishes; limit: E: mixed-method study of 26 ASHAs at Bengaluru medical-college urban/rural health centres identifies training and incentive needs. Small local sample; does not estimate national worker hours or prove that an extra NGO form is acceptable. PubMed abstract verified; full PMC access encountered a browser challenge.

S5

Source and timing: Carmichael et al., Bihar ICT-CCS cluster trial, 2019, doi:10.7189/jogh.09.020424; evaluation 2012-2014

What it establishes; limit: E: a digital job-aid intervention improved several home-visit/behaviour measures and ASHA-AWW coordination; supervision did not significantly improve. Historical district trial: supports testing task coordination, not a claim that software alone closes NGO cases or improves all clinical outcomes.

The ledger is deliberately small. Existing Ni-kshay community-support linkage, frontline task/case applications and referral platforms including Unite Us and findhelp already exist. Do not pitch "there is no referral platform." The narrower hypothesis is that a particular district lacks a usable connection between its approved visit workflow and **fresh, eligible, reachable nonclinical service capacity with accountable follow-through**. Platform presence, referral functionality, deployment, active use and service fulfillment are different facts; the companion platform catalogue supplies the product comparison.

Eleven handoffs: where the network may break

All fixes in the last column are **H**, proposed operating/product choices. Each needs a receiving human and a real channel, including assisted phone/paper intake when agreed.

1. Assigned household → today's visit

Evidence status and possible break: E S2: duplicate systems can impair timely records. **H:** imported due list, household availability or coordinates are stale.

Existing owner or required local decision: ASHA confirms assigned household; ANM/supervisor approves due rules and exceptions.

Practical fix and beneficiary protection: Reuse approved IDs/records; show source and last update; let worker correct availability/location. Keep unlocated/remote households visible with age of deferral. No out-of-catchment reassignment by an algorithm.

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2. Observation → clinical escalation

Evidence status and possible break: P, local 01: ASHA facilitates; trained clinical team owns clinical decisions. **H:** a support request diverts attention from urgent assessment.

Existing owner or required local decision: ANM/CHO/MO and established referral facility.

Practical fix and beneficiary protection: Clinical route always available; record escalation separately from social support. NGO response timers never determine emergency care or permission to refer. Use district-approved rules, not generated diagnoses.

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3. Need → public entitlement/service

Evidence status and possible break: P, local 04: government programmes have their own eligibility/agency pathways. **H:** an NGO substitutes for an accessible public service.

Existing owner or required local decision: Relevant facility, TB unit, AWW/ICDS, social-welfare office or implementing agency; coordinator follows access barrier.

Practical fix and beneficiary protection: Identify service and responsible public channel first; distinguish potentially eligible, applied, sanctioned and received. Track denial/delay/document barrier. Temporary charity must preserve the unresolved public-service issue.

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4. Request → beneficiary agreement

Evidence status and possible break: P S1: ethical engagement requires consent/confidentiality. **H:** family or worker assumes consent to sharing.

Existing owner or required local decision: Beneficiary; authorized caregiver participation where appropriate; trained worker records permitted decision.

Practical fix and beneficiary protection: Explain recipient, data and service in the preferred language. Support refusal, withdrawal and safe contact preferences. Do not make clinical visits/public benefits conditional on NGO consent. No forced patient app or OTP.

5. Registered organisation → operational service branch

Evidence status and possible break: H: legal listing/office distance is mistaken for service readiness.

Existing owner or required local decision: Participating NGO branch verifies offer; coordinator checks it with district-approved partner process.

Practical fix and beneficiary protection: Store branch, service, covered ward/block/hamlets, eligibility, fees, access needs, hours, contact and verified date. Organisation registration never establishes a funded local slot. Retain manual review for uncertainty.

6. Eligible match → accepted capacity

Evidence status and possible break: E S1: linkage delays/coordination problems occur within an existing support programme. **H:** analogous multi-service local offers become stale.

Existing owner or required local decision: Receiving NGO intake staff accept/decline; coordinator owns non-response and alternatives.

Practical fix and beneficiary protection: Explicit acceptance with expiry; no identifiable broadcast. Record full, outside catchment, ineligible, waitlisted and no response distinctly. Reserve one agreed allocation transactionally; release on cancellation.

7. Acceptance → feasible appointment/journey

Evidence status and possible break: E S3: travel, waits and wage loss were access barriers. **H:** a confirmed slot still cannot be reached.

Existing owner or required local decision: Facility/provider confirms appointment; coordinator arranges permitted logistics with recipient.

Practical fix and beneficiary protection: Confirm date, accessibility, documents, accompaniment, cost and return journey before travel. Combine appropriate tasks only with clinical/provider agreement. An NGO ride does not certify the destination can deliver care.

8. Funded promise → supplies and last-mile delivery

Evidence status and possible break: E S1: food-basket logistics/support quality gaps are acknowledged. **P S1:** DTO/district-supervised arrangements already apply to TB support.

Existing owner or required local decision: For TB use DTO/TU pathway; elsewhere name contracting NGO operations lead and approved coordinator.

Practical fix and beneficiary protection: Verify inventory/delivery responsibility and catchment; log short/failed delivery and replacement action. An approved request is not disbursement; a donor pledge is not inventory. Do not invent a general-purpose platform wallet.

9. Provider says delivered → recipient confirms

Evidence status and possible break: P S1: delivery validation is prescribed. **H:** mistaken delivery or unsafe verification is counted as success.

Existing owner or required local decision: Provider reports; recipient/authorized worker confirms; coordinator handles disputes.

Practical fix and beneficiary protection: Separate attempted, provider-reported, confirmed and disputed. Offer assisted confirmation without smartphone. Use proportionate private evidence; do not post patient photos, home locations or diagnoses. Production evidence rules must fit the programme rather than silently removing official requirements.

10. Facility discharge → local follow-up

Evidence status and possible break: E S2: reviewed pathways show downstream continuity gaps. **H:** NGO case closure hides outstanding health follow-up.

Existing owner or required local decision: Discharging facility supplies plan; authorized ANM/CHO/MO maps follow-up; ASHA performs assigned tasks.

Practical fix and beneficiary protection: Preserve separate clinical and support states. Return a permitted, actionable due task and owner to the primary team. Transport/food completion is never evidence that treatment or clinical risk is resolved.

11. Unresolved case → supervision, grievance and network repair

Evidence status and possible break: E S5: digital aids did not automatically improve supervision. **H:** dashboard exists but no one acts.

Existing owner or required local decision: Named coordinator, backup and supervisor; district authority resolves programme issues.

Practical fix and beneficiary protection: Queue with next action, due date, age and reason; human review of overdue cases and recipient complaints. Escalation windows are agreed per service. Close unresolved honestly; report empty capacity and refusals without punishing worker or beneficiary.

Ten-lens review matrix

Every feature/tradeoff/validation below is **H**, a design proposal. Frustrations are interview prompts supported by the indicated evidence where available, not quotations or universal claims.

ASHA

Frustration to test: "One more app, register and chasing task" (S2/S4); a remote family keeps slipping.

Practical feature: One short barrier card reusing permitted visit fields; worker-set availability; coordinator follow-up; unserved list.

Tradeoff or beneficiary risk: NGO capture can displace essential visits; automatic metrics may penalize terrain or refusal.

Field validation: Observe complete working days before/after; count extra entry, calls and escort minutes, missed due visits and unpaid work. Remove any parallel fields the district does not need.

Beneficiary / caregiver

Frustration to test: "Referral means another trip, lost wages, repeated story or unsafe disclosure" (S3).

Practical feature: Recipient-chosen contact/time; plain-language offer with costs/accessibility; paper/assisted status; refusal/withdrawal.

Tradeoff or beneficiary risk: An accessible slot may be farther away; reminders on shared phones can expose a private issue.

Field validation: Journey interview including declined/unreachable cases; measure journeys, waiting, expenditure and recipient-reported completion. Test with no smartphone.

ANM / CHO

Frustration to test: Social queues obscure clinical priority or return incomplete facility plans (S2).

Practical feature: Separate clinical task and support case; approved escalation route; joint-visit/resource constraints.

Tradeoff or beneficiary risk: Excessive alerts create another queue; an NGO cannot interpret clinical danger flags.

Field validation: Walk through urgent, routine and post-discharge cases with team; verify who receives/acts and which information is necessary.

NGO intake

Frustration to test: "We do not cover that hamlet or have those slots"; duplicate registration (H).

Practical feature: Branch-specific offers, limited intake fields, actual capacity, assisted accept/decline and reason.

Tradeoff or beneficiary risk: Frequent updates consume staff time; low volume partners may look unreliable in rankings.

Field validation: Shadow intake; compare declared and actual capacity; test ten sample offers across eligibility/catchment boundaries and measure update effort.

District / block

Frustration to test: Multiple queues, public-service failures and no funded follow-up owner (S1/S2).

Practical feature: Assign coordinator/backup and geography; overdue/unresolved queue; aggregated service gaps.

Tradeoff or beneficiary risk: Funding the coordinator is an operating cost; targets can encourage false closure.

Field validation: Agree a written role/time allocation; measure cases per coordinator, follow-up effort, unresolved age and existing review-meeting fit.

Scheme navigator

Frustration to test: "Eligible on paper, application stuck or camp not funded" (local 04; H locally).

Practical feature: Versioned rules, responsible agency, document checklist and distinct sanction/receipt states.

Tradeoff or beneficiary risk: Overconfident eligibility can cause wasted trips; more document uploads increase exposure.

Field validation: Have authorized local staff review synthetic and de-identified cases; sample approvals/denials and actual service calendars. Use "needs verification" when rules differ.

Donor / CSR

Frustration to test: "What can we fund and did the agreed support reach people?" (S1; H beyond TB).

Practical feature: Reviewed bounded service/project budget; aggregate fulfillment, reasons and reconciliation.

Tradeoff or beneficiary risk: Pressure for identifiable stories/photos; money may favor visible cases rather than service gaps.

Field validation: Test a permissible report with NGO finance/district staff; confirm restricted purpose and audit evidence. Measure funding decisions, not assumed access to historic CSR spend.

Privacy / safeguarding

Frustration to test: Recipient fears stigma or unsafe contact; volunteer has excessive access (S1).

Practical feature: Service-specific consent, role/assignment checks, minimum dispatch data, access log and revocation.

Tradeoff or beneficiary risk: More controls can slow assisted intake; diagnosis may be inferable from an NGO/category even when hidden.

Field validation: Threat-model shared phones, public maps, exports and small groups; check consent comprehension and withdrawal handling with recipients. No public diagnoses/household routes.

Offline engineer

Frustration to test: Worker has a plan but cannot sync; old capacity appears bookable (S2).

Practical feature: Cache authorized day plan; stable event IDs; visible last sync; pending/confirmed states; coordinator-assisted channel.

Tradeoff or beneficiary risk: Local storage creates device-loss exposure; offline acceptance cannot silently reserve live capacity.

Field validation: Flight-mode completion/reconnect duplicate test; simultaneous slot requests; consent withdrawal before sync; device loss/revocation exercise.

Independent evaluator

Frustration to test: Impressive counts mix visits, referrals, donations and help received (S5).

Practical feature: Cohort-based funnel with unresolved/refused states; auditable events and independent recipient sample.

Tradeoff or beneficiary risk: Confirmation calls can burden or expose households; excluding unreachable people biases success.

Field validation: Compare identical planning inputs; follow all referred cases for defined period; sample private confirmations; publish missingness and subgroup delays with disclosure protection.

Three service cases to validate

These are synthetic operating scenarios, not real beneficiaries, secured partners or promised benefits. No clinical thresholds, drug instructions or helpline numbers are included.

A. Rural pregnancy follow-up with a transport barrier

The health team has already marked a household due for follow-up. The optimizer includes the remote hamlet despite extra travel. At the visit, the person asks for help reaching the approved facility. ASHA records the barrier and permitted contact once; ANM/CHO handles any clinical concern through the existing route. Coordinator first checks the applicable public transport/service entitlement and actual booking channel, including JSSK where eligible; it is not a universal entitlement for every journey. If unavailable, the public-service failure stays open while an opted-in NGO is asked about a bridge ride.

The NGO office may be near the PHC but serve a different block, so it fails the catchment filter. Another offer needs an accessible vehicle, confirmed facility slot and return plan; its last confirmed capacity has expired, so coordinator reconfirms before promising the ride. If the person declines accompaniment, record the choice and offer permitted alternatives. **Validate:** completed facility attendance, public/NGO transport separately, actual household cost, essential visit coverage and ASHA/coordinator minutes. No NGO confirmation closes the medical follow-up.

B. Rural child referred for assessment/rehabilitation

An existing RBSK/clinical pathway identifies a child needing assessment. The caregiver can make one long journey but cannot afford repeated weekly travel or missed wages. The official service/DEIC confirms current eligibility, staffing, appointments and any onward pathway; the app must not copy an old condition list or fabricate a local rehabilitation provider. Coordinator checks public treatment, rehabilitation/device channels and authorized agency requirements before arranging NGO logistics.

A nearby "disability NGO" offers document navigation but no qualified therapy; it can accept that distinct request, not substitute for clinical assessment. For ADIP/RVY or camps, verify current authorization, appropriate assessment, local dates and funded device availability; portal presence is insufficient. A provider without home service or sustainable repeat capacity is not a complete solution. Caregiver has no smartphone: worker/coordinator provides an assisted status slip and private agreed follow-up. **Validate:** assessment attended, onward service initiated and sustained where required, journeys/wage loss, reasons for discontinuation, support refusal and extra coordination time. S3 supports investigating these barriers, not predicting this family's outcome.

C. Urban migrant household already under the TB programme

A consenting household in a vulnerable ward reports interrupted food support and has moved within the city. Care and support continue through the authorized NTEP/TB unit and Ni-kshay pathway; a new generic app does not duplicate TB registration. With permitted data, coordinator verifies current ward/contact, DTO/TU assignment, Ni-kshay Mitra linkage and actual basket delivery. Nutrition-payment state and food-support state remain separate. For other household needs, existing ration/ICDS/social-welfare channels are checked before supplementary NGO help.

S1 already supplies district-supervised responsibilities; reuse them. A local Mitra/NGO confirms whether the new ward is covered and whether the upcoming delivery has stock. An old donation pledge is insufficient. A public case map, diagnosis-bearing SMS or photograph publicity is unacceptable; offer a recipient-chosen discreet delivery/contact method. If support is declined or consent withdrawn, stop the voluntary support contact as agreed while preserving lawful programme care. **Validate:** authorized transfer/linkage acknowledged, confirmed delivery/quality, unresolved rounds, recipient privacy experience and coordinator effort. Do not interpret a delivered basket as treatment completion.

Minimal operating agreement and pilot decision

Before opening intake, the district/implementation partner and participating providers should agree: **who coordinates and funds the work; which households/catchments are included; who approves clinical/entitlement decisions; which services are actually accepting cases; update/response arrangements; minimal data/consent; backup and grievance route; and permissible completion evidence**. This is an H pilot recommendation, not a newly asserted statutory requirement. For TB, align with the established DTO/TU structure rather than substituting a private coordinator for programme authority.

A service record should include `branch`, `service`, `covered_localities`, `eligibility_version`, `cost`, `accessibility`, `intake_hours`, `contact_channel`, `capacity_quantity`, `capacity_confirmed_at`, `valid_until`, `accepting_intake`, `responsible_person`. A need needs `consent_scope`, `safe_contact`, `case_owner`, `backup`, `next_action_at`, `public_pathway_status` and separate support/clinical status. "Unknown" is an explicit state. Match by these service facts before distance; a nation-level registration total never supplies local capacity.

Start with one willing primary-care team and a few locally confirmed offers; the exact size is a proposal to fit local coordination resources. First trace recent successful, failed and refused cases across all eleven handoffs. Then run the planner/support workflow for a defined cohort and compare with the existing process. **Burden neutrality is an acceptance criterion to test**, not a claim: no net increase in ASHA administrative/chasing time and no reduction in essential due coverage. Coordinator/provider effort is measured separately rather than hidden as free labour.

Keep denominators honest: eligible due visits, actually attempted/completed visits, unique consented support cases, offers, accepted slots, reported delivery, recipient-confirmed delivery and unresolved/withdrawn cases. Report time to resolution and reasons for failure, including no capacity, inaccessible travel, public entitlement delay and refusal. Include remote/migrant/no-smartphone households and disclose missing confirmation rather than excluding them. Measure recipient cost, privacy incidents and complaints. These operational results cannot establish causal health benefits without a suitable evaluation.

The best first demonstration is a **failed match that is visibly recovered by its owner**, while a remote due visit stays protected. If a paper/phone pilot cannot identify real capacity and an accountable receiving person, a wider digital directory should wait.

Part 10. Proposed solution and operating model

ASHA Saath: proposed solution and operating model

Working name only; brand uniqueness has not been researched. Research date: 8 October 2026. All architecture, examples, targets and budgets below are proposals, not deployed capabilities or measured impact.

Recommendation

Build a visit planner with a small support-referral layer: **help the worker decide whom to visit today, then track whether a need discovered during that visit reaches an eligible, available provider and is actually addressed.** Keep government services, clinical care and NGO support as distinguishable pathways within the same need record.

The biggest implementation constraint is partner operations. Software cannot create transport, nutrition supplies, qualified staff or unrestricted funds. The minimum viable network is a willing local coordinator and a few providers with specific, current service offers. A national catalogue can help discovery later; national participation does not mean every NGO serves every village.

This is an adaptation hypothesis, not a first-ever invention. Indian frontline tools already support workplans and offline capture; overseas social-care platforms already document referral outcome tracking. The useful combination to validate is ASHA visit planning, Indian entitlement navigation, actual local service catchments, freshness of capacity, and follow-through with very little additional worker entry. See the platform catalogue and primary product documentation for the comparison.

Stay inside Healthcare 03

The original problem is **Community Health Worker (ASHA/ANM) Visit Scheduling Optimizer with Risk-Weighted Prioritization**. Its expanded requirements, read earlier in this session, cover caseload CSV ingestion, geography-based daily routes, rule-based priority, travel estimates, risk flags and coverage projection. Current extraction reconfirmed the title but did not expand the description. Official problem statements.

The NGO feature should start **after a visit** and feed relevant follow-up tasks back into the planner. A stand-alone donation website would leave the required scheduling problem unsolved. Do not add NGO financing urgency to a clinical risk score: medical priority and support-access urgency are separate fields with separate owners.

Four journeys that make the idea concrete

1. **Pregnancy follow-up and access barrier:** an ASHA visits a person already marked for priority follow-up by the health team. The person reports a transport or documentation problem. Show the applicable public-service pathway and local contact first. If the public service cannot be accessed in time, record the barrier and ask an opted-in provider whether a permitted bridge service is available. An NGO referral does not close the original medical task. A public-service failure remains visible to the supervisor even if temporary help is arranged.

2. **Child nutrition support:** route the assessment and programme follow-up to the authorized health/Anganwadi team. With consent, a non-clinical family-support need may be offered to a provider with suitable local services. Dietary treatment and clinical decisions remain with qualified staff; a food parcel is not proof that a clinical condition has resolved.

3. **Assistive-device access:** the worker identifies that an older or disabled person wants help navigating an assessment, certificate, device camp or transport. Match an eligibility navigator and a provider's actual programme catchment. Record separate milestones for assessment, application and device delivery; a successful form submission is not device receipt.

4. **TB-related social support:** only use an authorized and consented pathway aligned with the programme. Nutrition or livelihood assistance may complement treatment services. Keep sensitive identity and diagnosis out of donor listings. Support delivered is not treatment completion. Programme and beneficiary overlap with existing TB systems must be reconciled in a real pilot.

These are hypothetical journeys. No named beneficiary, local partnership or current service availability is asserted.

Network design from village to national level

Level	Who operates it	Data and decision	Boundary
Household / village / urban ward	ASHA and authorized health team	Today's visits, consented need, preferred contact and constraints	Worker sees assigned cases; household identity stays restricted
PHC / urban primary health team	Supervisor or designated coordinator	Review priority, handle failed service access, follow unresolved needs	Clinical escalation stays in the health chain
Block / municipality / district	Named network coordinator with participating NGOs	Validate offers, maintain service catchments, contact non-responsive providers	Operational accountability must be agreed, not assumed
State	Authorized programme administrators / implementation partner	Maintain state-specific programme rules and aggregate capacity gaps	State eligibility and rollout cannot be copied from another state
National	Discovery catalogue and approved programme/funder interfaces	Search registered organizations and national programmes	Provider office location is not proof of nationwide delivery

Geography belongs to a service offer, not just to an organization. A Jaipur-based NGO might serve two rural blocks for nutrition, provide counselling remotely in Rajasthan, and run a device camp only on one date. Those are three different offers. An organization marked "national" must still declare the catchment and eligibility of each offer.

Urban matching must handle wards, informal settlements, temporary addresses, migration and contact safety. Rural matching must handle hamlets, travel time, roads, outreach days and last-mile delivery. PIN codes help discovery but are often too coarse to establish eligibility or a feasible route. Keep geography configurable and validate it locally.

Minimum information for a service offer

• Organization and branch identifiers; authorized service contact; legal identity evidence and review date.

• Service category and plain-language description; clinical/non-clinical boundary; who may deliver it.

• Catchment expressed as selected administrative units and, where needed, boundaries; delivery mode and accessibility.

• Eligibility conditions, exclusions, required documents and rule source/version.

• Opening times, intake process, response expectation and whether the offer is open, waitlisted or paused.

• Capacity quantity and unit (for example appointments this week, not an unspecified number); last confirmation and expiry.

• Cost, grant restrictions and whether assistance is free, partly funded or requires approval.

• Outcome states and allowed delivery evidence; grievance contact and re-verification date.

Registration, service readiness and funding authorization are three separate checks. NGO Darpan supplies a registration ID, with an explicit disclaimer that the ID is not an endorsement. A "verified" badge should explain exactly which checks were performed rather than imply government approval. NPO Darpan.

Need capture that does not burden the worker

Proposed first version: after marking a visit, the worker selects a support category, confirms that the person wants assistance, notes locality and the preferred safe contact method, and chooses the observed access barrier. Reuse authorized data already in the caseload rather than asking for another full registration form. Free text is optional. Voice entry may produce a draft the worker must confirm.

Capture only what is needed for the requested service. Full clinical history, Aadhaar scans, family photographs and diagnoses are not default NGO intake fields. A case can be handled with an internal identifier; the provider receives only

the fields authorized for that referral. A person without a smartphone or OTP capability must have an assisted pathway. Missing identity documents should create a navigation task where possible rather than an automatic permanent rejection.

Separate refusal from unreachable status. Record "declined support" respectfully and stop repeated contact according to the person's preference. Household health consent is not blanket consent for every NGO, donor or future service.

Referral states and accountability

Suggested states:

```
draft -> consent recorded -> eligible offer selected -> offered -> accepted -> appointment/service arranged -> provider reports delivered -> beneficiary/worker confirmation -> closed
```

Side branches must support `ineligible`, `declined by provider`, `expired/no response`, `waitlisted`, `person declined`, `cancelled`, `consent withdrawn`, `delivery disputed` and `closed unresolved`.

"Offered" means a provider was asked; "accepted" means the provider agreed; "delivered" means the service was reported as supplied; "confirmed" means the agreed verification was recorded. Reports show each stage separately. Closing unresolved preserves the failure reason; it must not be counted as a successful resolution.

Each case has a named follow-up owner and a next action date. The implementation partner must budget and assign coordinator time; this responsibility must not silently become unpaid ASHA work. A district coordinator can review provider non-response and offer alternatives with appropriate permission. Escalation intervals depend on the service and are locally agreed; they are not universal clinical deadlines. Urgent clinical situations use the established health referral process immediately and do not wait for an NGO timeout timer.

In a real deployment, one service request can generate several offers, but only the agreed active allocation reserves a limited slot. Use transaction-safe reservations with expiry; release the reservation on cancellation. Avoid broadcasting identifiable cases to every NGO. Administrative duplicates can be reviewed without denying legitimate repeat needs.

Matching that can be explained

First apply hard filters: service fit, eligibility known or requiring review, verified provider readiness, true catchment, accessible delivery modality, open intake, valid capacity and beneficiary consent. Uncertainty goes to manual review. Do not manufacture an eligible match when no offer passes.

Then rank the remaining candidates using understandable factors: earliest feasible service, recipient's preference, travel/access burden, freshness of capacity, response history with enough sample context, and the requested support deadline. Show a short explanation: "serves this ward; document help available Tuesday; capacity confirmed today." A new NGO should not be permanently excluded because it has little platform history. Paying for promotion must not control vulnerable households' service ranking.

Clinical risk and affordability are not monetized lead-generation scores. Each suggested service has an accept/decline decision by the responsible human. Government eligibility is "potentially eligible" until the responsible authority confirms it; the application or benefit approval is not automated by an LLM.

Visit optimization

Use staff-supplied or approved rule-based priority, due dates, visit durations, household availability, location confidence, shift length and start/end location. Preserve the PS's geographic step: use basic geographic grouping or the stated nearest-neighbour approach, then generate a priority-aware route within the shift. Clearly name the implemented method in the demo. Advanced clustering, search or OR-Tools can come later if the team has time.

Optimize coverage subject to feasibility, then reduce unnecessary travel within the accepted coverage plan. A remote priority visit may legitimately increase travel. Show the tradeoff and all unserved visits, with reasons: shift full, location missing, household unavailable or requires supervisor review. Coordinate confidence and route assumptions must be visible. Straight-line distance multiplied by an assumed speed is an estimate, not a road ETA.

Repeatedly missed or remote households need an explicit coverage safeguard. Test a maximum permissible postponement policy approved by the supervisor rather than allowing a distance-first algorithm to keep deferring them.

Do not claim a mathematically optimal route unless the algorithm and solver status support that claim.

Support follow-up tasks consume worker/coordinator time too. A proposed NGO appointment should not silently fill an already infeasible ASHA day. Multi-worker reallocation requires authorization and realistic travel areas, rather than assuming any worker can cover any household.

Government service access before substituting donations

The scheme catalogue informs an entitlement checklist. Its minimal record contains programme, state applicability, beneficiary rules, support type, responsible facility/department, application or service channel, evidence link, effective date and last review. Include public services that need no conventional "application," not just cash transfers.

"Available on a website" is not equivalent to "accessible to this household." Record the actual barrier: unaware of benefit, eligibility unclear, documents missing, application pending, facility unreachable, stock unavailable, no response or person chose another option. An NGO bridge may help but should not erase the public-service gap. A scheduled review keeps rules from becoming stale.

Do not automatically merge related scheme amounts. For example, public delivery benefits, maternity transfers and nutrition programmes have different rules; one person may access several while still being only one beneficiary. Programmes/campaigns and entitlements are classified distinctly in the catalogue.

Funding and in-kind support

The first useful funding unit is a **bounded service offer or reviewed need**, not a public feed of identifiable medical stories. A provider can declare in-kind transport slots, document-navigation hours, nutrition-support inventory or a restricted project budget. A request remains pending until the authorized provider or funder approves it.

Proposed funding workflow: need and permitted support specified -> entitlement/access check -> authorized NGO cost review -> eligible restricted budget check -> funder/NGO approval -> external authorized disbursement or in-kind service -> receipt/service record -> permitted beneficiary confirmation -> reconciliation.

In the hackathon, demonstrate the request/approval/receipt states with synthetic amounts and simulated decisions. Do not implement a wallet, hold donations, infer unrestricted capacity from historic CSR expenditure, or promise a grant. Actual legal, financial, CSR, FCRA and tax eligibility varies by entity and funding source and needs partner-specific verification before money movement. The official CSR Exchange already connects eligible implementing agencies and corporates; project acceptance and funding remain their decisions. CSR Exchange FAQ.

For national/state funders, aggregated unresolved needs can help target a district programme. Suppress small groups and identifying combinations in donor reporting. The default donor view has service categories, catchment, capacity requested and aggregated fulfillment, not named patient routes.

Roles, privacy and offline behavior

Role	Proposed access
ASHA	Assigned visits, minimal support capture and status of assigned follow-ups
ANM / CHO / authorized clinician	Relevant clinical information and clinical priority/referral decisions
Supervisor / coordinator	Assigned geography and unresolved queue; provider operations where authorized
NGO service operator	Only referrals offered/accepted to their service and the permitted intake details
Funder	Approved project requests and aggregate outcomes; no automatic patient record access
Beneficiary / caregiver	Assisted or self-service view of their requested help, consent and grievance channel
Public user	General service information, with no identifiable household cases

Use role and assignment checks on the server, access logs, secure transport/storage, device lock/session controls, permission revocation and a retention policy appropriate to each record. A public map must not expose patient homes

or diagnoses. A working prototype's login screen is not a production security assessment.

India's DPDP commencement notification specifies staged commencement: some provisions on publication, others after one year and many substantive provisions after eighteen months. Do not label the entire Act fully effective on the report date. The exact notified text and any subsequent amendments must guide deployment review. Privacy controls above are product design choices regardless of that staged timeline. Official commencement notification, G.S.R. 843(E).

Offline first version: download an authorized day's plan; keep minimal permitted records locally; queue visit completion and draft needs. Display last sync time. A cached NGO offer is not a live reservation. Server-side capacity allocation and provider acceptance require connectivity or an explicitly logged coordinator-assisted channel. Reconnection uses stable event IDs, duplicate-safe writes, and a conflict state for withdrawn consent or changed allocations. Device loss, access revocation and persistence need explicit production design.

Practical architecture

One mobile-friendly worker view, one NGO intake view and one coordinator view can share a single backend and database for the hackathon. No microservice fleet is needed. Suggested entities are `WorkerAssignment`, `VisitTask`, `HouseholdRef`, `ConsentEvent`, `NeedCase`, `OrganizationBranch`, `ServiceOffer`, `EligibilityRuleVersion`, `ReferralOffer`, `CapacityReservation`, `DeliveryEvent` and `AuditEvent`. Restricted support requests add approval and reconciliation records later.

Maintain a local planning module, a versioned scheme catalogue, deterministic service matching and an explicit referral state machine. Server transactions prevent double-booked capacity. A light scheduled job marks offer/reservation expiry and creates coordinator follow-ups. Interfaces can expose an export/import adapter before any authenticated national-platform integration exists.

CSV import is the authorized MVP input in the problem statement. Live RCH, U-WIN, ABDM, NGO Darpan or government identity integrations must not be presented as connected unless approved access and working interfaces are demonstrated. FHIR compatibility is an architecture intention until implemented and tested; it is not permission to access records.

An LLM can draft a Hindi summary from permitted text or explain an approved scheme excerpt with its source date. It should not assign clinical priority, determine final benefit eligibility, invent NGOs, approve funding or certify delivery. The demonstration can succeed without an LLM.

24-hour build order

Proposed elapsed time	Concrete output
0-3 hours	Lock data fields and states; seed synthetic caseload and service offers; build three simple views
3-8 hours	CSV validation, priority list, daily route, shift feasibility, unserved queue and baseline comparison
8-14 hours	Consented need card, service eligibility/catchment filter, explanation, NGO accept/decline and outcome states
14-18 hours	Replan one blocked/unavailable visit; demonstrate rejected service and alternative; coordinator queue
18-21 hours	Cache the daily plan and queue completion if core works; permission checks and focused edge-case verification
21-24 hours	Demo rehearsal, evidence labels, pitch and restore/check seed data

This is a proposed schedule, not an elapsed-work claim. The authoritative MVP uses **one document-navigation support category**, three fictional offers and one reviewed public-service pathway. Additional transport and family-support categories are pilot extensions after this flow works. Retain the broader catalogue as research. The report's 24-programme coverage does not mean all 24 workflows belong in a 24-hour implementation.

MVP essentials

1. All required PS outputs: validated caseload, an explicit geography-based grouping/nearest-neighbour step, risk/due list, feasible route, travel estimate, flags and coverage projection.

2. One consented visit-to-support path, with actual hard filters on synthetic service data.

3. Provider accept, decline and delivered/confirmed outcomes, with unresolved reasons.

4. Supervisor override and coordinator handling of a failed offer.

5. A clear account of source dates and synthetic inputs in the demo.

Build later

Broader scheme eligibility, restricted budget requests, multiple workers, advanced road routing, voice interfaces, patient-facing accounts, provider integrations, referrals across districts and funder dashboards. Every extra feature has an operations and privacy cost. Defer autonomous diagnosis, open public patient feeds, donation custody and universal automatic matching.

Demo that shows a real workflow

Use **20-30 visibly synthetic visit tasks** in one fictional catchment, a declared shift, fixed visit durations and three fictional provider offers for one document-navigation service. This is the authoritative MVP dataset; 60-80 anonymous households are an optional larger stress scenario only after the core works. Mark provider identities as fictional or explicitly non-partner example organizations. Show nearest-first, priority-first and your chosen feasible planner with identical assumptions. Then:

1. Display a remote due/priority household that requires an explicit travel tradeoff.

2. Make a household unavailable, replan and expose remaining unserved work.

3. During a completed visit create a consented document-navigation need.

4. Exclude an NGO whose office is nearby but whose service does not cover the household's ward/block.

5. Offer the eligible service; provider declines because capacity has expired/full status.

6. With the person's permission, select a valid alternative; provider accepts and later reports service delivered.

7. Record beneficiary/worker confirmation and show the other unresolved cases still in the queue.

If offline completion is implemented, turn off network, capture a visit event, restore network and verify exactly one completion. Funding is an optional reviewed request, not the headline live action. Service outcomes are simulated; the demo does not prove improved health.

Metrics and evidence needed

Metric	Definition and interpretation
Planned due/priority coverage	Included eligible due/priority tasks divided by all such tasks in the same planning input; disclose exclusions
Unserved backlog	Tasks outside the feasible shift, with reason and age; do not hide hard cases
Travel and shift feasibility	Same declared travel model and visit durations across baselines; estimate until field measured
Offer acceptance	Accepted offers / valid offers made; distinguish providers and unique need cases
Delivered support	Need cases with service-specific delivered event and the agreed confirmation, over a defined cohort/window
Unresolved support	Reasons including capacity, eligibility, refusal, no response, access or disagreement
Extra worker time	Time to capture a need and handle follow-up, measured with practitioners rather than assumed
Remote/urban-migrant access	Coverage and delay by agreed geographic/access categories; safeguard privacy and small groups
Capacity freshness	Active offers confirmed within the local validity period; stale listings are separate

Do not report projected coverage as actual completed visits, an accepted referral as received help, or delivered food/transport as a life saved. Demo improvements are simulation results; even good field operational outcomes require a separate study before claiming causal clinical impact.

Field validation and adoption

Start with one willing PHC/urban health team, a named coordinator and three to five willing service providers. This is a proposed pilot size, not secured access. Interview ASHAs, a supervisor, beneficiaries/caregivers and NGO intake staff using recent cases. Ask where a case currently goes, who follows it, what data is already collected, how referrals are accepted, what service restrictions exist and why prior attempts failed.

Document the existing WhatsApp/phone/register workflow before replacing it. Offer assisted NGO intake and consolidate reporting so a partner is not forced to maintain another full system. Test a paper/coordinator version of the service graph before assuming software adoption. If local partners already use a capable referral tool, explore a visit-planning add-on or integration rather than replacing their platform.

Proceed only if the network has real offers, an accountable follow-up owner, beneficiary agreement and authorized data handling. Pilot in one rural catchment and one urban catchment when operationally feasible; they are different experiments. An indicative 4-8 week learning period and a 6-12 week rollout are planning assumptions, not promises. Local approvals and procurement can extend timelines.

Economics without invented market size

ASHAs and households should not be assumed to pay a personal subscription. Potential institutional buyers are a health implementation partner, district programme, NGO consortium or CSR-funded approved project. Willingness to pay and procurement routes are unvalidated. The costs to test are coordinator time, offer verification, worker training, beneficiary follow-up, support, secure hosting and integration. A cheap database does not remove those costs.

CSR reported expenditure is context, not an accessible pool for this app. A February 2026 official answer reports INR 34,908.75 crore for FY 2023-24 and states CSR decisions are board-driven. This does not establish available 2026 funding or what share would fund ASHA coordination. PIB/MCA, 10 February 2026.

Build a pilot budget from quantities after interviews: coordinator hours x local rate; provider verification visits x actual travel cost; onboarding/training sessions x cost; hosting/messages x use; and support/evaluation time. Financial estimates without local rates should remain clearly labelled placeholders. There is no defensible revenue forecast from ASHA headcount, ABHA registrations or Darpan registrations alone.

Pitch

"ASHA ghar tak pahunchti hai. Visit ke baad agar kisi family ko documents, transport ya permitted support ki zaroorat ho, us need ka follow-up aksar alag process mein chala jata hai. ASHA Saath daily visits plan karta hai, government service aur participating NGO ki actual local availability dikhata hai, aur case ko acceptance se delivery confirmation tak track karta hai. Jahan help nahi mili, woh bhi supervisor ko clear dikhta hai."

Treat "follow-up often separate" as the proposed problem to verify locally; do not claim every district loses every case. For mentors, show one complete journey, the original route requirements, the failed-offer recovery and honest simulation metrics. That is a stronger demonstration than a long list of unimplemented promises.

Part 11. MVP architecture and acceptance criteria

Healthcare 03: implementable 24-hour architecture and MVP

Prepared 8 October 2026. **Report only: no application, live integration, partner participation, funding or measured impact is claimed.** All algorithms, fields, states and build estimates below are our design proposals. Evidence comes from the companion research files and their primary-source URLs; operational assumptions still need local validation.

Product decision

Build **a feasible daily visit planner, followed by one consented referral for nonclinical entitlement-document navigation**. The winning interaction is: import a synthetic caseload, explain which visits are due, show a usable worker day and every unserved task, record an access barrier during a completed visit, obtain permission, secure a participating provider's acceptance, and distinguish reported help from confirmed help.

Keep the original Healthcare 03 visibly central. The official listing confirms the title, "Community Health Worker (ASHA/ANM) Visit Scheduling Optimizer with Risk-Weighted Prioritization." The detailed CSV, geography, daily route, rule-based risk, travel estimate, risk flag and coverage requirements are retained from the parent session's earlier expanded-panel reading; current extraction did not independently recover the expanded text. Save that panel before claiming an exact quotation. Official problem statements.

This is a companion workflow hypothesis, not a first-ever product category. The research catalogues **24 schemes/programmes and 30 platforms/services**; those are catalogue entries, not unique companies, households, markets or independent entitlements. ANMOL, CommCare and others already document worklists, prioritization and offline collection. Ni-kshay already provides a consented, district-coordinated community-support precedent. Shaishav was officially described in August 2026 with at-risk line-listing, visit tracking and referral follow-up; Poshan Tracker documents an AWW home-visit scheduler introduced in April 2026. Their documented scope does not establish universal local implementation, but it rules out a claim that scheduling and referral follow-up are new inventions. ANMOL manual, CommCare Precision Tasking, Ni-kshay support guidance, Shaishav workshop, Poshan Tracker backgrounder.

Bound the demo

Use 20-30 visibly synthetic visit tasks in one fictional catchment, two preassigned staff members, one planning date, a configurable daily time budget, and three fictional provider offers for the same document-navigation service. Label every screen **Synthetic demonstration - no live government or NGO connection**. Avoid realistic beneficiary identities, real household pins, Aadhaar numbers and named NGOs presented as participating partners.

Three views share the same application:

• **Worker:** CSV validation, today's route/list, priority explanations, unserved queue, visit completion and a short support-need form.

• **Coordinator:** approved rule versions, assignment review, coverage projection, unmatched/expired/disputed referrals and named follow-up ownership.

• **Provider:** only that provider's offered/accepted cases, acceptance/decline, appointment and service reporting.

The referral service is assistance understanding a public-programme document checklist and preparing the next application step. It supplies navigation; it does not issue a certificate, approve a benefit or guarantee payment. For the demo, seed one source-linked checklist from the scheme research and a synthetic partner offer; do not build all 24 programme rules. A "potentially applicable public pathway" card is separate from final authority-confirmed eligibility. Government services remain an explicit option and actual access barriers remain visible.

Caseload and rule contract

The extended demo CSV contract accepts `patient_id`, `location`, `staff_id`, `rule_id`, `priority` and an immutable `source_task_id` (a documented `task_id` alias is acceptable). The importer also binds a stable source/dataset identifier. These identity fields are required for repeat imports; missing identity goes to review rather than deduplication by patient alone. They extend the original PS input instead of being claimed as its exact required columns. A location must resolve to explicit coordinates or a manually confirmed locality reference; a text address alone is not silently converted into a precise home. Recommended columns are `latitude`, `longitude`, `location_confidence`, `locality_id`, `due_date`, `earliest_time`, `latest_time`, `visit_minutes`, `required_cadre`, `last_visit_date`, `defer_count`, and `risk_flags`. A worker/day configuration supplies start/end coordinates, mode, shift start/end, fixed commitments and buffer. Demonstration durations and speeds are labelled assumptions, editable by the coordinator.

Validate before publishing an import: missing/duplicate task IDs, conflicting staff assignments, unknown rules, invalid coordinates, invalid dates, negative durations, unsupported priority values and unresolved location. Show accepted rows and row-level errors. Use `(source, source_task_id)` as the import identity so repeating the same import updates through an explicit preview instead of creating duplicate visits. A patient can legitimately have multiple distinct tasks; do not deduplicate solely on patient ID or name. Do not manufacture clinical data from missing fields.

Rules are versioned configuration: `rule_id`, programme, source URL/date, effective date if known, district approval reference/status, due window, required cadre, priority bucket, supplied flag explanations, escalation path and review date. CSV priority may be used only when supplied by an approved authority or clearly marked synthetic. If priority conflicts with the approved rule version, put the row in review and display the discrepancy; no hidden override.

SSBSK was launched in June 2026 and changes the contemporary child-health context. The official 2026 schedule was not independently retrieved in the ground-truth research. Do not hard-code legacy HBNC/HBYC dates as the current SSBSK standard. Demonstrate generic tasks or visibly synthetic rules until the official table and district instructions are approved. SSBSK launch, official guideline catalogue.

Deterministic planning, with clinical boundaries

The software schedules **already classified tasks**. It neither diagnoses disease nor generates clinical risk from symptoms, photographs or an LLM. Imported risk flags show their source, owner and date. Unknown or contradictory classification is a review task. Incentive amounts, donor interest and fundraising urgency never alter clinical priority.

An urgent flag from an approved pathway creates **clinical escalation required** before routine route generation. Display the locally configured escalation contact/instruction and record acknowledgment and handoff. The worker can also invoke that pathway when a finding warrants escalation under her training. The planner cannot declare the situation safe, replace assessment or wait for NGO acceptance. The urgent task remains visible until an authorized health-team disposition; it is excluded from routine-route coverage with a separate escalation count, rather than shown as a successfully completed visit. ASHA referral and trained cadre boundaries are grounded in NHM/NHSRC guidance. NHM ASHA role, NHSRC Community Processes.

Proposed planning procedure:

1. Freeze an input snapshot and policy version for a selected staff member/date. Respect existing staff assignment and required cadre. Split tasks into escalation, review, eligible due, future and completed groups. Joint visits are deferred in the MVP: a task marked joint-required is visibly `coordination_required` and cannot be counted as independently completed by one cadre.

2. Perform required **geography-based grouping by confirmed locality**: tasks sharing a confirmed village/hamlet/ward locality ID form a group; tasks without a confirmed locality retain individual ungrouped markers rather than an invented common catchment. Retain group membership in planning output and show group-level coverage. Within an otherwise identical priority/deadline/deferral/due-date tier, construct a deterministic nearest-neighbour seed order from the staff start coordinate, repeatedly choosing the closest remaining task and using task ID to break distance ties. Geographic grouping and this route seed are explicit MVP outputs, not optional enhancements.

3. Order eligible due tasks by approved priority bucket, then approved latest permissible date, then deferral count descending, due date, and the nearest-neighbour seed order only within those ties. Geography cannot override those approved service/fairness fields. A supervisor-configured maximum postponement policy raises a visible mandatory-review constraint when exceeded; do not invent a universal medical interval or change clinical priority merely

because a case is remote. Display groups and distance tie-breaking cannot justify repeatedly deferring a remote household without this visible review.

4. Insert tasks one at a time in that deterministic order. For each task, evaluate every insertion position in the current route. Recompute travel, arrival, waiting, visit duration, fixed commitments, breaks, buffer and return-to-end time. Choose the feasible insertion with the lowest incremental travel; tie-break on earliest arrival, then insertion position. If none is feasible, retain a named unserved reason.

5. Optionally apply a deterministic travel-reduction pass only to the selected visit set, preserving priority coverage, availability and commitments. This is a heuristic feasible plan, not a mathematical optimum.

6. Persist route version, selected IDs, timings, assumptions and all rejected/review IDs. Manual edits trigger the same feasibility check. A coordinator may authorize an override with a reason; it does not erase a warning or make an impossible shift feasible.

The primary objective is approved priority and due-service coverage within a usable day; travel is a secondary choice within that coverage plan. A remote priority visit may increase kilometres and reduce total visit count. Show that tradeoff. If a high-priority task cannot fit even alone, give it a specific escalation to supervisor review; do not silently fill the day with easier tasks and claim all needs covered.

For a first build, compute a deterministic pairwise distance estimate using coordinates and a declared mode/speed assumption, optionally a separately calibrated detour factor. Label it **estimated travel from coordinates, not a road ETA**. Add service time, return travel and waiting to the time budget. Missing coordinates, a blocked road reported by the worker, an unavailable household or uncertain access require manual review or explicit correction. A haversine estimate cannot prove a traversable route. Show a confidence badge and last location confirmation. Road matrices and traffic can come later after access, cost and routing quality are evaluated.

Coverage and fairness output

Show due eligible visits covered/total, bucket-level coverage, planned travel/service/wait/buffer minutes, planned end time and all unserved cases with reason and next owner. Separately list escalations, review cases, joint-required cases and future tasks so denominator changes cannot hide them. Show remote/locality coverage and consecutive deferrals. Avoid a public household map or a punitive worker GPS score.

For the required **coverage projection**, rerun the same scheduler across the next three configured working days on the frozen task backlog, removing simulated scheduled tasks from subsequent simulated days and advancing the **planning date**, while preserving each task's approved due date/window. An authorized source update may change a due date through a separate reviewed event; the projection does not move deadlines. Respect each day's known availability. State assumptions: no new cases, no cancellations, the same travel model and supplied future household windows. Unknown future availability is marked unconfirmed; remaining unscheduled tasks remain visible. This is forecast coverage under assumptions, not a promise of visits or a health outcome.

One NGO flow: consented document-navigation assistance

After completing a visit, the worker chooses `document_navigation`, records the actual access barrier and public pathway already tried, confirms that assistance is wanted, and selects a safe contact preference. Reuse internal case/locality references; do not copy full health records. The new support task has its own coordinator owner and next-action date. The health task remains governed by its own completion/clinical state.

A **service offer** belongs to a branch and a specific service, not to the organisation's national address. It declares confirmed villages/wards or administrative IDs, delivery mode/accessibility, eligibility and exclusions, source/version, opening times, response deadline, cost/free status, grant restriction if any, current capacity unit/window, confirmation timestamp, expiry and responsible operator. A registered NGO is not automatically a qualified or available provider. Darpan's ID explicitly does not confer government endorsement. NPO Darpan notice.

Matching uses hard filters first: service category, actual catchment, readiness check still valid, eligibility confirmed for the navigation service, accessible modality, open intake window, unexpired capacity and permitted consent purpose. Unknown eligibility is `manual_review`; stale capacity is `capacity_reconfirmation_required`; no eligible offer yields `unmatched`, never an invented match. Eligibility to receive document help is distinct from eligibility for the underlying government benefit.

Rank only passing offers by earliest feasible appointment, beneficiary preference, accessibility/travel burden, capacity freshness, then stable offer ID. Explain each match in plain language. A newly participating provider need not have a response-history score; defer reputation-based ranking until there is meaningful, audited data. No paid promotion or donor appeal score influences matching.

For the demonstration, offer one case to one provider at a time. The case may have sequential attempts when an offer is declined or expires. The case owner arranges a fallback and records all attempts. Later parallel offers would still require exactly one authorized allocation for the same service request.

Internal matching can occur after agreement to seek assistance, without disclosing the case externally. Before each `offered` transition, show the selected recipient, service purpose, permitted intake fields and safe-contact step; record recipient-specific permission or verify that the current consent explicitly covers that recipient. Changing providers requires that check again. Acceptance cannot enlarge the consented field set.

State machine and permission to move state

The need and individual offer attempts are separate records; a failed attempt does not erase the need.

State	Allowed next action and owner	What the state proves
`draft`	Worker records category, barrier and owner; person may decline	A need draft exists; no sharing
`consent_pending`	Assisted or direct service-specific permission recorded; refusal closes as `person_declined`	Permission has not yet been granted
`ready_for_match`	Coordinator/system evaluates current offers and eligibility	Permission permits the selected purpose; no provider commitment
`manual_review` / `unmatched`	Coordinator resolves uncertainty or confirms another offer, with current permission	No valid allocation; remains on unresolved dashboard
`offered`	Identified provider accepts/declines before deadline; coordinator may cancel; system may expire	Provider has been asked, not booked
`accepted`	Provider reserves capacity atomically and sets appointment; person may decline; coordinator can cancel	Provider agreed to the bounded service
`arranged`	Provider records appointment/access plan; missed attempt creates follow-up	A service arrangement, not delivery
`delivery_reported`	Provider records completed navigation session and next-step outcome; authorized worker/person confirms or disputes	Provider says the session occurred
`confirmed`	Worker/person records agreed proportionate confirmation; coordinator closes	Confirmation of document-help session only
`closed_resolved`	Terminal; correction requires auditable reopening	This support service was confirmed, not benefit approval or improved health
`delivery_disputed`	Coordinator investigates, then resolves or closes unresolved	Conflict remains visible; not counted as confirmed
`closed_unresolved`	Coordinator records reason and remaining public-service barrier	Closure without successful support

State	Allowed next action and owner	What the state proves
`consent_withdrawn` / `cancelled` / `person_declined`	Stop new sharing/arrangements, release allocation, record contact preference	No further permitted active referral under that consent

Offer attempts have their own terminal `provider_declined`, `expired` and `cancelled` states. A waitlist is an explicit follow-up branch with consent, owner and review date; it cannot be displayed as accepted capacity. Decline reasons use bounded categories, with optional minimized text. Recontact after refusal follows the person's preference; do not equate refusal with unreachable status.

Use separate `service_outcome` values such as `checklist_explained`, `application_step_prepared`, `referred_to_authority` and `next_step_unresolved`. A completed navigation session can coexist with an unresolved entitlement barrier. Completion of the health visit, acceptance of NGO help, service reporting, beneficiary confirmation and public-benefit approval remain distinct events.

Acceptance and conflict transaction

Provider acceptance is an online server transaction, never a client-only button. Lock the need, current consent and relevant offer/capacity row in a consistent order. Recheck provider/assignment permission, state/version, consent still active, confirmed eligibility, catchment, valid capacity window, offer expiry and any required funding authorization. Atomically reserve one unit only if available, create the allocation, and transition the attempt/need to accepted. Commit together; failure leaves no reservation. Add database constraints for one active allocation per need/service request and nonnegative capacity.

Every mutating request includes stable event ID, actor, client timestamp and expected record version. Duplicate event IDs return the stored result without a second allocation or visit count. A different event against a stale version returns an explicit conflict and current authoritative state. Two provider acceptance requests for one remaining slot cannot both succeed; the loser receives `capacity_unavailable` or `allocation_conflict` and is sent for coordinator review. An expiry/cancellation race uses the same locks and state checks. Release capacity exactly once on cancellation or withdrawal; consumption after delivered service remains a historical fact, not a new free slot in the same accounting window.

Capacity is accounted as `total = available + active reservations + consumed` for the declared service window, with any authorised adjustment separately logged. A reservation expiry before arrangement releases its unit and returns the need to `manual_review` with `capacity_reconfirmation_required`; the UI must remove the booking promise. An arranged appointment uses the provider-confirmed appointment window and its cancellation policy, rather than silently expiring on a short offer-response timer. Delivered service atomically changes reserved to consumed once.

Withdrawal increments consent version and marks the case blocked for new sharing in the same transaction, cancels eligible active offers/arrangements, and releases unconsumed reservations. Notify the responsible provider through the product and log acknowledgment. It cannot recall information already legitimately viewed or undo completed service; retention/deletion follows the agreed policy. Late delivery evidence after withdrawal is quarantined for coordinator review, with factual chronology retained; it cannot silently reopen the case or imply renewed consent. Provider-reported delivery and beneficiary dispute are two append-only facts rather than last-write-wins edits.

Offline: a limited, honest promise

For the hackathon, cache only the synthetic assigned day's plan and minimal task references. A worker can read it and queue visit completion or a need draft with UUID event IDs; display **offline, pending server acknowledgment** and last-sync time. Downloaded matches are explicitly stale observations. Offline users cannot reserve shared NGO capacity, confirm provider acceptance, change approved clinical rules or finalize fund approval. An appointment slot becomes real only after online acceptance or a separately logged authorized coordinator channel.

Record local withdrawal immediately, stop further local sharing and prioritize its sync. Do not promise instantaneous revocation on another offline device. On reconnection, authenticate anew, recheck role/assignment and consent version, replay duplicate-safe events and show conflicts rather than merging clinically meaningful facts automatically. Server revocation rejects queued prohibited changes. Keep a visible pending/conflict count and allow safe correction; no hidden "sync successful" if some records failed. A production offline implementation needs device controls, encrypted

protected storage, expiry, loss handling, secure deletion and a defined revocation delay. A service worker caching synthetic data does not establish production privacy readiness.

Minimal stack and schema

Use one mobile-friendly TypeScript web application, one server process with relational transactions, and PostgreSQL. Keep planner, matcher, permissions, state transitions and CSV validation as modules in that process. A small scheduled job in the same deployment expires offers and creates follow-ups. No microservices, message broker, vector database, LLM dependency, public donor marketplace or payment gateway is needed. Use a locally drawn coordinate route and a table if map tiles cannot be loaded; the plan must remain reviewable without an external map service. Add deployment/tool versions only after the build environment is chosen and checked.

Record	Minimum useful fields and constraints
`User`, `StaffAssignment`	Role, organisation, permitted geography, assigned task/patient references, active/revoked status; server checks every query/mutation
`HouseholdRef`	Internal ID, restricted source patient ID, confirmed locality/coordinates/confidence; safe contact stored separately with restricted access
`RuleVersion`	Versioned programme/rule, authority/source, approval, due/priority/cadre/escalation policy; immutable once used
`VisitTask`	Source/task identity, household, staff/cadre, due window, supplied priority/flags, duration, availability, deferrals, independent visit state
`PlanSnapshot`, `PlanStop`	Date/staff, input/policy versions, travel assumptions, route/timings, status and rejected IDs/reasons; stale plan warning after changes
`NeedCase`	Household reference, category/barrier/public pathway, support owner/date, current version/state, active consent reference; clinical state not embedded
`ConsentEvent`	Purpose, intended recipient scope, approved fields, assisted/direct recording, actor/time, grant/withdrawal, version; append-only chronology
`OrganizationBranch`, `ServiceOffer`	Service/catchment/readiness, eligibility source/version, accessibility, hours, free/cost/restrictions, capacity unit/window/freshness/expiry
`ReferralAttempt`	Need, selected offer, permitted payload snapshot/version, deadline, accept/decline/expiry/cancel state and reason
`CapacityReservation`	Need/offer/window, quantity, active/consumed/released, reservation expiry; one active allocation per need/service request
`DeliveryEvent`, `FollowUp`	Provider report, permitted evidence, independent confirmation/dispute, owner/date and outcome; no compulsory beneficiary photos
`FundingRecord`	Optional budget/request reference, permitted cost/category, approval actor, external payment/in-kind receipt status; no balance/wallet
`EventReceipt`, `AuditEvent`	Unique event ID/idempotent result, entity/version, actor, transition, server time, source; logs omit unnecessary clinical/contact content

Minimal interface contracts: import/validate CSV; generate/read plan snapshot; submit visit event; create need/record consent; list explainable matches; send offer; accept/decline; arrange/report/confirm/dispute; withdraw consent; read coordinator unresolved queue. Server handlers enforce explicit transitions instead of letting clients set arbitrary status values. Rate limits, secure sessions, database constraints and ordinary error handling belong in the same backend. The exact database library and frontend framework are routine implementation choices, not distinguishing product features.

Access and data minimization

Worker access is limited to assigned cases. Supervisors see authorized geography; clinicians own clinical priority and health escalation. Providers see only requests offered to their service and fields consented for intake, with contact details released only at the agreed step. A provider cannot browse the caseload, route history, diagnosis or another NGO's cases. A funder receives approved project requests and sufficiently aggregated outcomes; there is no patient directory. Public access, if ever added, covers generic service information only.

Enforce role plus organisation, geography, assignment and purpose on the server; hiding a UI tab is insufficient. Demo accounts and fixtures are labelled. Record access/revocation and state transitions; provide a retention/deletion policy, safe-contact preference and grievance route before a pilot. Do not require a household smartphone, OTP, Aadhaar scan or public story for assistance. A provider badge states the exact identity/readiness/catchment checks and dates, rather than "government verified." Household health permission does not automatically authorize NGO disclosure or donor reporting.

Do not claim that CSV import is connected to RCH, U-WIN, Shaishav, ABDM or Darpan. Public standards and source-code availability are not record-access permission. ABDM can represent service requests, so the design also must not claim it is technically incapable of referrals. Production interoperability needs authorised agreements, tested mappings and security review. Official ABDM consent/sandbox/security description, published FHIR guide.

Funding is a record of decisions, without custody

The selected navigation demo uses synthetic in-kind appointment capacity at no beneficiary cost. Show a compact optional funding progression: `not_required` or `request_draft -> cost_reviewed -> approval_pending -> approved/declined -> external_disbursement_reported or in_kind_supplied -> receipt_recorded -> reconciled`. Approval has a named authorised NGO/funder owner; an app administrator does not invent that authority. If funding is necessary to provide the service, acceptance must be conditional until approval or a provider commitment to supply it without that funding is recorded.

These states neither transfer money nor certify financial compliance. No donations, custody, wallet, fund balance, escrow, "available CSR pool" or guaranteed grant is part of the 24-hour build. External receipt status is reported metadata with appropriate review, not payment-provider confirmation. Before actual money movement, partners need source/entity-specific financial and legal review. CSR Exchange itself distinguishes platform/project interaction from funding decisions. CSR Exchange FAQ.

24-hour delivery plan

Time budget	Buildable output
Hours 0-3	Freeze scope/contracts, seed fictional inputs/offers, create three role views and source/provenance labels
Hours 3-9	CSV preview/errors, staff-filtered deterministic plan, timings, risk explanations, unserved queue, locality groups and three-day projection
Hours 9-15	Need/permission capture, one service matcher, offered/accepted/declined/expired flow with atomic capacity
Hours 15-19	Appointment, delivery report/confirmation/dispute, coordinator queue, withdrawal and idempotent events
Hours 19-22	Limited synthetic offline read/queue, race/edge-case validation, baseline metrics and clear assumptions
Hours 22-24	Usability pass, seeded reset and rehearsed demonstration; fix core defects before optional features

Must: end-to-end CSV scheduling core; feasible route and return time; explicit priority provenance/urgent bypass; every unserved case visible; coverage projection; one source-linked public pathway; consented offer matching; provider acceptance with capacity transaction; independent reported/confirmed outcome; server role/assignment checks; visible

synthetic labels and honest baseline comparison.

Should: synthetic offline plan/read/queue with conflict display, capacity-expiry follow-up automation, Hindi labels reviewed by a fluent team member, assisted contact/consent, simple safe export, a read-only funding-state example. If time slips, keep offline claims limited and demonstrate its queue as a synthetic test scenario rather than asserting complete production support.

Later: jointly synchronized ASHA/ANM/AWW scheduling; cross-worker reassignment optimisation; realistic road matrices; authenticated state integration; validated multilingual voice; production device/offline security; multi-service referrals; financial integrations; generalized rule catalogue; large-scale provider onboarding. Joint-required cases must be flagged now even though joint-route optimization is later.

Ten meaningful demonstration/test cases

Scenario	Expected result
1. Repeat import; one patient has two valid task IDs	Repeat same source/task does not duplicate; distinct tasks remain visible; conflicting rows are reviewed
2. Remote high-priority household competes with three nearby routine tasks	Approved ordering preserves the remote case when feasible; increased travel is shown; routine misses remain visible
3. High-priority task cannot fit; repeated remote deferral	Explicit infeasible reason and supervisor review; fairness policy triggers visible action; no fake complete coverage
4. Missing coordinates, blocked access or expired rule	No precise invented location/ETA; task needs review; bucket denominators and review count are visible
5. Availability window, fixed session, buffer and return-to-base	Every selected stop and return is feasible; manual rearrangement that breaks constraints produces a warning
6. Approved urgent flag or joint-required task	Urgent pathway bypasses routine optimization; joint task awaits coordination; neither is counted as normal completed care
7. NGO outside catchment, capacity stale or eligibility uncertain	Offer fails hard filter or goes to review; a directory listing is not a booking; no fabricated fallback
8. Two acceptances race for one slot; same event retried	Exactly one allocation succeeds, retries are idempotent, losing request has a visible conflict without negative capacity
9. Offline completion followed by consent withdrawal/access revocation	Local draft remains pending; reconnect rechecks authority/consent; rejected updates cannot resurrect sharing or inflate completion
10. Offer timeout, reported delivery disputed, unauthorised provider lookup	Owner gets next-action queue; dispute excluded from confirmed success; provider cannot retrieve unrelated household/clinical data

Use assertions on state transitions, row counts, capacity, access and feasibility, not screenshots alone. These cover material safety and shared-state behavior; avoid spending the hackathon on tests that merely restate labels.

Metrics that do not manufacture impact

Compare against a reproducible baseline using the **same validated tasks, assignment, priorities, shift, availability and travel model**: priority/due-ordered task selection with append-only route construction in stable task-ID order inside ties. Show an optional simple distance-first route as an explicit tradeoff demonstration, with its priority misses exposed. Save the seed, inputs, algorithm versions and denominators. Do not compare an infeasible unrestricted baseline to a feasible constrained route without prominently reporting that difference.

Report estimated travel minutes, total planned minutes, visits served by bucket, due coverage, remote/locality coverage, overdue/deferral counts and the reasons for exclusions. A travel percentage is `(baseline - proposed) / baseline`

only when baseline is nonzero and comparable; report no improvement or increased travel when that is the result. Display **simulation estimate on synthetic data**. Do not promise 30% savings, lives saved, clinically safer classification, optimality or field-proven outcomes.

For referrals, show matching, offered, accepted, reported delivered, confirmed and unresolved counts separately. Demo wall-clock response times are scripted behavior, not service-level evidence. A pilot would measure operator response, time to accepted appointment, confirmed session completion, unresolved barrier aging, user burden and unserved fairness, with observation windows and exclusions stated. Programme benefit approval and health outcomes need their own evidence.

Shortcuts and pilot prerequisites

Adopted 24-hour shortcut	Deferred capability / condition for production
Fictional records, seeded providers, source-linked manual rule review	Actual partner authorisation, district rules, worker mapping and service due diligence
Single backend/database; assigned-worker planning	Joint/multi-worker optimisation only after realistic calendars, scope and responsibility are agreed
Coordinate estimates and editable assumptions	Validated travel modes, seasonal access, safe routes and usable local calibration
One sequential referral attempt at a time	Parallel attempts only with recipient-specific permission and conflict-safe unique allocation
Assisted permission and minimal case payload	Locally approved consent wording, withdrawal, retention, safeguarding and grievance procedure
Synthetic offline cache and queued drafts	Secure device/storage controls, loss response, remote revocation limitations and field sync testing
Manually seeded expiry/readiness rules	Named partner owner, operational reconfirmation process, agreed response/expiry policies
Funding metadata only	Authorised budget owners, reconciliation and entity/source-specific review before payments

Before a pilot, confirm what tools the catchment already uses and whether configuration or an add-on would serve it better. Obtain the actual 2026 programme rules and priority approval; a willing health team, coordinator and service providers; funded coordination and verification time; beneficiary/worker usability review; secure hosting, access auditing, tested backup/restore and incident handling; approved record-sharing and retention arrangements; a field-tested travel model; transaction/conflict tests; and realistic provider capacity/expiry/complaint procedures. Partner operations and physical service availability are prerequisites, not things software creates. Privacy implementation and legal review must use the applicable notified timeline rather than claiming that all DPDP provisions were fully effective on the report date. Official staged commencement notification.

Minimal winning demonstration

In four minutes: import the synthetic CSV; show a remote approved-priority visit preserved in a feasible day and one missed case with a reason; compare estimated travel and bucket coverage honestly; complete a visit and record a document-access barrier; show an out-of-catchment/stale provider rejected; obtain recipient-specific permission and have the eligible provider accept one current slot; record help delivered then independently confirmed; finish on the coordinator view showing the still-unresolved entitlement step. A brief replay of two competing acceptance events for the last slot demonstrates technical credibility. The story succeeds by showing one usable day and one accountable support handoff, with failure visible.

Part 12. Independent audit and remaining validation

Independent audit: research credibility and build readiness

Audit date: **8 October 2026, India**. This is the tenth assigned research review. I read the decision brief, method note, research contributions 01-09, solution design 11, additional comparators 12 and the report assembler. I independently counted the catalogue rows, checked cross-file consistency and reopened five material primary sources. No application was executed, authenticated platform tested, local partner contacted, legal certification performed or patient data accessed.

Verdict: suitable as a qualified decision report and for designing a synthetic prototype; not evidence of a ready production service. The five consistency findings raised during this audit were corrected and the updated text was reread. The strongest direction is the required visit planner plus one narrowly scoped, consented document-navigation referral. A live pilot depends on approved rules, willing providers, current local offers and funded coordination. Those conditions have not been secured.

The audit is independent in review responsibility, not an independent replication of every source in the book. Ten distinct assignments add perspectives; they do not turn shared government webpages or operator descriptions into ten independent confirmations. Agent completion, catalogue size and source count do not certify correctness or adoption.

Status definitions and audit table

PASS means the reviewed report represents the evidence or proposed boundary appropriately. It does not mean the capability exists in a built product. **FIX** is a concrete text/design correction. **BLOCKER - pilot** means the missing condition must be resolved before the relevant live operation; it does not prevent delivery of this desk-research report.

Exactly 24 additional programmes

Status: PASS

Finding and required interpretation: First comparison tables in 02, 04 and 06 each contain eight data rows: **8 + 8 + 8 = 24**. ASHA and SSBSK are core context outside this count. Umbrella components and access portals are not additional rows.

Exactly 30 platforms/services

Status: PASS

Finding and required interpretation: First comparison tables in 03, 05 and 07 contain eight rows each; 12 contains six: **8 + 8 + 8 + 6 = 30**. Give is one ecosystem; ABDM is one ecosystem; CSR data's historical/current views are one entry. These are not 30 unique independent companies.

Overlap and market accounting

Status: PASS

Finding and required interpretation: UIP/MI, NTEP/TB community support, maternal schemes and AAM medicines/diagnostics overlap. Counts are catalogue coverage, with no additive beneficiary total, obtainable market or available CSR balance.

Healthcare 03 remains the core

Status: PASS, provenance limit

Finding and required interpretation: CSV, supplied priority, geographic grouping/nearest-neighbour step, feasible route, travel assumptions, flags, coverage projection and unserved queue remain visible. Expanded requirements are explicitly carried forward from the earlier session reading; save/reread the official expanded panel before quoting it verbatim to judges.

Geographic grouping is mandatory in the proposal

Status: PASS - corrected

Finding and required interpretation: Earlier solution wording made clustering optional. The updated 11 and 09 now preserve an explicit basic geographic step. Advanced clustering can remain later. No claim of global route optimality is made.

Older/newer ANMOL deployment figures

Status: PASS - corrected

Finding and required interpretation: 01 now distinguishes the older 28-State/UT annual-report statement from the newer 29-State/UT statement as of 31 December 2025 used in 03. Neither proves every local worker actively uses the app.

Current child-health policy and prior art

Status: PASS

Finding and required interpretation: SSBSK is treated as a 2026 update. Shaishav is described/announced with local deployment unverified. Poshan Tracker's scheduler belongs to AWW work, without equating it to universal ASHA scheduling. Existing Indian and overseas referral products rule out first-ever claims.

Current approved visit/risk rules

Status: BLOCKER - pilot

Finding and required interpretation: The exact official SSBSK visit table, local risk rules, adoption, required cadre and escalation destinations remain unverified. Generic/synthetic rules can demonstrate software; legacy HBNC/HBYC dates cannot become current production defaults.

Clinical boundary

Status: PASS

Finding and required interpretation: Staff/approved rules supply priority; no AI diagnosis, autonomous final eligibility or funding-driven clinical score. Urgent care bypasses NGO matching. Joint-required visits remain visible and cannot be counted as independently fulfilled by one worker.

Local network readiness

Status: BLOCKER - pilot

Finding and required interpretation: No providers or catchments are secured. Registered office, national presence, Darpan identity and charity status do not establish quality, service fit, funded capacity or acceptance. Verify each branch/service offer, not only its organisation.

Coordinator ownership and economics

Status: PASS design; BLOCKER - pilot

Finding and required interpretation: The report assigns an owner, next action and funded coordination. No local staffing budget, procurement route or willingness to pay is verified. ASHA capture must not silently become unpaid donor chasing. Measure coordinator/provider effort separately.

Charity and public-service navigation

Status: PASS

Finding and required interpretation: Public pathways precede supplementary help. A private bridge preserves an unresolved entitlement barrier. Discovery, application, sanction, payment and receipt are distinct. Charity, directory presence and campaign publication never guarantee funds.

Consent and private data

Status: PASS design; BLOCKER - pilot

Finding and required interpretation: Recipient-specific permission, minimal payload, safe contact, refusal/withdrawal and restricted role/assignment access are specified. No public patient list, home map or fundraising story is needed. Approved wording, child/adolescent safeguards, retention and real security controls still require local review.

Offline behavior versus live capacity

Status: PASS design

Finding and required interpretation: Cached plans and queued events are limited promises. Offline matches cannot reserve shared NGO slots. 09 specifies reauthentication, stale-version conflicts, idempotency and withdrawal limitations across offline devices. Those are proposed controls, not tested capabilities.

Acceptance, delivery and closure

Status: PASS design

Finding and required interpretation: 09 separates a need from sequential offers, reserves capacity transactionally, distinguishes reported from confirmed delivery and preserves disputes/unresolved public barriers. A document-help session never proves benefit sanction or clinical recovery. Race, expiry and withdrawal tests remain to be implemented.

Coverage projection deadline wording

Status: PASS - corrected

Finding and required interpretation: 09 now advances the **planning date** while preserving approved task due dates/windows; only a separate reviewed source update can change deadlines. A build must test that forecast coverage never conceals overdue work by moving its deadline.

Stable import identity

Status: PASS - corrected

Finding and required interpretation: 09 now requires immutable source task identity, accepts a documented task-ID alias and binds a stable source/dataset identifier. It explicitly labels these as demo extensions to the PS. Missing identity goes to review; patient ID alone cannot distinguish legitimate multiple tasks.

One consistent demo scope

Status: PASS - corrected

Finding and required interpretation: The brief, 11 and 09 now agree on 20-30 synthetic tasks, one document-navigation service and three fictional offers. The larger 60-80-household scenario is an optional later stress test; other support categories are pilot extensions.

APIs and integration claims

Status: PASS

Finding and required interpretation: Public schemas, source code and operator API offerings are distinguished from production access. CSV is not a connected RCH/U-WIN/Shaishav/ABDM/Darpan integration. No fictitious public API, credential-sharing or deployed bidirectional sync is claimed.

Incentives, benefits, fees and funding

Status: PASS with dated limits

Finding and required interpretation: Cash amounts are tied to source dates/eligibility, ₹3,500 is not total ASHA salary, fees are campaign-specific rather than forecasts, historical CSR is not spendable cash. ADIP/RVY continuation and local funded availability have explicit gaps; obtain current authority confirmation before promising those services.

Impact, novelty and winning claims

Status: PASS

Finding and required interpretation: No lives saved, causal health improvement, guaranteed travel saving, NGO money raised or winning percentage is asserted. "World-best" may be an aspiration, never a researched ranking. Synthetic timings and acceptance outcomes are scripted demonstrations, not field evidence.

Assembly and presentation readiness

Status: PASS logic; final artifact verification pending

Finding and required interpretation: The assembler checks exact counts and requires all parts in final mode. Its source index is navigation, not independent-study count. The final report/PDF still needs assembly and visual verification by the parent; this audit does not certify those outputs.

Five primary-source spot checks

These checks support the narrow facts below, not every operational implication or universal implementation. Sources were reopened on the audit date using the built-in web tool; no additional broad Tavily search was used.

Primary document	Independently checked fact	What it does not prove
MoHFW SSBSK launch, 29 June 2026	Integrates HBNC/HBYC into birth-to-36-month home/community care; describes risk-differentiated and coordinated visits.	Exact local production calendar, trained readiness or live promised national integrations.
MoHFW SSBSK workshop, 25 August 2026	Describes Shaishav at-risk line-listing, visit scheduling/tracking, referral follow-up and supervision; discusses State implementation preparedness.	Universal live deployment, working third-party access or outcome effectiveness. One query variant timed out; the base release URL was successfully reopened.
Official POSHAN backgrounder, 14 April 2026	Describes the AWW home-visit scheduler integrated in April 2026, including 23 structured visits and automatic prompts.	Universal ASHA use, routing optimisation or independent proof of the source's broad delivery-success claims.
Rajya Sabha answer 276, 22 July 2025	Fixed routine/recurring incentive revised to ₹3,500 at the 4 March 2025 MSG meeting; performance incentives and State top-ups also exist.	October 2026 total earnings, timely receipt, salary status or current State-specific payment rules.
MeitY commencement notification G.S.R. 843(E), November 2025	Specifies commencement on publication for named provisions, after one year for others, and after eighteen months for many substantive provisions.	Complete legal compliance of a prototype, applicability of every other law, or absence of later changes. Deployment needs the applicable notified timeline and entity-specific review.

Prioritised corrections and evidence gaps

1. **Resolved in the report; verify in the build:** geographic grouping is explicit, ANMOL chronology is dated, projections preserve true deadlines, import identity is stable and demo scope is consistent. The deadline/identity corrections affect overdue visibility and duplicate visits; the proposed acceptance tests must demonstrate them before any implementation is called reliable.

2. **Keep the agreed hackathon scope:** one document-navigation service, a small synthetic caseload, basic geography, feasible time accounting and a visible unresolved case. Keep the 24-programme/30-platform breadth as research, with no obligation to implement every pathway.

3. **Before a real pilot:** obtain approved current rules, worker mappings and urgent clinical routes; onboard genuinely willing providers; verify actual service catchments and capacity; fund a named coordinator; agree safe assisted consent, safeguarding, data handling, complaints and completion evidence. If there is no accountable receiving person or physical capacity, do not promise service fulfillment.

4. **Before market or impact claims:** interview local workers, beneficiaries/caregivers and intake staff; trace recent successful, refused and failed cases; compare against the existing phone/register/app process. Evaluate build-versus-configure with the systems already used locally. No current field research establishes willingness to adopt, burden neutrality, procurement, prices, demand conversion or causal clinical benefit.

5. **Before promising specific entitlements or funding:** resolve programme continuation gaps and applicable State rules, verify local service readiness, and obtain provider/funder-specific authorisation. Operator documentation and government entitlement design are different from a funded slot today.

The concise decision brief is the right entry point for mentors. The longer report is a reference book; preserve the reading-order guide, clickable sources and visibly labelled limitations. A four-minute story should explain the actual planner, one failed match recovered by its coordinator, independently confirmed support and an unresolved public-service step. Ambition is credible when the working scope and evidence boundary are clear.

Primary-source and operator-document index

URLs are indexed for navigation, not a count of independent studies. Several URLs refer to related official documents or different language/query versions. Sources retain the dates and evidence limits specified in the report.

1. Official Codefiesta 5.0 problem statements

2. NHM: About ASHA

3. NHSRC: Community Processes-ASHA

4. Community Processes Guidelines, catalogued as 2014; forewords dated 26 June 2013; pp. 5-13

5. NUHM implementation framework, 2013-era framework

6. NHM orientation module: selection norms

7. Rajya Sabha question 2830, annexure I

8. PIB/MoHFW launch release, 29 June 2026

9. PIB/MoHFW SSBSK workshop, 24-25 August 2026

10. NHM SSBSK guideline catalogue, website updated 7 October 2026

11. NHM community-based child health programme

12. NHSRC urban ASHA induction module, publication day not stated in retrieved text; hosted under 2021 directory

13. Official Parliament answer

14. Lok Sabha question 1082, 2025

15. Primary study: work sampling assessment

16. NHSRC work report FY 2024-25, p. 38

17. MoHFW Annual Report 2024-25, section 1.2.6

18. MoHFW Annual Report 2025-26, p.50

19. PIB: Update on U-WIN, 24 March 2026

20. Official Swasth Bharat portal

21. NUHM guidelines for ASHA and MAS, NGO section p. 23

22. NHM: Janani-Shishu Suraksha Karyakram

23. NHM JSY page

24. CAG Haryana public-health audit

25. NHM JSSK page

26. WCD PMMVY page

27. Eligibility order

28. age order

29. time-limit order

30. Mission Shakti implementation guidelines

31. PMMVY citizen portal

32. PIB/WCD update

33. PMMVY 2.0 FAQ

34. Helpline announcement, 24 Oct 2025

35. Official PMSMA portal

36. PMSMA operational guidelines

37. Official SUMAN portal and charter

38. MoHFW/PIB IMI 5.0 release, 12 Oct 2023

39. MoHFW Annual Report 2024-25, immunization chapter and schedule, p. 72

40. Maharashtra NHM routine immunisation page, 17 Apr 2025

41. NHM Immunization Handbook for Health Workers, 2018

42. MoHFW RML Hospital official key-features page

43. Cabinet approval, 11 Sep 2024

44. NHA senior-citizen FAQ

45. PIB Delhi implementation update, 5 Apr 2025

46. MoRD, 1 Jan 2026

47. MoRD, 17 Mar 2026

48. NSAP guidelines, Oct 2014

49. Revised guidelines, effective 26 Sep 2024

50. ARJUN

51. ALIMCO official FAQ, undated

52. DSJE annual report 2025-26, pp.90-92

53. MoHFW, 24 Jul 2026

54. MoHFW, 3 May 2026

55. 2013 operational framework

56. NHM adolescent-health page, updated 3 Aug 2026

57. NHM AFHC page

58. NHM peer programme, updated 14 Sep 2026

59. Government reply, 1 Apr 2026

60. DGHS NMHP page, undated

61. MoHFW, 19 Aug 2025

62. MoHFW, 9 Dec 2025

63. DGHS NTEP page

64. CTD guidance, 2026

65. community-support portal

66. DEPwD scheme page

67. e-Anudaan portal

68. MoHFW launch, 29 Jun 2026

69. MyScheme entry

70. NHM programme page

71. NHSRC programme description

72. MoHFW parliamentary reply, PIB

73. MoHFW year-end report, PIB

74. NHSRC Community Process-ASHA

75. MoRD parliamentary reply, PIB

76. PIB programme backgrounder

77. official explainer

78. DFPD/NFSA responsibilities

79. NFSA salient features

80. Cabinet decision, PIB

81. WCD scheme guidelines

82. current PIB backgrounder

83. Union Budget expenditure profile

84. Cabinet approval, PIB

85. 2025 service description

86. government guidelines index

87. PMBI Kendra opening guidelines

88. operator agreement

89. new JJM 2.0 English guidelines

90. December 2019 operational guidelines

91. December 2021 official Samvad publication

92. online grievance utility

93. v5.0.12 manual, 28 May 2024

94. NHM IT initiatives e-booklet

95. NHM Maharashtra official workflow

96. PIB official update, 29 Nov 2024

97. Rajya Sabha answer, 10 Mar 2026

98. PIB 2026 factsheet

99. Official product page, undated

100. Operator FLW repository

101. FLW REST API microservice

102. AMRIT platform module map

103. Current product site

104. Precision Tasking article, Sep 2021, still publicly available

105. official India page

106. operator's 2025 security playbook
107. Operator API documentation
108. 4 Mar 2026 operator announcement
109. Primary study, 2019, using 2012-14 data
110. Official MoHFW service summary
111. 2026 parliamentary response
112. C-DAC product page
113. NHM Maharashtra workflow
114. official Android listing, updated 24 Aug 2026
115. Official HFR announcement, 6 Jul 2022
116. PIB, 5 Aug 2025
117. NRCeS current published FHIR guide v6.5.0, generated 8 May 2025
118. Official About page
119. 2022 official community-support guide
120. 2023 family-care guide
121. linked official knowledge base
122. MoHFW Play listing
123. API Authentication page, updated 19 Apr 2024
124. Mathematica issue brief, 20 Oct 2015
125. v7 preview
126. VAC
127. directory use
128. Portal notice
129. Official site
130. active certification resources
131. Platform terms
132. Discover
133. Give.do overview
134. Diligence service
135. seal criteria
136. Claim process
137. Nonprofit terms
138. how vetting works
139. 2025 diligence explanation
140. API
141. developer terms

142. About and operator footer
143. Terms
144. nonprofit service
145. Vetting service
146. REST docs
147. MCP announcement
148. MITRA
149. iVolunteer scope
150. NGO services
151. Organization signup
152. About
153. NGO service
154. volunteer filters
155. corporates
156. Published onboarding terms
157. official signup guide
158. Ongoing compliance
159. Operator
160. FAQ
161. NeGD project
162. Current service homepage
163. NeGD RFP
164. State department description
165. Tariff
166. Portal
167. Official FAQ
168. Current CSR data
169. Operator
170. Developer listing
171. About
172. Pricing
173. Operator overview
174. Pricing
175. Withdrawal help
176. Terms
177. Digital India Week release

178. official eBook

179. PIB June 2026 account

180. 6 June 2022 MCA/PIB release

181. Lok Sabha answer dated 18 December 2023

182. 9 February 2018 MCA/PIB release

183. CDM FAQ

184. CDM about page

185. terms

186. operator NGO article dated 13 July 2024

187. Operator overview

188. 2021-22 annual report

189. Operator documentation, features as of release v1.0.1 / Sep 2023

190. overview

191. Immunization package

192. Official referral product

193. Community organization offering

194. Official platform page

195. Sending programme referrals

196. NHSRC/MoHFW 16th Common Review Mission, 2024

197. Kar et al., RBSK caregiver study, 2020

198. Shetty et al., ASHA work sampling, 2025

199. Carmichael et al., Bihar ICT-CCS cluster trial, 2019

200. Official commencement notification, G.S.R. 843(E)

201. PIB/MCA, 10 February 2026

202. MoHFW SSBSK workshop, 25 August 2026